BerryDunn

Iowa (IA) Department of Health and Human Services (HHS)

Comprehensive Child Welfare Information System (CCWIS) Procurement Support

Implementation Advance Planning Document (IAPD)

Version 0.1

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berrydunn.com

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Version	Delivered Date	Version Notes
0.1	12/8/2025	Submitted to ACF

1.0 Executive Summary

Beginning January 1, 2014, the State of Iowa (Iowa) Department of Health and Human Services (HHS), in consultation with the Administration for Children and Families (ACF) Children's Bureau (CB), started to transition from their legacy child welfare system, Family and Children Services (FACS), to a non-SACWIS (Statewide Automated Child Welfare Information System) system. Iowa HHS has conducted planning activities for several years to develop a new system. In compliance with 45 Code of Federal Regulations (CFR) 1355.52 (i), Iowa transitioned the Safety, Timeliness, Assessment, and Response (STAR) Intake & Assessment module within Joining Applications and Reports from Various Information Systems (JARVIS) in August 2018 as one component of a new Comprehensive Child Welfare Information Systems (CCWIS) system.

In June 2021, HHS submitted an Implementation Advance Planning Document (IAPD) detailing its multiyear plan to build a modular system to comply with federal regulations. The IAPD was approved by the ACF in July 2021.

In 2021, the VISION project began with the goal of modernizing HHS's child welfare system by replacing outdated technology with a streamlined, user-friendly platform that meets federal CCWIS standards. This upgrade would aim to improve case management, reduce administrative burdens, and give staff more time to focus on families. Alongside technological improvements, the project focused on incorporating data-driven tools, policy updates, and workforce support to enhance prevention efforts and help ensure better outcomes for children and families through more efficient, integrated services.

In July 2022, HHS submitted a Data Quality Plan and a combined Operation Advanced Planning Document (OAPD) and Annual Advanced Planning Document Update (AAPDU) in accordance with federal regulations; ACF approved both submissions.

In both 2023 and 2024, HHS submitted an AAPDU with the attached CCWIS Automated Function Checklist, in accordance with federal regulations, and ACF approved both.

Effective June 2024, HHS paused the VISION project, including solutions design and software development, to assess the internal design and development strategy. This pause enabled a thorough review of the project's direction, design, and approach. HHS intended to enhance the project's overall efficiency, effectiveness, and long-term success by doing so. In November 2024, after carefully considering its options and ongoing discussions with the ACF technical partners, HHS determined that it would formally shift back to a planning phase at ACF's request.

In late 2025, Google Public Sector (Google) provided Iowa HHS a novel opportunity to engage their Rapid Innovation Team (RIT) to develop a prototype by using the Lucid chart diagrams and process maps that had previously been designed and developed by HHS. The initial prototyping work, focused on case management, proved to be a great success. Due to the detailed design work completed by the HHS VISION team, Google's team was able to complete the prototype in less than 3 weeks, when they had estimated at least 6 weeks. Google's technical expertise, combined with sophisticated work products completed by HHS, allowed for a prototype development that was lightning fast and demonstrated the transformative power of

what is possible with CCWIS when harnessing the tools available with one of the world's leading IT companies.

HHS has identified Google as the best entity able to meet HHS's technical, financial, and timeline requirements for CCWIS modernization. This decision is based on a singular convergence of technical capability, executive commitment, and operational risk assumption that effectively precludes competition. Currently, Google is the only hyperscale technology provider willing and able to serve as the prime system integrator, assuming total accountability for the outcome while providing a vertically integrated technology stack (Infrastructure, Security, AI, and Geospatial Data) that can be deployed within 12 months. This document will serve as HHS's IAPD submission to ACF for the implementation of CCWIS.

1.1 Overview of Project Expectations and Goals

The expectations of the child welfare system in Iowa have increased while staffing to support practice has continually decreased. Based on these constraints, HHS determined that improving the safety, stability, and well-being of children in Iowa requires enhancing business processes and leveraging proven technology to improve the efficiency and effectiveness of programs and services. In agreement with ACF, HHS determined the best course of action was to become non-SACWIS compliant while pursuing a new CCWIS solution.

By acknowledging the limitations and shortcomings of existing applications, HHS is committed to establishing a CCWIS that meets the federally prescribed CCWIS requirements, conforms to Iowa's needs and HHS's enterprise architecture standards, and effectively aligns with child welfare best practices. The move to a CCWIS will also support HHS's vision of modernizing its systems and processes, as it will retire one of the primary mainframe applications that exists today.

Upon initial review, HHS determined that a custom-built system would best meet its needs. However, after further review, HHS decided that an Accelerator Solution or a combination of out-of-the-box and customized modules would leverage current and emerging technologies and provide the most efficient use of resources.

HHS completed an analysis of child welfare case management systems and found the CCWIS-compliant system must include the following functionalities:

- Interface with STAR Intake and Assessment in JARVIS
- Plan and Manage Cases (Case Plan and Review; Court Management; Safety Assessment; Risk Reassessment; Contact and Visit Notes; Caseload Dashboards)
- Manage Providers for Services and Placements (Service/Placement Matching; Contract and Licensing Support; Recruitment, Retention, Training, and Support)
- Manage Funding (Eligibility for IV-E and Other Programs; Initial Eligibility Determinations; Changes to Eligibility Determinations)
- Manage Payments (Payment Approval Flows)

- Manage Data Distribution and Exchange (Deliver and Exchange Information through Portals; Manage Interfaces)
- Manage Closure or Transfer (Case Closure; Case Transfer; Interstate Compact)
- Develop Vision and Strategy (Federal and State Reporting)
- Develop Product and Services (Manage Configuration; Develop and Deliver System Support)
- Underlying Architecture (Mobility, Electronic Document Management, E-Signature, Microsoft Exchange Integration)

1.2 Statement of Executive Support for the Project

HHS is committed to the successful development and implementation of a CCWIS. HHS, the Department of Management (DOM), and the Department of Information Technology (DoIT) leadership and professional staff will collaboratively guide initial planning efforts and remain involved in ongoing planning and development throughout the project.

Executive and Steering Committees

The executive steering committee, listed in **Table 1-1**, meets quarterly or more frequently as needed to make executive level decisions on project direction, approach, and funding. **Table 1-2** lists the steering committee, which meets monthly or more frequently as needed to make leadership decisions on project resources, timelines, risk and issue management, and is responsible for overseeing the project team.

Table 1-1: Executive Steering Committee

Organization	Name	Title	Role
Iowa HHS	Larry Johnson	Director of Health and Human Services	Executive Leader
Iowa HHS	Sarah Reisetter	Deputy Director of Health and Human Services	Executive Leader
Iowa HHS	Janee Harvey	Division Director	Executive Sponsor
Iowa HHS	Matt Haynes	Bureau Chief	Lead Business Owner
Iowa HHS	Julia Webb	Bureau Chief	Advisor
Google	Krystal Van Der Brink	Project Manager	HHS-Side Project Manager
Contractor	TBD	Project Manager	DOM DoIT-Side Project Manager

Contractor	Edward Young	Architect	Project Technical Architect
Iowa DOM DoIT	Steve Kinney	Information Technology Administrator 4	Advisor

Table 1-2: Steering Committee

Organization	Name	Title	Role
Iowa HHS	Janee Harvey	Division Director	Executive Sponsor
Iowa HHS	Matt Haynes	Bureau Chief	Lead Business Owner
Iowa HHS	Julia Webb	Bureau Chief	Advisor
Iowa HHS	Mike Kitzman	Executive Officer 3	Advisor
Google	Krystal Van Der Brink	Project Manager	HHS-Side Project Manager
Contractor	TBD	Project Manager	DOM DoIT-Side Project Manager
Contractor	Edward Young	Architect	Project Technical Architect
Iowa DOM DoIT	Bob Norris	Information Technology Enterprise Expert	Product Owner
Iowa DOM DoIT	Steve Kinney	Information Technology Administrator 4	Advisor

1.3 Summary of the Planning Phase Activities

HHS has completed the necessary planning tasks to implement the new CCWIS and make it operational. **Table 1-1** below highlights the significant planning activities to date.

Table 1-3: Planning Activities to Date

Planning Activity	Activity Summary	Year Completed
Request for Information (RFI)	Completed an RFI to solicit information and responses from vendors regarding solutions to develop and build a CCWIS.	2018
Current State Mapping	Performed a mapping assessment of business processes to support a gap analysis.	2018
Work Breakdown Structures	Collaborated with all areas involved in child welfare to guide requirements gathering and the gap analysis.	2018
Gap Analysis	Documented the gaps between the existing systems and the envisioned future system.	2018
Planning Advance Planning Document (PAPD)	Submitted notice of intent to become CCWIS compliant to federal partners.	2018
Leading Agile Training	Completed training for fundamental techniques, concepts, and tools to prepare HHS for the design, development, and implementation (DDI) phase of the project.	2019
Data Dictionary	Identified data elements and purposes to support the development of a data conversion plan by identifying data dependencies and data usage requirements.	2019
Data Governance Strategy	Developed a strategy to guide future data quality and data management plans.	2019
First IAPD	Submitted the initial IAPD funding request for the CCWIS, including justification of its need and value.	2021
Project Team Creation	Establish a project team consisting of: project manager, business analysts, solutions architect, product owners, field users, and IT support.	2021
Communication Plan	Identifies stakeholders, their level of communication needs, and strategies for appropriately involving them in the project.	2021
Data Quality Plan	Submitted a Data Quality Plan for approval by federal partners.	2022
Annual Advance Planning Document (APD) Update	Submitted an AAPD-U for approval by federal partners.	2022
Memorandums of Understanding and Interface Agreement Specifications	Documented the data sharing agreement between internal and/or external systems.	2022

Planning Activity	Activity Summary	Year Completed
Data Conversion Plan	Developed a plan and any activities associated with data conversion.	2022
Develop Request for Proposal (RFP) with BerryDunn	Developed an RFP to solicit information and responses from vendors regarding solutions to develop and build a CCWIS.	2025
Second PAPD	Per the request of ACF in November 2024, submitted second PAPD. Given changes at ACF in 2025, HHS was advised to withdraw the PAPD and submit an IAPD in its place.	2025
Business Requirements Documentation	Documented, tracked, and validated requirements for each system module.	2025
Requirements Traceability Matrix (RTM)	Traced all requirements through the project to their test cases.	2025
Google Prototype of Initial Case Setup	Google's Rapid Innovation Team (RIT) developed a prototype of the initial case setup in 3 weeks based on Iowa's detailed Lucid design work and Joint Application Development (JAD) sessions.	2025
Lucid Design and Google Prototype Demonstration	Iowa's VISION Field Team provided an overview of the depth and breadth of its existing Lucid design work and Google demoed its prototype for members of Governor Reynolds' cabinet, HHS leadership, DOM DoIT leadership, ACF Principal Deputy Assistant Secretary Andrew Gradison, and several of Mr. Gardison's staff. Iowa's VISION Field Team indicated it was ready to move into active development immediately, as Google demonstrated an ability to understand and develop functionality that surpassed Iowa's CCWIS expectations, coupling Iowa's design with an AI assistant utility.	2025
Two-Day On-Site Workshop with Google	Given the success of the joint HHS and Google demonstration, HHS and DOM DoIT representatives met with Google leadership and system development representatives to answer questions and draft a plan to replace FACS by the end of 2026.	2025
Alternatives Analysis	Examination and comparison of system solutions to meet Iowa's requirements.	2025
Cost Benefit Analysis	Estimation of costs and benefits for viable alternatives.	2025
Project Roadmap and Release Strategy	Plan to capture the project's life and communicate any dependencies between releases.	2025

Planning Activity	Activity Summary	Year Completed
Risk Management Plan	Identifies and analyzes risk and develops risk mitigation strategies.	2025
Business Continuity Plan	Outline preparedness, response, and recovery strategy for CCWIS operations and systems	2025
Second IAPD	Funding request for the CCWIS, including justification of its need and value.	2025

1.4 Summary of Any Requests for Waivers

Though HHS intends to eventually rebuild them into the new CCWIS, the JARVIS child protection Intake and Assessment modules were granted a waiver in 2019 to remain in place. .

1.5 Summary of Total Project Cost Estimate

HHS has conducted all necessary planning activities required by ACF to justify replacing HHS's current child welfare information system, FACS, as well as JARVIS functionality immediately thereafter. These planning efforts culminate in this IAPD, which serves as the final plan, business case, and detailed project management requirements for the CCWIS project.

Google's vendor-hosted, managed, and supported approach will meet Iowa's CCWIS needs as proposed in their contract. Google's solution will provide HHS with a complete, unified CCWIS system to replace both FACS and JARVIS entirely within 2.5 years. This delivery is significantly faster and less costly. **Table 1-4** provides the projected cost of Google services, licenses, software, and state staff resources beginning in state fiscal year 2026 quarter three (SFY2026 Q3) through 2027 quarter two (Q2). Additional cost details are included in **Section 9.1** budget figures.

Table 1-4: Alternative Google Solution Cost Summary

Alternative Google Solution	
DDI Costs Years 1 - 3	
DDI Costs Year 1 (SFY26 – 27)	$45,786,250
DDI Year 2-3	$38,000,000
Subtotal DDI Costs	
Maintenance and Operations (M&O) Costs Years 4 - 8	
M&O/Enhancement Costs	$25,000,000
Subtotal M&O Costs	
Total System Costs Years 1 – 8	**$108,786,250**

Table 1-5 summarizes the total project costs with 50% Federal Financial Participation (FFP) and the 50% total cost required by Iowa (state match).

Table 1-5: Summary of Potential Costs and Net Impact on State Funds

Alternative Google Solution Total System Costs Years 1 – 8	
Summary of DDI and M&O Costs	
Subtotal DDI Costs	$83,786,250
Subtotal Modernized System's M&O Costs	$25,000,000
Total DDI and M&O Costs	
CCWIS FFP at 50%	$54,393,125
Total Quantitative Benefits	
Net Impact on State Funds (State Match at 50%)	$54,393,125

1.6 Summary of Initial Project Funding Request

Table 1-3 summarizes the total cumulative annual costs included in Section 1.5.

Table 1-3: Summary of Potential Costs and Net Impact on State Funds

Year	Cumulative Cost – Vendor-Hosted, Managed, and Supported
2026	$45,786,250
2027	$67,786,250
2028	$83,786,250
2029	$88,786,250
2030	$93,786,250
2031	$98,786,250
2032	$103,786,250
2033	$108,786,250

2.0 Results of Activities Under the PAPD

2.1 Background

HHS implemented the FACS application in 1995 to facilitate the exchange of child welfare information more easily and efficiently. Still in operation, FACS has become an outdated, complex system that hinders the implementation and facilitation of policy and practice updates, rather than supporting them. The application was designed around client-server technology and currently focuses on initial case setup, case reviews, service and placement selection, payment setup, and payment approvals. Due to policy, practice, and technology changes, the JARVIS application was introduced in 2014. This application contains the intake and assessment functions for child protective assessments and current non-CCWIS tasks, including case planning, safety and risk assessment, IV-E eligibility documentation, and contact and visit notes. The non-CCWIS items in JARVIS will be incorporated into the new CCWIS system.

Due to the current age and limitations of FACS, numerous inefficient processes have been created to enter data and ensure the same information can be viewed, added to, and updated across both applications (JARVIS and FACS). **Through a detailed analysis and evaluation of the FACS application, HHS has identified a list of key system shortfalls, which include, but are not limited to:**

- Current technology is expensive to maintain
- System maintenance is time-intensive
- FACS is a decreasingly viable product, and HHS cannot run the risk of it becoming technologically obsolete
- Disjointed data entry with no logical workflow or support to guide targeted decision-making for users
- Code-heavy structure requires users to look up and memorize numerous alphanumeric codes for entries
- Duplication of entry due to the inability to completely interface or communicate between systems
- Limited capability for systems to generate required and necessary reporting without time-intensive data extraction and manipulation
- Poor data quality or missing data
- Lack of support for mobile technologies
- Lag time in communication between systems and reporting

2.2 Assessing the Current Environment

Each year, over 47,000 children interact with Iowa's child welfare system. HHS's priorities are child safety, family preservation when possible, and supporting permanency and well-being.

Modern technology is critical to improving efficiency, addressing staffing challenges, and enhancing outcomes for children and families.

The Family First Prevention Services Act (FFPSA) necessitates updates to Iowa's Child Welfare Model of Practice, but the FACS cannot support these changes. Implemented in 1995 on outdated mainframe technology, FACS is costly to maintain, difficult to learn, and slows casework.

FACS handles core tasks such as case setup, court tracking, eligibility, service authorizations, provider payments, licensing, and Adoption and Foster Care Analysis and Reporting System (AFCARS) reporting. However, it lacks prompts, family identifiers, and form integration, causing delays, duplicate work, and payment errors.

JARVIS, introduced in 2014, improved intake and assessment with a user-friendly interface and added features like case planning, risk assessment, and IV-E eligibility. Yet, FACS and JARVIS do not integrate well, leading to duplicate data entry and delays due to batch processing.

Additional externally supported systems - Results Oriented Management (ROM) and CareMatch - further fragment the workflow, creating inefficiencies and inconsistent reporting. A modern, integrated solution is essential to meet federal requirements and improve service delivery. The detailed current system environment and system needs are included in **Section 3**.

2.3 Understanding the Alternatives

HHS conducted an extensive review of options for replacing its outdated child welfare systems, FACS and JARVIS, to meet federal CCWIS requirements and modernize service delivery. This process began in fall 2017, when HHS engaged in discussions with ACF, consulted with the Casey Foundation, and held calls with human services staff in other states to understand best practices and considerations for system replacement.

To further inform decision-making, HHS issued an RFI to identify available products and approaches. Insights from the RFI led to the creation of the VISION project in **January 2021**, which pursued a custom-developed solution supported by internal and contracted staff.

However, by June 2024, the VISION project was paused due to delayed production releases, escalating costs, and the anticipated burden of maintaining a custom-built system. During this period, the DOM DoIT reorganized and adopted a policy to discontinue support for large scale internal custom builds, reinforcing the need to pivot toward vendor-supported solutions.

Following this, HHS identified guiding principles for the new solution:

- It must achieve CCWIS certification.
- It must support modularity and interoperability through well-documented Application Programming Interfaces (APIs).
- It must align with existing enterprise technologies such as identity management and document management.

The following alternative types were assessed using the required streamlined approach recommended by ACF:

- **New Development Options**:
    - *Full Internal Build*: Building the entire system internally was eliminated following DOM DoIT's policy change and the resource constraints highlighted by the VISION project.
    - *Hybrid Internal Build with Commercial-off-the-Shelf (COTS) Modules*: Combining internal development with specialized COTS modules, which conflicted with DOM DoIT's policy against large scale internal builds.
- **Accelerator Solutions (COTS):**
    - *(COTS) Vendor-Hosted, Managed, and Supported Solution*: A vendor-managed and hosted system licensed to HHS, offering vendor expertise but introducing cost and dependency risks.
    - *(COTS) with IT Participation*: Purchasing a COTS product with rights for HHS to customize and maintain, providing flexibility but requiring significant resources that would need to be largely procured externally.
- **Status Quo**: Retaining FACS and JARVIS was deemed infeasible due to their outdated architecture, difficulty in maintenance, and inability to meet CCWIS modularity requirements.

Ultimately, internal build options were ruled out due to organizational capacity and policy constraints, leaving the accelerator solution, vendor-hosted/COTS-based solutions as the most viable path forward to best support compliance, extensibility, rapid deployment, and align with HHS's modernization goals and lessons learned from prior efforts.

In early 2025, Iowa partnered with BerryDunn to develop detailed requirements to procure a vendor-hosted/COTS solution. BerryDunn helped identify and refine CCWIS requirements using federal standards, user feedback, technology strengths, and best practices. Through an iterative process involving subject matter experts (SMEs), HHS developed a detailed RTM. This RTM was initially intended to guide the selected vendor's CCWIS development and implementation process through the RFP process. However, HHS instead used the RTM and other RFP documents to further enrich Google's understanding of its CCWIS requirements and system environments, allowing Google to better plan an approach to replace FACS and develop the proposed contract.

3.0 Statement of Needs and Objectives of the IAPD

The purpose of this project is to enhance the child welfare system to support HHS's goal of helping Iowans achieve healthy, safe, stable, and self-sufficient lives through the programs and services provided. HHS had previously pursued a SACWIS solution that failed to deliver significant business value. In agreement with the ACF, it was determined that the best course of action was to become non-SACWIS compliant while pursuing a new CCWIS solution. By acknowledging the limitations and shortcomings of existing applications, HHS is committed to establishing a CCWIS that meets the federally prescribed CCWIS requirements, aligns with Iowa's needs and HHS's enterprise architecture standards, and effectively aligns with child welfare best practices. The move to a CCWIS will also support HHS's vision of modernizing its systems and processes, as it will retire one of the primary mainframe applications that exists today. HHS will still need to support the legacy FACS as a non-CCWIS until a new system is available to replace it entirely.

3.1 Current System Environment

The implementation of the FFPSA has necessitated updates to HHS's Child Welfare Model of Practice. As HHS continues implementing the FFPSA, the system must be flexible enough to accommodate various changes. FACS is not capable of this. FACS is currently primarily responsible for the following functionality, as captured in **Figure 3-1** on the following page.

- Initial case setup
- Court tracking
- Referrals for child support
- Setting up a fund source for eligibility determinations
- Opening new services and/or placements
- Setting up and authorizing provider payments
- Licensing and regulatory functions
- Data provider for hundreds of reports and data extracts, including AFCARS



3.2 System Needs

HHS implemented the FACS application in 1995 as an efficient and effective method of exchanging child welfare information. Today, FACS is operated and maintained, and has become an outdated, complex system that hinders the practice and implementation of policy and practice updates rather than supporting them. The application was designed around client-server technology and currently focuses on initial case setup, case reviews, service and placement selection, payment setup, and payment approvals. Due to policy, practice, and technology changes, the JARVIS application was introduced in 2014. This application contains intake and assessment functions for child protective assessments, including case planning, safety and risk assessment, IV-E eligibility documentation, and contact and visit notes, which are currently non-CCWIS. The non-CCWIS items in JARVIS will be rewritten into the new CCWIS system.

HHS implemented FACS using client-server technology and an Integrated Database Management System (IDMS) architecture. HHS attempted to become SACWIS-compliant in the past, but the transition would have been too time-intensive and costly and would not have delivered significant business value.

Today, FACS operates as an outdated system that costs over $1 million annually to maintain and requires extensive training and time to become competent, resulting in operational challenges.

FACS user challenges include:

- For a new social worker, learning FACS is one of the biggest challenges at a time when it is most important to understand policy and practice associated with the direct work done with children and families. Training for FACS alone requires a minimum of 10 hours of intensive classroom work and relies on memorization and repetition to feel proficient with the system.
- The mainframe system is difficult to see and tricky to navigate, even for the most experienced users. Completing a single task often requires navigating across multiple screens, placing the cursor in the correct location, and manually entering entries. There are no quick drop-down selection options in FACS; therefore, clicks and typed entries quickly slow end-user completion of work.
- Learning FACS is more challenging for new staff who may be used to updated technology and intuitive efficiencies.
- A social worker must individually determine which tasks comprise their workday without the aid of helpful system prompts to help them figure out what is left to be done.
- The time spent completing initial and ongoing entries in FACS is significant and can delay service referrals.
- FACS does not include functionality that helps workers identify families in the system, making it challenging to identify a family's history with HHS.
- Payment errors are challenging to identify and time intensive to fix, causing delays in payment to providers.
- The use of forms is not supported by FACS, requiring workers to produce written work that duplicates what has already been entered.

Additional FACS functionality limitations and challenges have been compiled by category in the following **Table 3-1**.

Table 3-1: FACS Functionality Limitations and Challenges

Category	Limitation/Challenge
Person-Centered Data	Inability to capture detailed demographic information (e.g., height, weight, tattoos)
	Limited functionality for adding or editing health-related data (e.g., appointments, diagnoses, community services)
	No capability to record Native American heritage, tribal enrollment, or Indian Child Welfare Act (ICWA) data for children
	Lacks features to connect children with relatives or fictive kin for placement purposes

Category	Limitation/Challenge
	Inability to document/track community-based services
System Usability and Navigation	FACS is a person-based system, which does not align with task or case-based workflows
	Staff must independently determine the tasks required to manage their caseloads
	The system lacks intuitive workflow guidance and user-friendly navigation
	Staff must memorize codes or rely on repetitive steps to perform basic functions
	No role-based dashboard to provide a comprehensive view of caseloads, tasks, and calendars
Data Entry and Processing	Redundant and repetitive data entry is required across multiple records
	Lack of auto-population features and absence of electronic signature functionality
	Real-time data processing is not supported; the system relies on overnight batch processing
	Data quality is inconsistent, with missing or outdated information
	System availability is often interrupted during batch processing windows
Technology and Integration	The system is not mobile-friendly and cannot be used efficiently remotely
	No integration with external systems; users must log into multiple platforms to access necessary data
	Does not support bidirectional data exchange with key partner agencies (e.g., Juvenile Court Services [JCS], Iowa Department of Education [IDOE], Iowa Department of Corrections [IDOC], Medicaid, National Electronic Interstate Compact Enterprise [NEICE])
	Communication within the system is delayed, impacting timely reporting
	FACS is not compliant with AFCARS requirements
Supervisory and Reporting	Supervisors lack visibility of their staff's caseloads, including task completion statuses
	No ability to flag important indicators, such as parents with a prior TPR (Termination of Parental Rights)
	Limited reporting capabilities hinder data-driven decision-making

Category	Limitation/Challenge
	The system lacks critical indicators such as allergies, Safe Haven status, ICWA involvement, and runaway alerts
Communication and Contact Efficiency	No one-click functionality to initiate phone calls or emails directly from the system
Sustainability and Viability Concerns	Current technology is costly and time-consuming to maintain
	FACS is becoming increasingly obsolete, posing a significant operational risk for HHS

Due to the current age and limitations of FACS, numerous inefficient processes have been created to enter data and ensure the same information is visible across both applications (JARVIS and FACS). Through a detailed analysis and evaluation of the FACS application, HHS has identified key system shortfalls.

FACS key system shortfalls include:

- Current technology is expensive to maintain and time intensive to perform maintenance
- FACS is a decreasingly viable product, and HHS cannot run the risk of it becoming technologically obsolete
- Disjointed entry with no logical workflow or support to guide targeted decision-making
- Reliance on memorization and repetition through a code-heavy structure
- Duplication of entry due to the inability to completely interface or communicate between systems
- Limited reporting capabilities
- Poor data quality or missing data
- Lack of support for mobile technologies
- Lag time in communication between systems and reporting

In 2014, JARVIS was developed to improve intake and assessment processes. JARVIS is a [redacted]. Due to a friendlier user interface, additional case management functionality was later introduced into JARVIS. This functionality includes case planning, safety and risk assessment, and documenting contacts and visits. Recent developments to JARVIS include a module to determine IV-E eligibility and a portal for contractors to document information and reports regarding the children and families they serve. The functionality outside of intake and assessment introduced into JARVIS will be reworked to align with the new functions and requirements of the CCWIS.

Despite the improvements from the JARVIS implementation, issues between the two systems persist. FACS does not easily communicate or work collaboratively with other systems. Several separate systems are in place to support functions that the FACS cannot do. This requires information already present in FACS to be entered again. Furthermore, there is a lag time in

communication between FACS and other child welfare applications due to batch processing, leading to waiting one – two business days for visible data.

Along with FACS and JARVIS, several other legacy systems support child welfare practice. The number of systems connecting to and from FACS and JARVIS creates a disjointed user experience and can often yield different answers to the same question or reporting measure.

Legacy systems include:

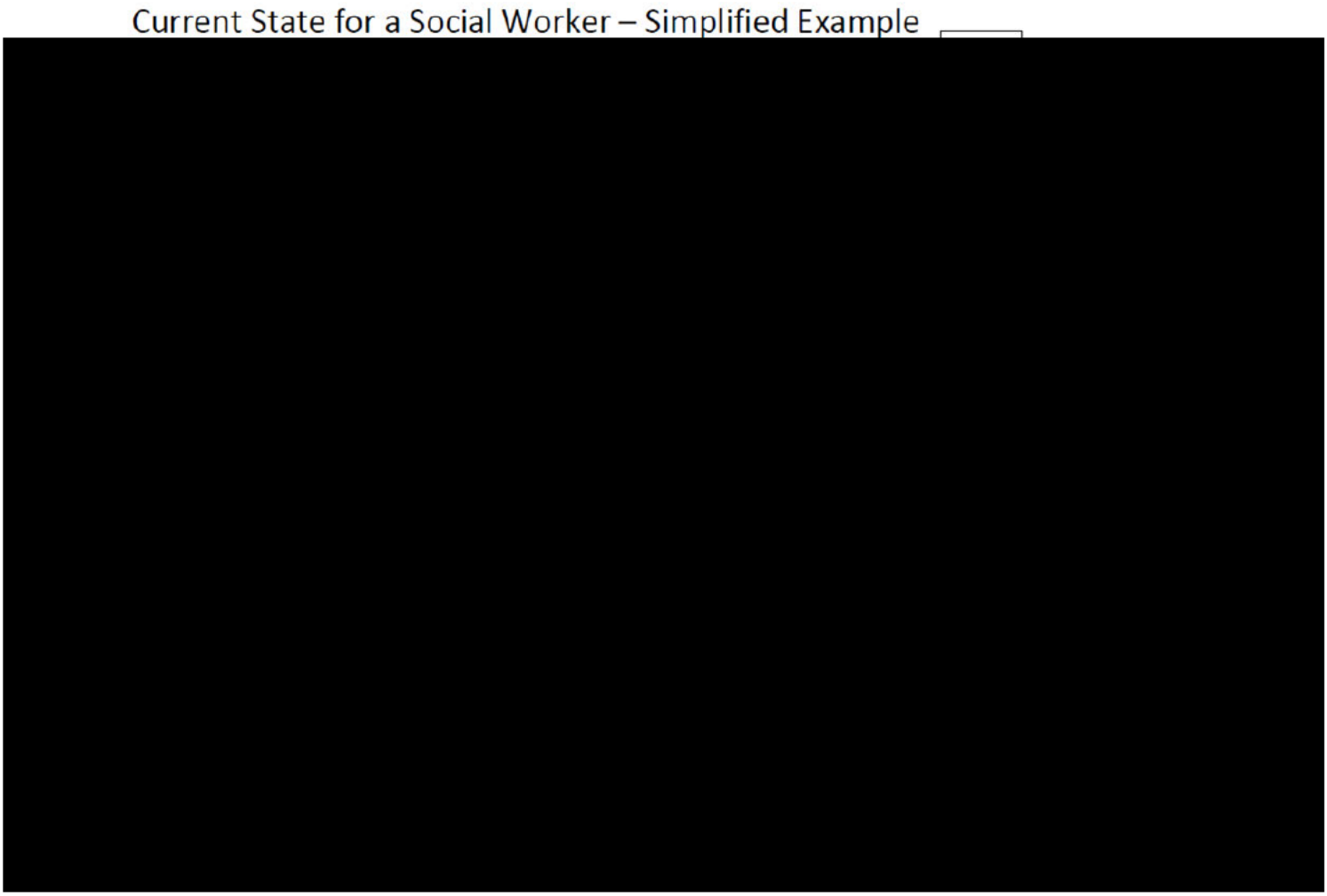
- ROM for child welfare reporting
- CareMatch
- Electronic Invoicing
- National Youth in Transition Database (NYTD)
- Prevention Plan Reporting

These legacy systems, while separate, do not provide extensive support for child welfare staff; they must be used together to complete tasks or answer questions.

3.2.1 Conceptual User Model

A worker must currently log in to multiple systems to retrieve or enter data, often manually duplicating it across systems. The data entry processes are intensive and have an impact on caseload and data quality for the entire organization. **Figure 3-2** depicts a simplified example of the current user process with the multiple data systems required to perform their roles.

Figure 3-2: Current State for a Social Worker



As HHS advances, the ideal state for workers is to access case work and child welfare information from a single application and to provide additional reporting analytics from a standard reporting service. All other required data and systems would be integrated with the application and reporting service via a common integration platform, providing the worker with a seamless, productive experience. Access would be available on desktop PCs, laptops, or mobile phones. This same conceptual model would be created for providers and customers of all HHS services. **Figure 3-3** on the following page depicts the single point of access envisioned for the target future-state user process.

Figure 3-3: Target State for a Social Worker



3.3 Objectives of the New System

The expectations of the child welfare system in Iowa have increased while staffing resources have decreased. Therefore, a significant way to improve the safety, stability, and well-being of children in Iowa is by becoming more efficient and effective through business process refinement and the use of new technologies. Through the analysis of current processes, completion of initial requirements gathering, and review of state and federal requirements, HHS has developed a list of primary expectations and goals as part of the DDI of the CCWIS.

The new CCWIS will achieve the following goals:

- **Intuitive Workflow**: The new system will offer a streamlined entry sequence with system prompts notifying the worker if information is missing or appears incorrect. The system will guide field staff throughout the life of a case, mirroring child welfare best practices and policies.

- **Ease of Data Entry**: The new system will eliminate the reliance on memorization and repetition by replacing the key-code-heavy structure. It will also provide clarity on confusing terms or policies, encouraging a more standardized entry of information and increased data quality.
- **Decision Support**: The new system will be built on business rules and alerts developed from child welfare policies and practices. These rules and alerts will help guide social workers through a case by providing workflow prompts.
- **Personalized Dashboard and Reporting**: The new system will provide users of the application with customized dashboards based on their work responsibilities and needs. Dashboards will help field staff view their caseloads, allowing them to see upcoming tasks and provide more consistent reporting across HHS. Leadership, policy, and other stakeholders will likewise have on-demand access to data and a variety of dashboards.
- **Document Management**: The new system will support the use of electronic forms and will allow the electronic storage of documents at both the case and individual levels. The system will also help with the collection of electronic signatures.
- **Eliminate Duplication**: The new system will reuse and auto-populate information that has been previously entered to populate in appropriate locations, documents, and forms. This will improve data quality by reducing the opportunity for data entry errors.
- **Clear Relationship Connections and Mapping**: The new system will provide field staff and analysts with a clear picture of a family system and various other relationships for each primary case member (professionals, informal family supports, etc.). The ability to view a family system will help workers understand a family's history with Health and Human Services (HHS), easily identify potential kin or fictive kinship supports and placement options, and accurately report on both individual and family (case level) outcomes.
- **Comprehensive Data Collection**: The new system will collect child welfare data more robustly through interfaces with both internal and external partners and a review of data reporting needs.
- **Real-Time Data Processing**: The new system will manage data in real time whenever possible to eliminate lag time in communication between systems and in reporting.
- **Mobility**: The new system will offer mobile functionality, allowing field staff to view and enter information, manage electronic forms, and obtain electronic signatures in the field.
- **Flexibility**: The new system will be flexible enough to support changes to child welfare practice and policy and any reporting requirements that may arise.

Along with the overarching goals above, the automated functions of the new CCWIS will follow a modular design, as required by 45 CFR 1355.53. These functions will interface with one another and run seamlessly. The application will include data exchanges and/or portal interfaces with third parties, including the Iowa courts, contractors, caregivers, Iowa Medicaid, and the Department of Education (DOE). HHS will review current reporting

applications to determine whether any may be absorbed into the new system and examine and revise processes as needed to ensure all reporting requirements are met. Any outstanding data quality issues (e.g., AFCARS) or performance improvement plans (PIP) will also be reviewed and addressed.

As previously noted, HHS has declared the STAR Intake and Assessment functions within JARVIS as a transitional CCWIS while continuing to explore the procurement of a system that meets program needs and complies with the CCWIS requirements. HHS will still need to support the legacy FACS as a non-CCWIS until a new system is available to replace it.

3.4 Anticipated Benefits

HHS believes that a CCWIS that successfully meets the needs and objectives stated above will have the following positive benefits:

- **Enhance Service to Children and Families**: A CCWIS that meets HHS's stated needs and objectives will significantly strengthen the agency's ability to support Iowa's children and families. By modernizing technology and streamlining casework processes, the new system will enhance service delivery, work to ensure more timely and accurate documentation, and promote consistent, high-quality practice across Iowa. Staff will benefit from increased efficiency through reduced duplication of work, intuitive workflows, and more precise guidance, enabling them to spend more time engaging with families rather than managing outdated systems.
- **Increase Efficiency**: As expectations on the child welfare system increase while staffing decreases, a new system helps workers do more with less by streamlining workflows, reducing duplication of effort by capturing information once, and eliminating the need for memorization and repetitive data entry. It replaces these inefficiencies with guided, intuitive data entry that enhances staff productivity.
- **Reduce Risk**: The new CCWIS will reduce organizational security and technical risk by replacing aging systems with a reliable, sustainable, and secure platform that improves data quality by standardizing how information is captured and used. With more complete, timely, and accurate data, HHS will be better equipped to monitor outcomes, make informed decisions, and support the safety, stability, and well-being of Iowa's families.
- **Increase Data Quality**: The new CCWIS could enhance data quality by standardizing data entry, enabling real-time processing, and eliminating duplicate entries, working to ensure that information is accurate, complete, and available. It would also provide robust reporting functionality, allowing users to access timely, actionable information. Improved data visibility supports data-driven decision-making and strengthens Iowa's ability to meet federal reporting requirements.
- **Increase Timeliness of Data Entry**: The current FACS system requires extensive manual workarounds, duplicate entry, and reliance on memorization to navigate a heavy code structure. These inefficiencies are driven by outdated technology and the system's inability to communicate fully with other systems. A new CCWIS will eliminate redundant processes, streamline data entry, and support logical, intuitive workflows, significantly

reducing staff workload. The new system would also be designed to evolve as child welfare policies and federal regulations are updated.

- **Standardization of Practice**: FACS currently lacks a logical workflow and provides no systematic guidance to caseworkers, resulting in inconsistent data entry and decision-making. The new CCWIS could incorporate guided workflows and decision-support capabilities that align with child welfare policies and best practices. Current system shortcomings—including missing data, incorrect information, and delayed communication between applications—limit Iowa's ability to rely on its data for case management and reporting.
- **Cost-Effective Tools**: The legacy FACS platform has become increasingly expensive and time intensive to maintain. With the product nearing technological obsolescence, Iowa faces growing operational risk. A modern CCWIS mitigates this risk by replacing aging architecture with a stable, sustainable platform, reducing long-term costs associated with maintenance and the custom fixes required to support outdated technology.

3.5 Project Stakeholders and Personas

HHS developed a list of primary stakeholders that will benefit from the new CCWIS. These include:

- **Children and Families of Iowa served through HHS**: Each child and family will have a complete and accurate case history and the necessary details for caseworkers and the courts to make informed and timely decisions that will improve safety, permanence, and well-being outcomes.
- **Placement and Service Providers:** Providers will have immediate access to child-specific information, including health, mental health, education, and other relevant information, when placement occurs, as well as submit invoices and receive timely payments.
- **Child Welfare Service Workers, Leadership, and the Field Support Units:** Our child welfare teams will be able to review case-level data to improve efficiency, streamline workflow, and achieve positive outcomes for children and families.
- **HHS Leadership:** Leaders throughout HHS will have accurate and reliable information to make data-driven decisions regarding child welfare policy, practice, and service delivery.
- **Data Analysts within HHS:** The HHS reporting analyst will have timely and accurate data to use for federal and state reporting requirements, including IV-E and IV-B reimbursement, AFCARS, NCANDS, NYTD, and Prevention Plan reporting.
- **Policy Makers:** HHS will proactively collaborate and educate policy makers using the child welfare data available at their fingertips to inform the legislative process and analyze child welfare reporting for the children and families of Iowa.
- **Community Partners:** HHS community partners can utilize aggregate child welfare data to organize local efforts around meeting the needs of children and families and grow natural supports for families in need.

After identifying the stakeholders, the team also created personas for the system's users and prioritized them based on the greatest impact. A persona is a written description that defines a user role and the ways the user would interact with the system. This was done to help ensure the system was completed with each user's experience in mind, identify and assess each persona's touchpoints, and better understand each persona's path through each module. **Figure 3-4** includes a diagram of the personas identified.

Figure 3-4: Persona Diagram



4.0 Requirements Analysis

HHS has invested significant time and resources in understanding the current system's challenges and learning child welfare best practices and the processes that should occur. This process of gathering relevant information has also led to the development of functional and non-functional requirements that reflect the needs and objectives identified by HHS and its stakeholders.

4.1 Key Requirements, Considerations, and Methodology

HHS utilized the established Service Business Team (SBT), comprised of child welfare leadership and senior IT staff, to initially oversee the requirements gathering exercise and the CCWIS project implementation. The SBT began the requirement gathering exercise through various events. A Lean Design session for case managers was held to carefully document technological challenges and to make recommendations on how the new application could address them. The Lean event provided a foundation for subsequent internal information-gathering sessions, including meetings with other states, underscoring the importance of clear, well-defined requirements.

Through business process design sessions, the SBT successfully defined and validated core requirements while concurrently laying the groundwork for the data platform, transformation, modeling, and the analysis of downstream impacts. This coordinated approach created a methodology that aligned HHS with technical execution, helping to ensure the system would be built on a stable foundation.

Next, the project team was identified to assess and evaluate the strengths of current processes and to determine the gaps and challenges they present. Interviews and brainstorming sessions with key stakeholders and users were held to document initial, high-level requirements to mitigate the identified gaps and challenges. The requirements developed were used to create a Lean Business Case for the overall CCWIS and for each module. The exercise of creating these documents illustrated an initial split between functional and non-functional requirements, helped HHS understand the Minimum Viable Product (MVP), and developed an initial prioritization of requirements. The documents were reviewed and approved by the SBT.

The final stage of the initial requirements elicitation process consisted of working sessions facilitated by key project stakeholders to refine, develop, and validate requirements. These requirements were incorporated into the Business Requirements Document (BRD) and the Functional Requirements Specification (FRS).

4.2 Highlights of Functional Requirements by Module

The creation of the BRD and FRS, the documents were routed to relevant child welfare SMEs and stakeholders for feedback on accuracy and to identify any gaps in the finalized documents. Each module is detailed below, with a high-level description, followed by a table showing the functional requirements. The modules are organized to emulate a linear end-to-end process that a case follows, with supporting modules that help drive the overall case process.

Every module will follow federal regulation 45 CFR1355.52(b)[1] regarding the overarching collection of data described below:

- Administration of programs, including what is needed for federal reports, audits, reviews, and monitoring; and eligibility determinations, authorizations of services, and expenditures under IV-B and IV-E
- Data to support state child welfare laws, regulations, policies, practices, reporting requirements, audits, program evaluations, and reviews
- Data needed to comply with ICWA
- Data needed for AFCARS reporting
- Data needed for Prevention Plan reporting

The modules will also comply with the data quality requirements set forth in 45 CFR 1355.52(d), meeting the completeness, timeliness, and accuracy standards. Any data collected will be exchanged and maintained in accordance with confidentiality requirements. The modules will be designed, to the extent possible, to help prevent the need to reenter captured data and to ensure no data is created by default or inappropriately. Any data exchanges will be defined through a data sharing agreement (DSA)/memorandum of understanding (MOU), which establishes a single data exchange standard for data definitions, formats, and other specifications.

The following are the main modules needed to meet the needs of HHS and its users:

Vision & Strategy Management Module

High-Level Description: The Vision & Strategy Management Module will help drive and assess the Department's vision and goals. It will focus on data and reporting needs for budgeting, data quality auditing, and federal or state reporting. The module will pull from data that has been entered or updated in the other modules, resulting in a direct or indirect impact on all personas.

Product & Services Development Module

High-Level Description: The Product & Services Module will focus primarily on the configuration and system administration of the CCWIS. It will aid in the development and delivery of training and support for the system, including a knowledge base of policy guidance for best practices and Help Desk functionality. It will manage user groups and roles and allow users to manage their own profiles and preferences. All internal persona groups will use this module.

[1] 45 C.F.R. 1355.52(b) (2025). https://www.ecfr.gov/current/title-45/subtitle-B/chapter-XIII/subchapter-G/part-1355

Provider Maintenance Management Module

High-Level Description: The Provider Maintenance Module will manage the contracting, licensing, and regulatory processes for service and placement providers, ensuring that information for foster and adoptive homes, facilities, and contractor agencies providing services is appropriately maintained. Licensing workers and contract specialists can manage their caseload of providers within the module. It will also serve as the entry point for the provider portal, where contractors can enter and update information and submit documents. This module will be primarily used by the following personas: Contract Specialists, Specialized Staff (Licensing Workers), and Program Managers.

STAR Intake & Assessment

High-Level Description: The STAR module, which comprises child abuse intake and assessment functions, currently resides in JARVIS and will remain in place until replaced in the new CCWIS. The new CCWIS will populate data entered in STAR to the appropriate locations in the new system. Any updates to the current STAR module will be considered service or maintenance requests. The following personas will primarily use this module: Social Workers, Supervisors, Specialized Staff, Service Area Leadership, and Management.

Case Management Module

High-Level Description: The Case Management Module will support processes for ongoing case functions, including case planning, opening and monitoring services and placements, managing court activities, and case closure. The following personas will primarily use this module: Social Workers, Supervisors, Specialized Staff, Service Area Leaders, and Support Staff.

Financial Management Module

High-Level Description: The Financial Management Module will encompass all functions that help ensure payments are processed efficiently and accurately, and eligibility funding streams are determined and set. It will serve as the location for the automated IV-E determination and will establish the funding hierarchy based on the best fit for the child or family. The following personas will primarily use this module: IV-E Staff, Social Workers, Supervisors, and Service Area Leaders.

Data Distribution and Exchange Module

High-Level Description: The data distribution and exchange module will manage the system interfaces and deliver and exchange information with the portals. Using this module, HHS partners can submit information through a secure portal, which will provide case managers with the information they need to serve children and families effectively. The information entered into the portal will be updated in the electronic case file through a data exchange between modules. The following personas will primarily use this module: Specialized Staff, External Partner Agencies, Foster/Adoptive Parents, Management, and Children and Families.

The modules were later expanded, and the following business processes were identified:

- Manually Initiated Case Pathway

- JARVIS Initiated Pathway
- VISION Case Setup (CA/Children in Need of Assistance (CINA) Pathway}
- Manual Case Setup (Direct File/Interstate Compact on the Placement of Children (ICPC) /Extended Foster Care/Payment Only Pathways)
- VISION Case Setup (CA/CINA Pathway) and Manual Case Setup (Direct File/ICPC /Extended Foster Care/Payment Only Pathways) business process diagrams
- Court Case
- Living Arrangements
- Services
- Associations
- Adoption Transfer Workflow
- Adoption Workflow
- Post Adoption Workflow
- Subsidized Guardianship Workflow
- Subsidy Payment Management

4.3 Highlights of Non-Functional Requirements

Non-functional requirements describe how a system should perform rather than what the system should do. These requirements often define quality attributes, performance expectations, and operational constraints, and are typically detailed in the system architecture.

For the CCWIS, the non-functional requirements will address:

- **Accessibility:** Guidelines that help ensure products are accessible and usable by people with disabilities.
- **Configurability:** The ability to adjust the system to user needs or operational requirements without significant changes to underlying code.
- **Data Management:** The process of collecting, storing, organizing, and protecting data.
- **Interoperability:** The ability of the system to exchange information and operate effectively with other systems and applications.
- **Performance:** The ability of the system to operate efficiently, measured by system speed, responsiveness, and stability.
- **Security:** The practices used to protect systems and data from threats.

4.3.1 Interface/Data Exchange Requirements

During requirements gathering and information sharing, HHS identified current and potential interfaces to the new CCWIS. The current interfaces that will potentially also occur in the new

system will be listed in both tables. Also identified for each system is the type of interface, which includes inquiry services, batch processes, or manual processes. Inquiry services are real-time interfaces, whereas batch processes involve exchanging a file and then feeding it into the system. Manual processes are those in which data from the system is entered into the database through manual look-up and entry, or data is exchanged between systems manually.

Each interface planned for the new system will include an Interface Agreement Specification detailing the data to be exchanged and the manner in which it will be exchanged. The external interfaces will also include an MOU to solidify the agreement between HHS and partnering agencies.

Table 4-1 below displays the current FACS and/or JARVIS interfaces that occur:

Table 4-1: Current Internal and External Data Interfaces

System	Description	Internal/ External	Type of Interface
AFCARS	Collects case-level information from state and tribal Title IV-E agencies on all children in foster care and those who have been adopted with Title IV-E agency involvement	External	Batch
Central Print	Central printing and mailing system	External	Batch
Children's Justice	Weekly data file sent to the Courts	External	Batch
CJIS	System that uploads and downloads information to and from the Courts	External	Batch
Data Warehouse	Data analytics and reporting	Internal	Batch
Drug Testing	System automating the drug testing authorization process	External	Inquiry
Early ACCESS	Education and health information for children under the age of three who have a condition or disability that could cause growth or developmental delays	External	Inquiry
Electronic Invoices	Application where providers can submit invoices	Internal	Batch
EPS	Bridge system between HHS systems and Iowa Advantage.	Internal	Batch
FCRU	Exchanges financial data for Social Security (SSA) or Supplemental Security Income (SSI) benefits	External	Batch
FFPSA Prevention Plan Data Reporting	Reporting elements required for Prevention Plan data reporting.	External	Batch
Foster Care Youth Credit	Youth credit report information	Internal	Batch and Manual
Global 360	Electronic document storage	Internal	Inquiry

System	Description	Internal/ External	Type of Interface
Iowa Advantage	State's financial system	External	Inquiry
Iowa Automated Benefits Calculation (ABC)	Establishes benefit eligibility and calculates benefit levels for Supplemental Nutrition Assistance Program (SNAP), Temporary Assistance for Needy Families (TANF), and Medicaid	Internal	Inquiry and Batch
Iowa Collections and Reporting (ICAR)	Establishes and enforces child and medical support orders, and processes support payments	Internal	Batch
IDOE	Education related information	External	Batch
Iowa Medicaid	Portal application which provides several services	Internal	Inquiry
JARVIS – DARES Modules	Dependent adult abuse evaluations	Internal	Inquiry
JARVIS – STAR Modules	Intake and assessment of child protective assessments including child abuse assessments, family assessments and CINA assessments. Child abuse registry.	Internal	Inquiry
JARVIS – Child Services Modules	Case Plans, Contact Notes, Relative Notices, Prevention Plans, Safe Plans of Care, etc.	Internal	Inquiry
JARVIS – IVE Eligibility Module	Title IV-E program eligibility determination.	Internal	Inquiry
NCANDS	A data collection system that gathers information about reports of child abuse and neglect	External	Batch
NYTD	Tracks which independent living services states provide and assesses the collective outcomes of youth	Internal	Batch
Okta	Statewide identity access management (IAM) system	External	Inquiry
Open Text	Electronic document storage	Internal	Inquiry
ROM	Results-Oriented Management reporting tool	External	Batch
SSA	Foster care updates for SSA file.	External	Batch

Table 4-2 displays the potential interfaces that will be implemented with the new CCWIS.

Table 4-2: Potential Internal and External Data Interfaces

System	Description	Internal/ External	Type of Interface
ARTS	System operated by the DOT used for driver's license scoping	External	Inquiry

System	Description	Internal/ External	Type of Interface
CareMatch	System maintained by FivePoints Technology Group used predominantly by foster/adoption parent recruitment, retention, and child matching provider.	External	Inquiry
CLM	Contracting system for Iowa HHS.	Internal	Inquiry
Eligibility Integrated Application Solution (ELIAS)	Establishes eligibility and calculates benefit levels for Modified Adjusted Gross Income (MAGI) Medical	Internal	Inquiry
Google Maps	Performs geographical lookups both for addresses and mapping.	External	Inquiry
Income Eligibility Verification System (IEVS)	Requests, receives, processes and stores information from various data sources (e.g. Internal Revenue Service (IRS), Social Security Administration (SSA), National Directory of New Hires (NDNH), Iowa Department of Public Health (IDPH), etc.)	Internal	Inquiry
Iowa Courts Online	Criminal history used for system checks during intake	External	Inquiry
Public Health	Immunization and vital statistics related information	Internal	Inquiry
ISIS/IoWANS	Waiver service information	Internal	Inquiry
Iowa Workforce Development	State agency providing employment services	External	Inquiry
KinderTrack	System to manage and regulate childcare	Internal	Inquiry
NEICE Clearinghouse	System to create and process ICPC cases	External	Inquiry
SendGrid	Provides email and calendar-based functionality.	Internal	Inquiry
Sex Offender Registry	Access to sex offender registry for background check information	External	Inquiry
WISE	Stores and retrieves data related to the management of Income Maintenance and financial eligibility programs	Internal	Inquiry

4.4 RTM Development

In January 2025, HHS contracted with the consulting firm, BerryDunn, to provide HHS with a reasonable path to success that would incorporate the ACF CCWIS requirements, leverage knowledge of today's technology, and advance HHS's child welfare practice forward. BerryDunn reviewed the inventory of HHS's systems in use, assessed the functionality leveraged within them, and identified the strengths and challenges HHS faces in the current environment. The

BerryDunn team reviewed business processes through background document review, including the FRS and discovery sessions with SMEs, to guide the additional deliverables required for the procurement of a CCWIS as described in **Section 7.3**.

Based on the information gathered and assessed, as well as the known CCWIS federal requirements, BerryDunn identified additional requirements informed by challenges faced by HHS users, strengths of the current technology, lessons learned from peer states, and best practices in child welfare. An iterative process was used over several weeks to repurpose the previous requirements development work and engage functional and technical HHS SMEs for feedback to identify any gaps in the business processes. These activities resulted in the creation of the RTM for the RFP packet and the selected vendor. This collaborative effort provided HHS with a detailed RTM to support the chosen vendor in developing and implementing the new CCWIS.

The requirements include a tracking identification and were divided into categories, which include:

- **Adoption:** Requirements that focus on adoption-related documents, procedures, and the sequence of steps necessary to move from the start of the adoption process through its completion.
- **Case Management:** Requirements that address the functions, standards, and capabilities needed to support the oversight of a case from initiation through closure.
- **CCWIS Guidelines:** Refers to the federal regulations and standards established by the ACF for CCWIS. These guidelines outline the functional requirements, data sharing protocols, and design principles states must follow to develop child welfare systems that support case management, improve data quality, and enhance service delivery.
- **Court:** Requirements that contain records related to court proceedings and documenting the legal journey, helping to ensure accurate tracking and compliance with judicial mandates. These requirements include court dates, orders, legal documents, and outcomes.
- **Data Management:** Requirements that involve processes such as data integration, quality control (QC), governance, and life cycle management within the context of specific applications.
- **Education:** Requirements that include educational history, educational needs, and school registration information to aid in ensuring educational needs are addressed.
- **Fiscal Management:** Requirements that address billing and payment information. These requirements help establish standards for budgeting, accounting, and financial reporting.
- **Health:** Requirements that focus on health records, including medical history, immunizations, and mental health services.
- **ICPC:** Requirements that define the management of interstate placements under the ICPC agreement. Includes documentation, status tracking, and compliance checks.

- **ICWA:** Requirements that focus on Native American children, including tribal affiliations, notifications, and court compliance.
- **Income:** Requirements that capture income sources and employment.
- **Integrations:** Requirements that focus on the process of connecting different IT systems, applications, or components to work together as a coordinated whole to exchange data.
- **Person Management:** Requirements that capture relationships and connections between a person record and other people, providers, and system elements.
- **Planning:** Requirements that define the goals, action steps, and strategies for case resolution. These requirements include permanency plans and service goals.
- **Reporting:** Requirements that define reports and/or dashboards will be available within a specific module.
- **Risk and Safety Assessment:** Reflects the structured evaluations used to determine risks, strengths, and service needs. It includes activities related to safety determination, such as risk assessments and safety plans.
- **Security:** Requirements that focus on protecting applications from threats by identifying and mitigating vulnerabilities in code, configurations, and third-party components.
- **System Management:** Requirements that address the administration and oversight of the organization's IT systems, including hardware, software, networks, and related infrastructure.
- **Tasks:** Requirements that include alerts, notifications, tasks, and calendar events to support system processes and workflows.

5.0 Alternatives Analysis

5.1 Analysis Criteria

HHS conducted an extensive review of options for replacing its outdated child welfare systems, FACS and JARVIS, to meet federal CCWIS requirements and modernize service delivery. This process began in fall 2017, when HHS engaged in discussions with ACF, consulted with the Casey Foundation, and held calls with human services staff in other states to understand best practices and considerations for system replacement.

To further inform decision-making, HHS issued an RFI to identify available products and approaches. Insights from the RFI led to the creation of the VISION project in January 2021, which pursued a custom-developed solution supported by internal and contracted staff.

However, by June 2024, the VISION project was paused due to delayed production releases, escalating costs, and the anticipated burden of maintaining a custom-built system. During this period, the DOM DoIT reorganized and adopted a policy to discontinue support for large scale internal custom builds, reinforcing the need to pivot toward vendor-supported solutions.

The following lessons learned continue to be applied as HHS moves forward:

- HHS and DOM DoIT technical staff are better suited to implement and maintain a solution that is developed, supported, and hosted by a third-party vendor.
- Strong executive-level support is required for a successful project.
- Communication is critical in all aspects and phases of the project.
- SMEs from the field, policy, and IT must be fully available to the project and involved in all phases.
- Managing expectations is important throughout the life of the project.
- It is critical for HHS to define system functionality for its future CCWIS system clearly.
- It will be critical to identify quick wins during the early stages of the project.
- Data cleanup and conversion activities should start as soon as possible.
- Desired interfaces, both within HHS and with other external agencies, should be identified as soon as possible, followed quickly by Data Sharing Agreement discussions.

While gathering requirements, **a set of guiding principles was also identified** that will significantly impact the selected solution:

- HHS must have a CCWIS-certified system once the project is complete.
- The final solution must promote systems extensibility via well-formed, documented programmatic interfaces that enable systems integration and data sharing with HHS, Iowa, and third-party technology partner software applications.

- The final solution will utilize development technologies that are in line with current HHS technologies to leverage the enterprise technology capabilities, such as IAM, document management, and publicly exposed APIs.
- The initial case management solution (MVP) must be delivered and made available for staff use by the end of 2026, with any remaining FACS and JARVIS components incorporated shortly thereafter, and allowing for the decommission of both.
- The solution must maximize meeting HHS's specific policy and practice needs and minimize the need to compromise or potentially develop system workarounds.
- The solution must be a fiscally reasonable and fiscally responsible choice.

The purpose of the alternatives analysis is to compare development options for replacing the current HHS child welfare systems or maintaining the status quo.

5.2 Alternatives Considered

5.2.2.1 Alternative Comparisons

The following alternative types were assessed using the required streamlined approach recommended by ACF:

- **New Development Options:**
    - *Full Internal Build*: Building the entire system internally was eliminated following DOM DoIT's policy change and the resource constraints highlighted by the VISION project.
    - *Hybrid Internal Build with Commercial-off-the-Shelf (COTS) Modules*: Combining internal development with specialized COTS modules, which conflicted with DOM DoIT's policy against large scale internal builds.
- **Accelerator Solutions (COTS):**
    - *(COTS) Vendor-Hosted, Managed, and Supported Solution*: A vendor-managed and hosted system licensed to HHS, offering vendor expertise but introducing cost and dependency risks.
    - *(COTS) with IT Participation*: Purchasing a COTS product with rights for HHS to customize and maintain, providing flexibility but requiring significant resources that would need to be procured to a great extent externally.
- **Status Quo:** Retaining FACS and JARVIS was deemed infeasible due to their outdated architecture, difficulty in maintenance, and inability to meet CCWIS modularity requirements.

Ultimately, internal build options were ruled out due to organizational capacity and policy constraints, leaving the accelerator solution and vendor-hosted/COTS-based solutions as the most viable paths forward to best support compliance, extensibility, and rapid deployment, while aligning with HHS's modernization goals and lessons learned from prior efforts.

In early 2025, Iowa partnered with BerryDunn to develop detailed requirements to procure a vendor-hosted/COTS solution. BerryDunn helped identify and refine CCWIS requirements using federal standards, user feedback, technology strengths, and best practices. Through an iterative process involving subject matter experts (SMEs), HHS developed a detailed RTM. This RTM was initially intended to guide the selected vendor's CCWIS development and implementation process through the RFP process. However, HHS instead used the RTM and other RFP documents to further enrich Google's understanding of its CCWIS requirements and system environments, allowing Google to better plan an approach to replace FACS and develop the proposed contract.

Table 5-1 outlines the pros and cons identified during the alternatives analysis for HHS's CCWIS transition. This comparison aims to provide a clear view of the potential benefits and challenges of each option, as summarized in **Section 2.3**. The alternatives compared supported informed decision-making and alignment with federal requirements and agency goals.

Table 5-1: Alternative Pros and Cons

Alternative	Pros	Cons
New Development: Full Internal Build	• Offers the most flexibility to build a fully customized application specific to Iowa's needs • Built entirely by internal staff, so it is more responsive to department needs and priorities than an external vendor • Builds a high level of business knowledge into the team to support the application, resulting in better long-term support of the application • No ongoing license fees from a COTS product	• Lack of dynamic reporting and graphing seen in vendor solutions • By starting from a blank slate, it can be overwhelming for project participants to define the scope and functionality needed clearly • It may be more challenging to dedicate internal staff to development and maintenance than external staff • Need to invest development time to build configurability into the system • This option has the longest development time, and costs can become unwieldy • Without dedicated support to treat this as a living or ongoing product that needs enhancement, this option can shorten the lifespan of the application • Additional development time required to create and manage technical capabilities that may be part of a COTS service • DOM DoIT does not support large scale, internal, custom coded system builds
New Development: Hybrid Internal	• Offers the most flexibility to build a fully customized application specific to Iowa's needs	• Additional software to license each year for the COTS product

Alternative	Pros	Cons
Build with COTS Modules	• HHS could implement COTS modules, such as mobility, while the ongoing core product is being built • The core system is built by internal IT staff who are more responsive to department needs and priorities • Builds a high level of business knowledge into the team to support the application, resulting in better long-term support of the application • Allows internal IT staff to focus on core system technologies and purchase specialized knowledge for other areas of the system • It is easier to replace COTS vendors for specific supporting modules than for a fully purchased solution • Potentially faster development time than a full internal build of all modules	• Purchasing modules from multiple vendors can be more complex to manage • Increased complexities in reconciling business rules throughout the application, depending on the number of purchased modules • Potential for module compatibility issues • DOM DoIT does not support large scale, internal, custom coded system builds
Accelerator Solutions (COTS): Vendor Hosted, Managed, and Supported Solution	• Potentially one of the quickest solutions to configure and implement • The vendor-provided software platform provides a significant percentage of the CCWIS functionality (greater than 70%), reducing the need for additional software components • Minimizes custom coding requirements while providing more flexibility than a fully developed COTS system • The platform is configuration based allowing for faster change management and requiring fewer technical resources for making configuration changes • The platform provides industry standard microservices or comparable technology options for systems integration and data sharing • Operational support, platform maintenance, and future CCWIS requirements provided by vendor	• There may be limitations to implementing Iowa-specific functionality with the preconfigured solution • The addition of technical capabilities may require vendor involvement and additional cost • A systems integrator may be needed to establish communication between the CCWIS solution, existing HHS applications, and external third-party software components

Alternative	Pros	Cons
Accelerator Solutions (COTS): COTS Product with IT Participation	• HHS could potentially receive vendor updates to the product to keep it up to date • Potentially the quickest solution to build and implement	• Least configurable option with the least amount of staff input into the final product • Least flexible solution to customize to meet state needs • Vendor updates can negatively impact the functionality of customizations and can lead to additional costs for regression testing and implementation of the customizations • Large upfront costs with ongoing license fees each year • The RFP process would be time intensive to be as specific as possible to get a product that meets current needs • It could be difficult to find a COTS solution that allows HHS ownership of the final source code for completing ongoing customizations and enhancements

5.3 Analysis

5.3.1 Top-Ranked Development Options

The following two options were considered as potential viable alternatives for the solution:

1. **Vendor-Hosted, Managed, and Supported Solution:** HHS-owned solutions hosted and managed by the vendor but licensed by HHS to be used. Vendors will customize and configure the application, but it can be expensive, and HHS would be constrained by the vendor's responsiveness and ability to complete changes or enhancements.
2. **Purchase a Commercial-Off-the-Shelf (COTS) Product with IT Participation:** Purchase a COTS product, including the rights to update and modify the code by state or contracted IT staff. Vendor staff will complete most of the customizations and configurations. HHS staff will also assume the rights and the ability to complete all ongoing and future customizations and configurations, with less need for vendor development staff.

5.3.2 Low Scoring Development Options

The following three options were not strongly considered due to the reasons provided below:

1. **Status Quo:** No changes are made to FACS or JARVIS, and the systems remain as-is. This is not a viable option due to the FACS's outdated and inefficient nature. It is difficult

to maintain and almost impossible to update with changes to state and federal regulations. Also, the only way either system could be part of a CCWIS-certified solution would be for them to be exempted from modularity requirements.

2. **Full Internal Build:** The entire application is built internally by state or contracted IT staff using current technologies of .NET and SQL Server. No modules would be purchased from outside vendors, except for software and middleware products used for development. Effective July 2024, DOM DoIT no longer supports large scale, internal, custom coded system builds.
3. **Hybrid Internal Build with COTS Modules:** Internal state or contracted IT staff will build the core system, and COTS modules will be purchased for specialized functionality, such as mobile solutions and placement matching. Effective July 2024, DOM DoIT no longer supports large scale, internal, custom coded system builds.

5.3.3 Cost Benefit Analysis

The HHS CBA outlines and compares the costs and benefits of three different options. First, the cost of maintaining the current child welfare systems as the status quo was identified. The two top-ranked options from the alternatives analysis were then compared. All CBA methodologies are consistent with the ACF Guidelines for CBA, and the tools were configured to meet the specific challenges faced by HHS.

The CBA was completed based on the work performed in Iowa to date, including building system requirements as outlined in the IAPD. The processes and expected benefits of the new system, as detailed in the IAPD, were used to evaluate alternatives and to build the CBA.

A thorough CBA revealed that the new CCWIS could yield substantial gains in staff productivity across almost every level of HHS. FACS may be a functional system, but its outdated nature requires significant human effort to maintain data and implement needed enhancements to maintain functionality or meet policy standards.

Figure 5-1 compares the total cost of ownership over the eight-year timeline for the status quo and the two viable options from the alternatives analysis.

Table5-2: Cost Comparisons of Status Quo and Alternatives

Year	Cost – Status Quo	Cost – COTS Product w/ IT Participation	Cost - Vendor-Hosted, Managed, and Supported
2026	$6,321,678	$85,974,823	$45,786,250
2027	$10,114,685	$98,618,179	$67,786,250
2028	$12,643,356	$101,146,851	$83,786,250
2029	$15,804,195	$107,468,529	$88,786,250
2030	$18,965,034	$112,525,871	$93,786,250
2031	$22,125,874	$115,054,543	$98,786,250

Year	Cost – Status Quo	Cost – COTS Product w/ IT Participation	Cost - Vendor-Hosted, Managed, and Supported
2032	$25,416,181	$120,358,652	$103,786,250
2033	$27,824,008	$131,760,949	$108,786,250

5.4 Identified Benefits

Based on Iowa's findings, the following benefits were identified as potentially providing tangible dollar benefits to HHS:

1. **Workflow Automation and Policy Compliance**: With intuitive workflows, the system will guide users through each stage of a case, using prompts and alerts to help ensure compliance with child welfare policies. Automated processes will reduce manual errors and eliminate redundant tasks, lowering operational costs and improving efficiency.
2. **Enhanced Usability and Productivity**: The CCWIS will replace the code-heavy structure of legacy systems with ease of data entry features, including clear terminology and standardized forms. This will minimize the learning curve, accelerate onboarding, and reduce training costs, enabling staff to focus on service delivery rather than navigating the system.
3. **Advanced Search and Matching Capabilities**: Through relationship mapping and robust data collection, workers will have a comprehensive view of family systems and connections. Enhanced search and matching algorithms will streamline eligibility determinations and case planning, reducing time spent on manual checks.
4. **Mobile Accessibility**: Field staff having access to secure, real-time case information, electronic forms, and signature capabilities will enable them to complete tasks in the field and respond quickly to emerging needs.
5. **Robust Reporting and Analytics**: Leadership and administrators will have access to performance metrics and compliance data. This supports data-driven decision-making and resource optimization.
6. **Workforce Management Dashboards**: Customized dashboards will help balance workloads and track performance. These dashboards will provide real-time visibility into caseloads, upcoming tasks, and performance metrics, enabling supervisors to balance workloads effectively and monitor progress.
7. **Electronic Case File and Paperless Processes**: Document management capabilities will support electronic forms and signatures, eliminating paper-based processes. This reduces storage costs, speeds document retrieval, and enhances security.
8. **Integrated Collaboration and Data Access**: The CCWIS will eliminate duplication by auto-populating previously entered information and integrating with internal and external partners such as Iowa Courts, Medicaid, and DOE. This interoperability ensures a unified platform for collaboration and reporting.

In addition, several qualitative benefits were identified. These benefits cannot be assigned a dollar value, but they are key indicators of a system that successfully supports business processes. Based on Iowa's findings, these qualitative benefits are:

1. **Improved Outcomes for Children and Families**: By enabling timely interventions and comprehensive case management, the system will help ensure that accurate, real-time data inform critical decisions. This leads to better safety, stability, and permanency outcomes for children and families across Iowa.
2. **Data Quality and Integrity**: Real-time processing and standardized data entry will significantly reduce errors and inconsistencies. High-quality data supports compliance with federal and state reporting requirements and strengthens the ability to monitor performance and identify trends.
3. **Employee Satisfaction and Retention**: Intuitive tools and streamlined workflows will reduce frustration and eliminate unnecessary administrative burdens. By simplifying complex processes, the system empowers staff to focus on meaningful work, improving morale and reducing turnover.
4. **Statewide Standardization of Practice**: Consistent workflows and uniform data entry standards will help to ensure equitable service delivery across all regions. This standardization promotes fairness, transparency, and accountability in child welfare practices statewide.
5. **System Flexibility and Future-Readiness**: Built on a modular, adaptable architecture, the CCWIS will easily accommodate policy changes, new reporting requirements, and emerging technologies. This flexibility ensures the system remains relevant and sustainable for years to come.

5.5 Evaluating Benefits for Each Option

The quantitative benefits mentioned above, and the guiding principles listed under Section 5.1, were collectively used to calculate the total cumulative benefits for each option analyzed. Below is a comparison between the three options.

5.5.1 Cost Benefit Analysis per Option

Option 1: Vendor-Hosted, Managed, and Supported Solution

HHS will work with a third-party vendor to develop, host, and support a CCWIS software product that includes most of the required functionality and to supplement the overall solution with additional technical capabilities to achieve the desired technology enablement. The vendor will also be responsible for serving as the system integrator, ensuring appropriate communication between the new CCWIS system and the HHS legacy systems. This option will promote systems interoperability and efficiency with programmatic APIs. Vendor resources will be used for all development, configuration, and customization efforts. New or revised business processes, data elements, reports, and analyses will be configured by vendors without significant disruption to the existing operational application.

This alternative is highly desirable to DOM DoIT, given it requires a minimal level of support by DOM DoIT staff. It is expected to replace the majority of FACS within one year, and replace the remainder of FACS and all JARVIS functionality within the following year and a half. This means staff will complete all their work in a single, integrated CCWIS within 2.5 years. This alternative will be tailored to meet HHS's specific policy and practice needs, eliminating any need to compromise or potentially develop system workarounds. This alternative is ultimately less expensive than a COTS Product with an IT Participation solution.

Table 5-3: Vendor-Hosted, Managed, and Supported Solution

Year	Cost – Vendor-Hosted, Managed, and Supported
2026	$45,786,250
2027	$67,786,250
2028	$83,786,250
2029	$88,786,250
2030	$93,786,250
2031	$98,786,250
2032	$103,786,250
2033	$108,786,250

Option 2: COTS Product with IT Participation

HHS will purchase a COTS product, including the rights to update and modify the code by internal IT staff. Internal staff will work with the vendor and system integrator to complete the customizations and configurations. The vendor will be responsible for training IT staff, as they will assume the ability and rights to implement future updates and enhancements without relying on the vendor's development staff. The costs associated with this option include an initial purchase price for the software, annual license fees, annual space and support costs from DOM DoIT, and additional consultants to configure the environment and develop the product. The initial cost is the most significant factor and increases due to a pre-built product, with the help of vendor experts to further customize it. This will improve interoperability and efficiency, provide greater flexibility and ease of enhancement, and allow the various system elements to be configured within the framework without significant disruption.

This alternative is less desirable to DOM DoIT given the amount of time, level of expertise, and support it might require of its staff. It is not clear how long it would take to replace FACS and JARVIS fully and to decommission both if this alternative were selected fully. This alternative increases HHS's need to compromise or potentially develop system workarounds and minimizes meeting HHS's specific policy and practice needs. This alternative is ultimately projected to be more expensive than the Vendor-Hosted, Managed, and Supported Solution.

Table 5-4: COTS Product with IT Participation

Year	Cost – COTS Product w/ IT Participation
2026	$85,974,823
2027	$98,618,179
2028	$101,146,851
2029	$107,468,529
2030	$112,525,871
2031	$115,054,543
2032	$120,358,652
2033	$131,760,949

Option 3: Status Quo

Retaining FACS and JARVIS was deemed infeasible due to their outdated architecture, maintenance difficulties, and inability to meet CCWIS modularity requirements.

5.6 Importance of the Feasibility Analysis

Information from the CBA plays a crucial role in identifying the recommended solution. The deliverable required more careful consideration of the prioritized list of improvements in the gap analysis and the identification of how those priorities would benefit HHS. For an option to be considered feasible, there must be evidence that the benefits of implementing a new system will outweigh the costs of building, deploying, and maintaining it within a reasonable time frame. While a solution may meet all the needs of HHS, if the benefits associated with that implementation do not surpass the costs in the foreseeable future, then that option is not realistic.

5.7 Description of the Selected Alternative

After weighing the quantitative benefits and guiding principles listed above, and carefully considering its needs, goals, and requirements for a new CCWIS, HHS was able to rank the alternatives detailed above. The time and cost to implement a core child welfare case management system were two factors that highly impacted the ranking.

HHS has selected Google Public Sector, LLC (a division of Alphabet, Inc.) as the primary technology partner and platform provider, enabling the rapid deployment of a secure, interoperable CCWIS foundation within one year.

This decision is based on a singular convergence of technical capability, executive commitment, and operational risk assumption that effectively precludes competition. At this specific moment in time, Google is the only hyperscale technology provider willing and able to serve as the prime system integrator, assuming total accountability for the outcome, while providing a vertically integrated technology stack (Infrastructure, Security, AI, and Geospatial Data) that can be deployed within a 12-month timeline.

Alphabet's leadership has expressed a willingness to co-invest and leverage its ecosystem of development partners to deliver the platform at a fraction of traditional market costs, enabling Iowa to achieve federal compliance and operational transformation within one year.

The purpose of this award is to modernize the State of Iowa's 40-year-old child welfare system into a modern, cloud-based, modular Comprehensive Child Welfare Information System (CCWIS) that improves caseworker effectiveness, strengthens data-driven decision-making, and enhances outcomes for children and families. This phase will focus on the co-development of the case management module, in a partnership between Iowa and Google Public Sector, LLC, that is distinguished by:

- **Upskilling and Autonomy:** Iowa's workforce will be upskilled through direct participation in all phases of the project, gaining hands-on experience with modern cloud-native and AI-powered technologies, and will have autonomy in deciding how and by whom the system is maintained.
- **Alignment with Federal and State Priorities:** This approach directly supports the President's Executive Order 14359, "Fostering the Future for American Children and Families," and the recommendations of the Iowa Department of Government Efficiency (DOGE) Task Force, by leveraging public-private innovation, empowering state staff, and creating reusable, modern technology assets for the broader child welfare community.

Key objectives include:

1. **Compliance:** Achieve full technical and functional compliance with CCWIS and all requisite federal/state security standards, including FedRAMP cloud certified, IRS 1075, and Criminal Justice Information Systems (CJIS).
2. **Modernization:** Deliver a modular, cloud-native core system architecture that is scalable, flexible, and future-proof.
3. **Caseworker Support:** Implement advanced Gemini AI-powered tools to automate administrative burden, enhance decision-making, and maximize caseworker time spent with children and families.
4. **Interoperability:** Establish a robust data exchange hub to seamlessly connect child welfare data with external data sources (e.g., Medicaid, Courts, Education, and providers).
5. **Co-Development and Upskilling:** Iowa's child welfare and IT staff will work side-by-side with Google engineers, helping to ensure the solution is tailored to Iowa's needs and that state staff are upskilled in modern software development, cloud operations, and AI integration.
6. **Workforce Autonomy:** This partnership invests in Iowa's human capital, preparing state staff to maintain, extend, and govern the system long term and with full autonomy over future decisions.

This accelerated approach helps to ensure the State meets both federal CCWIS modernization guidance and the State's goal of having a functioning system in production within a 12-month time frame.

Google is the only provider that has publicly and formally committed to delivering a fully functional CCWIS platform that replaces the core legacy system within the required 12-month time frame. A standard competitive procurement process would require 12–18 months just for vendor selection and contract award, making the project timeline infeasible.

Iowa desires to enter a noncompetitive, sole-source contract with Google Public Sector, LLC, under 2 CFR §200.320(c)(4), justified by express authorization and the unique nature of the partnership.

The proposed award to Google Public Sector, LLC is justified under 2 CFR §200.320(c)(2) and (c)(4) because Google offers a unique and proprietary combination of technical architecture, embedded Artificial Intelligence, and a financial partnership model that other vendors cannot replicate.

Additional considerations are as follows:

Timing and Technical Imperative

The proposed solution leverages an AI-first, cloud-native design with proprietary Google technologies to mitigate project risk and deliver high operational efficiency.

1. **Unique Technology Stack:** The architecture is built on containerized microservices (GKE/Cloud Run), providing superior system resilience by isolating failures and improving cost efficiency through elastic, real-time computing scaling.
2. **Proprietary AI and Risk Mitigation:** The system incorporates proprietary Gemini AI and Document AI to reduce administrative burden through automated voice-to-note summarization and document data extraction. Additionally, Gemini Canvas enables rapid prototyping of requirements and auto-generation of development blueprints, significantly reducing requirements-related risk in ways not currently available from other providers.

Unique Technical and Partnership Qualifications

The delivery timeline and operational requirements necessitate a noncompetitive award.

- **Accelerated Delivery Requirements:** Replacing a 40-year-old legacy system and deploying a federally compliant CCWIS foundation within 12 months cannot be achieved if a traditional 12–18-month competitive procurement cycle is initiated.
- **Co-Investment and Resource Mobilization**: Google's commitment to co-invest and mobilize its global partner ecosystem reduces overall project cost and lowers risk by working to ensure immediate access to specialized expertise and delivery capacity.
- **Security and Compliance:** Google Cloud offers a hyperscale environment meeting FedRAMP, IRS 1075, and CJIS standards—capabilities essential for secure and compliant handling of child welfare data. Assured Workloads and zero-trust architecture help to further ensure that sensitive child and family information remains fully protected.

Prior and Ongoing Relationship

Google has been adopted as the State of Iowa's foundational Enterprise Data Platform (EDP), serving as the strategic backbone for ongoing modernization efforts. This includes replacing the State's core Data Hub and Justice Data Warehouse and supporting advanced AI initiatives with agencies such as Department of Transportation and Department of Public Safety.

Leveraging Google Cloud for this CCWIS initiative aligns with the State's existing investments in Google Cloud, Workspace, and Security Services, enabling accelerated deployment within a familiar, approved, and supported infrastructure.

Continuation of an Ongoing Activity

This initiative represents the next phase of Iowa's broader modernization strategy and builds upon existing cloud, data, and AI transformation efforts already underway.

Evidence of Uniqueness

Market research indicates that while several vendors offer cloud services, only Google has committed to:

- Delivering a CCWIS platform within one year
- Providing the solution at a reduced cost through co-investment
- Engaging a global partner ecosystem to accelerate delivery and innovation in child welfare technology
- Presenting a comparable combination of capabilities, delivery readiness, co-investment, and timeline commitment
- Providing all templates, reference architectures, and foundational code developed for this implementation to other child welfare systems at no cost. This level of open contribution is atypical in the commercial marketplace and reflects a unique public-sector partnership model that strengthens the justification for a non-competitive award

Upskilling and Workforce Autonomy

Google is the only provider that has agreed to include Iowa state resources in all phases of the build. Iowa's staff will be upskilled in modern software development, cloud operations, and AI integration, working to ensure the State's workforce is prepared to maintain, extend, and govern the system long-term, with full autonomy to decide how and by whom the system is supported and enhanced.

Alignment with Federal and State Priorities

The initiative is fully aligned with Executive Order 14359, "Fostering the Future for American Children and Families," which calls for modernizing state child welfare information systems, expanding the use of AI, and promoting partnerships between states and leading private sector organizations.

Iowa concludes that Google Public Sector, LLC is the only entity capable of fulfilling both the technical and policy-driven objectives of the CCWIS modernization within the required time

frame due to the unique combination of the containerized microservices platform, embedded Gemini AI capabilities, and the accelerated delivery partnership. A competitive procurement would increase costs, delay implementation, and risk failure to meet federal and state commitments. Google's partnership approach, executive alignment, and unique cost-sharing model create an unprecedented opportunity for Iowa to modernize at scale and speed.

5.7.1 Functional Considerations

5.7.1.1 Business Process Management

A primary initiative for the new CCWIS solution is to enable HHS resources to be more efficient. After an analysis of the existing business processes, HHS has determined that there are two (2) major factors to achieving greater efficiency:

- **Decrease information delays:** This goal requires the new CCWIS solution to obtain information from external sources when needed, ingest and associate reference data with child protection data, work to ensure data quality is adequate, and provide self-service reporting.
- **Decrease human interaction:** This goal requires the new CCWIS solution to automate business workflows, validate user inputs, reduce decision-making via business/data rules and automation, and provide monitoring and alerting on specific business events.

Figure 5-1: Business Process Vision for VISION



5.7.1.2 Business-focused Technical Capabilities

The following list details the business-focused technical capabilities that will enable the business objective of increasing efficiency.

- **Business Process Automation:** The new CCWIS solution must provide automated workflows, monitoring, and alerting on business events, and user input validation to effectively automate child protection business processes, reducing the need for manual intervention, decreasing manual mistakes, and ultimately increasing HHS resource productivity.
- **Automated Decision-Making:** The new CCWIS solution must promote the use of configurable business rules to enable data-driven decision-making, reducing the effort required to make business decisions while minimizing incorrect determinations.
- **System-to-System Communication:** The new CCWIS solution must communicate with existing HHS systems, State of Iowa systems, and external systems (including federal systems) to enable automated workflows and reduce manual intervention (e.g., data entry).
- **Data Sharing:** The new CCWIS solution must be capable of sharing enterprise data with the Unified Data Platform and external data sources to minimize data latency, reduce manual intervention currently required to obtain data from external systems, and aggregate data to provide a holistic view of a child protection case.
- **Data Quality Management:** Adequate data quality is critical for increasing HHS resource efficiency, working to ensure social workers and administrative staff are making accurate data-driven decisions. The new CCWIS solution must work in conjunction with the Unified Data Platform to help ensure HHS resources use high-quality data.

5.7.1.3 Business User Experience

Currently, HHS business users, including social workers, must utilize a multitude of HHS systems, including the child welfare system, eligibility systems, document management systems, and payment systems. This process has resulted in the following:

- Additional training required for each system
- Increased delays to processes related to ensuring child welfare
- Increased risk of errors due to manual mistakes
- Duplicate data entry may be required, increasing the risk of inconsistencies between disparate data sources
- Limited adoption of new system features and upgrades

In addition, the current system does not support a mobile interface, requiring business users to use their state-issued laptops and an internet connection to access HHS applications. In addition, the current suite of HHS applications does not support offline work. Social workers are

often working from remote locations where internet access is limited or unavailable. This creates process delays and increases the effort required from business users.

Selecting Google Public Sector as Iowa's technology partner will significantly change the user experience within the child welfare system. A modern, cloud-native CCWIS will replace outdated systems with an intuitive interface that streamlines workflows and reduces administrative burden. Personalized dashboards, mobile access, and real-time data will improve efficiency and responsiveness, while Gemini AI tools automate documentation and support decision-making. Seamless integration with courts, Medicaid, and education systems will reduce duplication and help ensure data accuracy. Combined with workforce upskilling and autonomy, these innovations create a faster, smarter system aligned with best practices, empowering staff and improving outcomes for children and families.



The new CCWIS solution must improve the user experience by minimizing the number of interfaces required to perform social work and other business duties, promoting system-to-system communication for information gathering, and providing a mobile experience that includes offline working capabilities. The future user experience will provide the following:

- A user interface that reduces the required skill set(s) by providing a unified, cohesive user experience.
- A reduction in process delays by displaying information and automating process workflows based on data from disparate data sources.

- An increase in process efficiencies and user productivity by supporting offline work.
- Increased accuracy and a reduction in manual mistakes by eliminating duplicate data entry and prepopulating data from disparate data sources.
- Increased adoption of new system features and upgrades.

Figure 5-3: Future User Experience



5.7.2 Technical Considerations

5.7.2.1 Recommended Solution Conceptual Model

One of the objectives of the CCWIS project is to provide a modular based solution that allows for extensibility, flexibility, and rapid change. The recommended solution helps meet this objective by providing the following:

- A data warehousing platform, known as the **Unified Data Platform**, to facilitate data analytics, data sharing, and data integration efforts.
- [redacted]
- [redacted]

Figure 5-4: Recommended Solution Conceptual Model

[redacted]

5.7.2.2 Networking Architecture



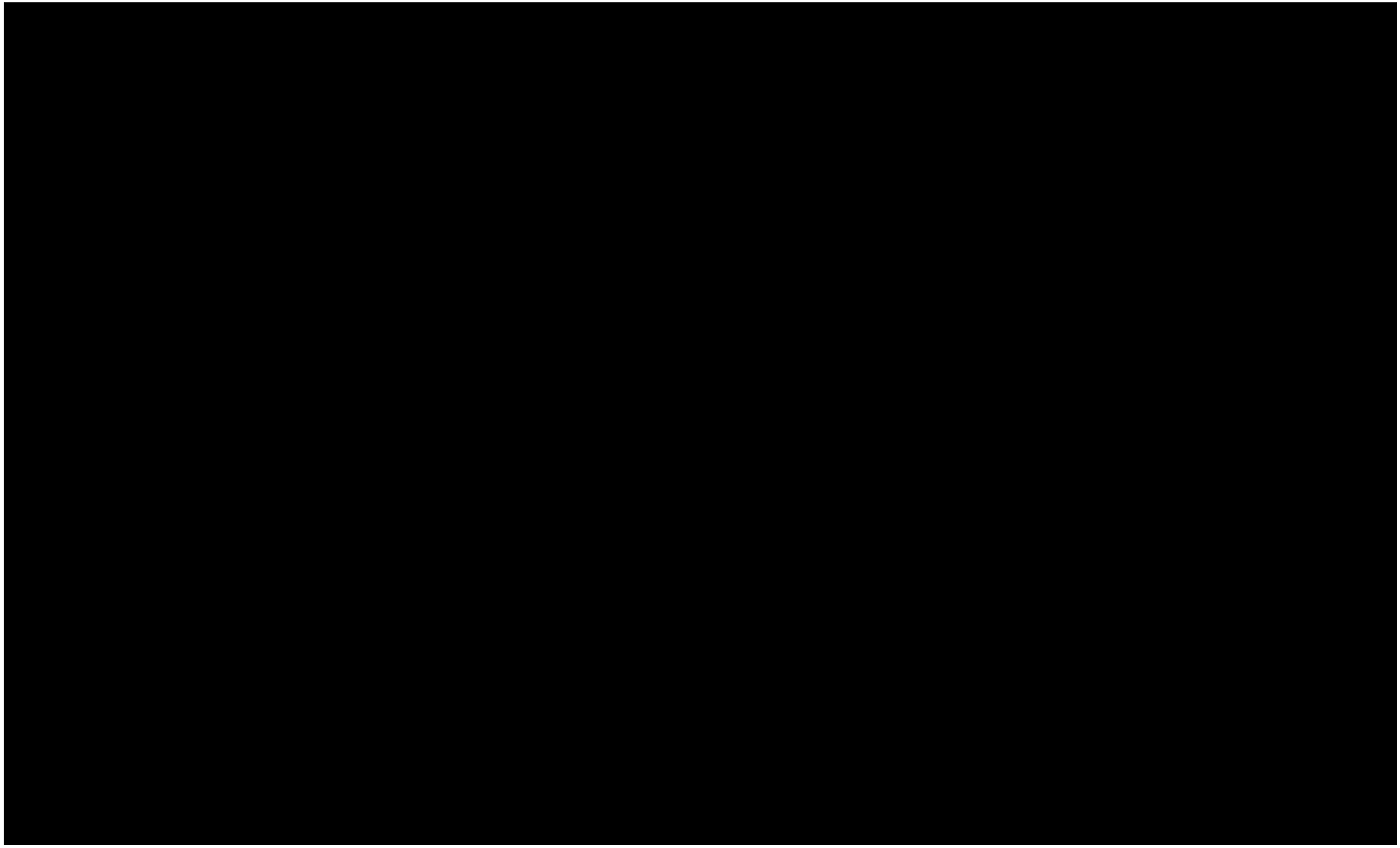
Figure 5-5: Current Contextual Network Diagram



[redacted]

5.7.2.3 Unified Data Platform Architecture

Figure 5-7 depicts the logical architecture for the Unified Data Platform based on the data warehousing technical capabilities that are required to meet the desired business outcomes.



Enterprise Data and Business Intelligence Strategy

The purpose of the Vision Unified Data Platform (VUDP) and Business Intelligence Strategy is to promote a Family Wellbeing and Protection (FWP) data strategy that leverages the correct data, delivered at the right time, to drive actionable programs to support its specific initiatives and align with enterprise-wide strategic initiatives for Iowa.

Specifically, VUDP **Data Strategy** positions data assets and associated data management processes to improve the acquisition, storage, management, transformation, sharing, and use of data across the division, supporting and aligning with business objectives.

To accomplish this, the strategy documents addressed findings (where we are today), desired outcomes (where we want to be), gaps and solutions, by focusing in detail on the different disciplines within its data management:

- **Data access:** The ability to retrieve and utilize data from various storage systems. It encompasses the process of making data available to authorized users or systems, controlling who can access what data, and under what conditions.
- **Data sharing:** The practice of making data accessible to multiple users, applications, or organizations. It is a fundamental aspect of modern data management, enabling collaboration, efficiency, and innovation across various domains.
- **Data security:** The practice of protecting digital information from unauthorized access, use, disclosure, disruption, modification, or destruction. It encompasses a range of measures, including physical and logical security, access controls, and organizational policies, designed to help ensure the confidentiality, integrity, and availability of data.
- **Data usage:** the standards and processes for collecting, storing, organizing, and analyzing data related to how users interact with products or services.

Further, an effective **Business Intelligence Strategy** matures the organization's competencies to leverage data into actionable programs that meet business requirements. Specifically, it articulates the maturation of data and analytics necessary across the enterprise from a resource, process, and technology perspective. It focuses on best practices in data staging, assembling, reporting, and analytics.

VISION Unified Data Management Objectives

Best practices recommend VUDP Data Management to align with the HHS business vision by enabling data strategies that support the activities within the above initiatives.

VUDP Data Management objectives:

- Cultivate and promote FWP data as an HHS-wide enterprise asset.
- Govern, secure, and store enterprise data associated with citizen engagement processes in a manner that improves operational efficiencies and business process effectiveness.

- Utilizing optimal data strategies to improve the availability of enterprise data via a well-reasoned data sharing and analytics platform with data exchange connectivity to existing data sources, including HHS intra-agency sources, Iowa inter-agency sources, and external data sources, such as federal or other state data sources.
- Improve divisional capacity for collecting and using complex data that spans Family Well-being and Protection services across all exchanges, including the data internally produced and data that is externally sourced and collected.
- Secure and manage VUDP data in alignment with federal, state, and HHS compliance and reporting requirements.

Figure 5-8: VISION Unified Data Flow

VISION Unified Data Flow



5.7.2.4 Recommended Solution Reusable Components

HHS has existing architecture components that can be reused with the recommended solution in addition to newly proposed reusable components. Existing and reusable components are listed in **Table 5-5**: Data Services Layer.

Table 5-5: Data Services Layer

Data Services Layer	
HHS Legacy Data Warehouse	Currently integrated and in use by child welfare.
Unified Data Platform	Proposed cloud-based data platform used for data warehousing, data analytics, and data sharing purposes.
Cloud Integration Services	Proposed Cloud-to-Cloud-based data exchanges that do not require connection to on-premises systems.
API Management	Proposed service to manage system-to-system authentication, authorization, monitoring, throttling, and troubleshooting capabilities for data exchange.
Enterprise Service Bus/Message Queue	Proposed data exchange service to utilize data exchange queueing and asynchronous process management.
CCWIS APIs / REST Microservices	Provided by the selected CCWIS solution to enable system integration with external software components.

In addition to the project governance identified in Google's statement of work (SOW) and **Section 7.6.1**, HHS will include the standards listed in **Table 5-6.**

Table 5-6: Data Governance Standards

Data Governance Standards	
Enterprise Data Model	Developed by the Iowa DOM Enterprise Architecture Bureau. Recommended solution data model will be mapped to the Enterprise Data Model for implementation in the data warehouse and data lake consumption
Data Exchange Models	Data exchange document templates that include security model, security access groups, process model, data model, data exchange format specifications between systems
VISION Unified Data and Business Intelligence Strategy	Comprehensive strategy document detailing the acquisition, curation, and storage of enterprise data for data analytics and business intelligence purposes.
VISION Unified Data and Regulatory Compliance Policy	Policy that details the technology, processes, and resources required to ensure compliance with HHS, Iowa, and federal compliance and regulatory requirements.

Data Governance Standards	
VISION Data Classification Policy	Policy detailing the data classification process used by HHS in conjunction with data security, data access, and data retention policies.
VISION Data Retention Policy	Policy detailing the data retention processes for data storage technologies, including the administration of retention schedules based on state and federal requirements.
VISION Data Quality Management Policy	Policy detailing data quality technologies, processes, and data quality rules (business and technical) to be used to ensure an adequate level of data quality.
VISION Data Encryption Policy	Policy that defines data encryption standards, encryption technologies, and associated processes to ensure data encryption is used effectively to deter security attacks.

Application Layer Reusable Components

Table 5-7: Application Services Layer

Application Services Layer	
Geospatial Analytics and 3D Mapping	Proposed service to be utilized for route planning and address validation.
Central Print Services	Existing internal print services for HHS are currently integrated with the transitional CCWIS and legacy child welfare systems.
Electronic Forms & Digital Signature Management Services	Proposed service that will be used to administer electronic forms, including the development, distribution, and processing of forms.
API Management	New public cloud API Management solution that the modules in the CCWIS implementation will use.

Infrastructure Layer Reusable Components

Table 5-8: Infrastructure Services Layer

Application Services Layer	
Security (Okta)	Reuse existing statewide identity access management (IAM) solution for authentication and authorization purposes.
Mobile Device Management (Intune)	Extended use of the existing Intune implementation with additional capabilities beyond mobile email access. Extended capabilities include security and access controls for mobile phone devices.
Email, Task, & Calendar Services (Exchange Online)	Continued use of Exchange Online services with potential extension with synchronization for email, tasks, and calendars
Cloud Network Infrastructure Services	Use of public cloud network infrastructure services for security and integration requirements

6.0 Nature and Scope of Activities to be Performed

The systems development methodology will be Agile in the sense that it is incremental but major work will be sequenced to ensure a progression from validated requirements to testing and deployment.

The following lists the case components and activities that will serve as the core of HHS's case management CCWIS from beginning to end at both the case and person levels, including adoption and ongoing subsidy payments. The asterisked components and activities fall outside build scope in Year 1 (Minimal Viable Product) and will be addressed immediately following in Year 2.

1. Draft
2. Pending Assignment
3. Case Set Up
4. Pending Approval
5. CPW/SWCM Transfer
6. Dashboards
7. Search
8. Active case
9. Identifiers
10. Subsidized Guardianship*
    Excludes the ability to Initiate Guardianship Payment Details, negotiate subsidies, or manage payment approval/adjustment logic. The complex financial management of the subsidy itself will not be included with MVP release.
11. Runaway
12. Indicators
13. Case Overview
14. Person Profile
15. Associations
16. Living Arrangements*
    Excludes write-capabilities to FACS for data collected in VISION related to living arrangements (placements). The financial processing and approval of payments tied to living arrangement types will not be included with MVP release.
17. Services
18. Court*
    Excludes automated statutory finalization of cases, including bidirectional integration with Iowa Courts. The UI captures the data, but the automatic exchange of legal documents and final disposition updates with Iowa Courts will not be included with MVP release.
19. Notes
20. ECF
21. Health
22. Education/Income
23. Planning
24. Assessments
25. Abuse/Criminal History*

Data pushed from JARVIS assessments will be read-only. MVP release will focus on consolidating the read view, deferring the complex, high-risk bidirectional write logic back to external systems.

26. Billing/Payments*
    Excludes all functionality related to financial transactions, payments, subsidy payments, billing, or financial reconciliation and will not be included with MVP release. This includes the ability to process claims, make payment adjustments, or auto-approve payments.
27. Timeline
28. IVE*
    Excludes all functionality related to IV-E eligibility determination and calculation. The complex logic involving AFDC rules, income tests, and grouping functions will not be included with MVP release.
29. ICPC
30. Adoption*
    Excludes automated statutory finalization of cases. The creation of the new person ID/State ID for the adopted child post-finalization is excluded. Adoption Subsidy Negotiation and payment workflows are also deferred as part of the financial exclusion and will not be included with MVP release.
31. Native American/ICWA
32. Left Navigation
33. Case Closure
34. Provider Management*
    Excludes full end-to-end module functionality, business rules, or financial logic for Provider Management. Developing write-capabilities to update provider records (licensing, credentialing, contract management) is deferred and will not be included with MVP release.
35. Non-Agency Services
36. User Management/Security

HHS and Google Public Sector Services (GPSS or Google) developed a scope and proposed contract Statement of Work to execute the mission-critical modernization of its CCWIS. This initiative replaces the legacy FACS mainframe and JARVIS application with a modular, cloud-native system designated as "VISION." This SOW governs the first 12 months of the project, focusing strictly on the “Initial Case Setup" and "Core Case Management" modules. This will encompass producing a "single source of truth" for case information and achieving operational readiness within the fixed timeline through rigorous scope containment and governance.

Together, HHS and Google developed the proposed project scope as stated in the contract as **Workstream 2.1. The Services under Workstream 2.1 are governed by the Implementation Services Schedule, which is incorporated into the contract.**

Section 8 includes the project schedule for each workstream included in the proposed Google contract. **Workstream 2.1 includes the following sub-workstreams as negotiated with Iowa:**

- Workstream 0: Project Oversight and Governance
- Workstream 1: System Development

- Workstream 2: Data Platform
- Workstream 3: Training Enablement
- Workstream 4: Transition to Managed Services Provider

Workstream 0: Project Oversight and Governance

Strategic Program Enablement:

- Engage with key program stakeholders to define goals, gather requirements, and establish expected outcomes of the program reflected in a high-level plan of deployment activities to be completed based on Google's leading practice methodology
- Host a pre-project kick off meeting with the customer to determine if the prerequisites have been met, set expectations, and discuss project execution strategy
- Provide status reports
- External reporting:
    - Provide necessary information and artifacts related to the Google scope of work to support the Customer in its development and submission of required advance planning document (APD) updates

Establish the Program Governance Office (PGO):

- Outline the purpose of the Program Governance Office (PGO), how it relates to other bodies at the customer, and the extent of governance authority
- Document the role and responsibilities of the PGO, including, but not limited to:
    - Change Control Board
    - Standardized communication methods
    - Backlog curation and approval of scope modifications
- Define roles, responsibilities, and accountability across workstreams and decide how they will interact with the PGO
- Provide communication requirements to be used as a guide for teams who will need to escalate requests through the PGO and how decisions will be communicated through the PGO

Change Control Board:

- Empower customer stakeholders to manage collisions between various workflows
- Provision and manage a product backlog and approve or deny scope changes within 48 business hours
- Maintain a formal "Not Now" list for this 12-month release to record desired future features, functions, or requirements that are out of scope for the current SOW; this

mechanism supports transparent backlog management and validates joint alignment with the State

- Engage with the customer to prioritize the capabilities/features within the scope of this project.

Workstream 1: System Development

Business Context and Requirement Knowledge Transfer:

- Conduct stakeholder group sessions (as required by Google) to conduct knowledge transfer of the existing documentation
- Identify business workarounds currently dependent on application or off-system processes
- Use tooling to extract business rules, logic flows, and calculations from legacy code

User Interface Design:

- Conduct review meetings with stakeholders to existing designs and determine remaining refinement
- Finalize up to 40 existing customer webpage/tab designs and/or UI/UX workflows, as identified in the business process documentation, by finishing design details, minor adjustments, or adherence to brand guidelines
- Conduct sessions to gather input from stakeholders, caseworkers, and supervisors, optimizing business processes where possible to inform the final design
- Apply human-centered design principles to layout, navigation, and interaction models
- Provide mobile-responsive designs that incorporate Section 508 accessibility tenets to support broad accessibility and flexibility
- Provide a material UI component library of design elements
- Provide UI workflows based on business process maps and user feedback; verify designs are mobile-responsive

Application Implementation:

- Integration Component Development:
    - Build and configure the ingestion/export pipelines specific to Core Case Management
    - Provision the APIs, data feeds, or event-driven components required for Core Case Management
    - Provide new translation or mapping layers needed to reconcile the Core Case Management data model with legacy systems.

- Design and write the initial case setup and Core Case Management functionality using cloud-native services, covering the following In-Scope Workflows, Tabs, Widgets and Dashboards:
    - Person Search and Profile:
        - Person Search interface
        - Person Details widget
        - Person Profile
        - Person-Level Electronic Case File
    - Case Setup Workflow:
        - Case Overview (central case hub)
        - Associations Workflow Step
            - Person-to-case linking, participant list
        - Services Workflow Step
            - Record Out-of-Home Living Arrangements and essential services
        - Checklist Workflow Step
        - Approval Workflow Step
        - Social Worker Case Manager (SWCM) Checklist Workflow Step
        - Workflow Progress Bar overlay
        - Living Arrangements
    - Electronic Case File:
        - Electronic Case File (ECF)
        - Case-Level ECF
        - Upload Document Widget (accessible throughout VISION)
    - Worker Safety:
        - Abuse and Criminal History
    - Minimum Court Development:
        - Court (Case Level)
        - Person-Level Court
    - Child Protection Worker (CPW) Dashboard:
        - All Cases

        - Draft Cases
        - Pending Assignment
        - Pending Transfer
    - SWCM Dashboard:
        - Active Cases
        - Pending
        - Pending
        - Active
        - Transferred
        - Shared With
- Analytical and Generative AI (GenAI) Capabilities:
    - Implement GenAI capability to read narrative reports, and extract key risk indicators (e.g., “drug use,” “unattended child,” “physical injury”).
    - Implement the ability for the system to identify why certain words or patterns triggered risk signals (for explainability).
    - Define three GenAI capabilities to implement for case management such as:
        - Draft case notes, court summaries, or service referrals using structured case data.
    - Implement defined GenAI capabilities into Case Management workflows
    - Implement speech-to-text and summarization for dictating notes into structured entries
    - Implement document parsing capabilities
- Application Testing & Validation:
    - Provide an automated (where possible) test suite covering unit, integration, and functional test cases
    - Align test scenarios with traceable requirements verified against Business Process and Business Logic
    - Execute continuous testing within the CI/CD pipeline
    - Collaborate with QA team and UAT testers to validate functional and non-functional behaviors
    - Conduct initial performance profiling to establish baseline performance benchmarks and monitor thresholds for key user flows

- Observability and Service Level Agreement (SLA) Monitoring:
    - Set up dashboards to monitor integration health (e.g., throughput, error rate, latency)
    - Define alerting thresholds for service-level breaches or data sync anomalies
    - Enable audit-level tracing for compliance and forensics

Future Module Roadmap Development:

- Prepare a roadmap for the remaining case management business process workflows and integrations that are not part of the critical path
- Prepare a roadmap for modernizing the remaining CCWIS modules:
    - Use tooling to extract business rules, logic flows, and calculations from legacy code
    - Provide a business rules catalog with rule descriptions, code references, and triggering conditions
    - Segment the remaining module capabilities based on business capability and technical cohesion
    - Provide recommended sequencing based on priority, complexity, and risk
    - Engage with the customer to prioritize the capabilities and develop a notional release plan based on the discovery and design activities

Cloud Foundation Deployment:

Assessment:

- Conduct three – six remotely-hosted, two-hour sessions, as determined by Google, to collect and review the technical requirements and objectives. This activity will cover the following areas:
    - Organization structure and resource hierarchy:
        - Define requirements for organizational hierarchy, including folders, projects, and organizational policies
        - Define requirements for naming conventions and labeling approaches for folders, projects, and resources
        - Define requirements for access and security controls for folders, projects, and resources
        - Define requirements for cost management model, including spend visibility and budget alerting
    - User and access management:

        - Identify the process and requirements for user authentication and authorization and/or Single Sign-On (SSO) federation to an Identity Provider (IdP)
        - Define conventions for users, groups, and service accounts roles in Google Cloud Platform (GCP) and the policies associated with these roles
    - Technical Infrastructure:
        - Define Virtual Private Cloud (VPC) requirements
        - Define specific roles for specific service administration (storage, identity, etc.)
        - Define any network connectivity requirements (site-to-site VPN, cloud interconnect)
    - Infrastructure-as-Code (IaC):
        - Define Git-based repository for storing and managing Infrastructure-As-Code (IaC)
    - Logging:
        - Define framework to integrate Google Cloud logs to customer's Security Incident Event Management System (SIEM)
    - Migration:
        - Review current storage assets, analyze data volume, access patterns, and performance requirements to define migration patterns and strategies
    - Assured Workloads:
        - Review customer requirements for the advanced security features to store or process regulated or sensitive data (e.g. HIPAA, FedRAMP, ICWA, CJIS)

Design:

- Provide a solution that meets customer requirements collected in the assessment phase and provide leading practices, recommendations, and design guidance
- Provide a Technical Design Document (TDD) draft that describes the future state and design details in the following areas:
    - Organization structure and access:
        - Design organizational hierarchy, user access, and roles
    - Network:

- Design initial network topology to facilitate connectivity to service applications that will be deployed in Customer's GCP, on-premises, and/or other cloud environments
- Design network security controls (firewall configuration)

- Storage:
  - Identify policy needs for areas including storage, data storage, data residency
  - Design for backup, recovery, disaster recovery, high availability
  - Provide guidance on data governance, data management, data life cycle management, data ingestion, data processing, naming conventions, environments, data sharing and federation
  - Provide guidance on data security and encryption policies
- Security:



- Billing and Cost Control:
  - Validate billing account and billing administrator roles are correctly configured
- Logging/Monitoring/Alerting:
  - Define resource and audit logging tooling and processes for accessing relevant logs
- Automation:
  - Recommend automation strategy for repeatable deployments and management of the GCP environment
- Disaster Recovery:
  - Design for failure based on customer defined recovery time objective (RTO) and recovery point objective (RPO) requirements

- High Availability:
  - Design for customer high availability requirements
- Assured Workloads:
  - [redacted]

Implementation Support:

- Infrastructure Deployment:
  - Implement IaC to deploy all infrastructure resources
- User and Access Management:
  - Integrate identity provider according to the design specifications and conduct functional testing
- Logging and Monitoring:
  - Activate Cloud Logging for all relevant GCP projects and services
  - Define log retention policies
  - Activate Cloud Monitoring for all relevant GCP projects and services
  - Define alerting policies and configure notification channels
  - Implement Google Cloud log shipment to the Customer's SIEM
- Security:
  - [redacted]

- Test all delivered resources, including:
  - Conduct testing to validate the functionality of the logging and monitoring setup
  - Trigger test alerts to verify notification channels and response procedures
  - Simulate load and stress conditions to validate that the logging and monitoring system can handle production volumes

Standard Implementation Support:

- Guide the customer's technical team for the customer's design and implementation of any SSO federation requirements.
- Implement and validate the following:
  - IaC source repository and CI/CD using Cloud Build
  - Root GCP organization and the organization administrator access and roles
  - Billing account, payment profile, and billing administrator access
  - Folder and project hierarchical structure and labels according to the defined labeling convention
  - Initial user groups in Cloud Identity and user groups role assignment in Identity and Access Management (IAM) according to the defined user access management convention
  - Access control for the foundational resource hierarchy
  - Network topology including VPC (single or shared mode) and network security controls (firewalls, routing policies)
  - Cost management and tracking model
  - Cloud Operations Suite project for Cloud Monitoring and Cloud Logging
  - SCC Standard (SCC Premium or Enterprise may be substituted if customer has purchased the required SKUs)

Integration Services Design:

- Define source to target mapping for VISION Integrations
- Evaluate and design data migration process to migrate data FACS to VISION
- Evaluate requirements for receiving data from JARVIS to VISION for specified tables
- Define integration patterns (real-time API vs. batch) for required interfaces supporting the case management application

- Design API contracts and endpoints for synchronous data (table) exchanges
- Design batch (table) integrations from required systems
- Validate integration availability and usability
- Define integration boundaries and service interfaces for mainframe-to-cloud system interactions
- Generate early-stage infrastructure and integration service recommendations

Implementation:

- Build and configure up to 28 required integrations, as requested by customer, to VISION (e.g., overnight case updates), including necessary data transformations.
- Expose up to five, as determined by Google, integrations that make VISION data available to other internal systems.
- Implement retry logic, error capture for failed exports.
- Integrations:
    - Person Lookup and State ID Management:
        - Implement API client wrapper for Iowa Automated Benefit Calculation System (IABC) person search and State ID operations
        - Implement duplicate prevention logic using search results to identify correct identities
        - Provision data transformation layer mapping VISION person model to/from IABC formats
    - Global360 Document Retrieval:
        - Implement an API client for document search and retrieval for files
    - FACS Data Loads:
        - Implement FACS to VISION data integration for required tables
    - JARVIS ODS:
        - Implement integration for the required JARVIS ODS data to VISION
    - JARVIS Magic Button Draft Case Creation:
        - Provision pipeline extracting completed incident data from JARVIS ODS
- Implement integrations or read-only views to utilize required data from out-of-scope systems or functionality:
    - Living Arrangements:

        - Configure read-only provider lookup from FACS I3 for licensed home selection
        - Provide Read, Update Delete operations for Living Arrangement type, start/end dates, AFCARS minimum data elements
    - Associations:
        - Leverage existing JARVIS Magic Button to auto-pull case members/associations
        - Leverage Person Search for duplicate prevention when adding associations
    - Subsidized Guardianship:
        - Read-only display of existing subsidy payment history from FACS mainframe

Workstream 2: Data Platform

App Deployment, Data Deployment, CI/CD:

- Set up CI/CD pipelines to support automated deployment, testing, and policy validation across environments
- CI/CD pipeline(s) will implement the following:
    - Artifact repository for pipeline templates and scripts
    - Provision artifacts (containers/functions) and push to the registry with versioning
    - Utilize secrets management for credentials and API keys
    - Multi-stage pipeline with approval gates and additional approvals for Prod deployment
- Implement logging patterns and monitoring configurations for pipeline runs

Data Strategy and Parallel Migration:

- Data Profiling
    - Conduct four, 60-minute meetings to review existing Data Dictionary documentation and source-to-target mapping documents
    - Validate or conduct script-based profiling of source tables, gathering volume, size and shape data to inform the target data model
    - Perform a Data Loss Prevention review of required data to identify potential PII/PHI
- Data Model Discovery and Mapping

- Use AI-driven assessment tools to scan and inventory legacy code artifacts (COBOL, JCL, DDL) to extract business rules and data structures
- Confirm design for VUDP
- Design data model including core entities, relationships, constraints, and dependencies relevant to child welfare domains:
  - [redacted]
  - [redacted]
- Provide field-level mappings from source to target schema
- Provide Entity-Relationship Diagrams (ERDs) and mapping diagrams
- Provide a physical data model

- Data Pipeline
  - Provision reusable pipeline templates with error handling and logging
  - Produce up to 50 data pipelines, as determined by Google, for ingestion and transformation datasets
  - Implement transformations and business logic
  - Utilize code conversation techniques to convert legacy business logic to modern language
  - Integrate automated data quality checks (up to 10 checks per pipeline, as determined by Google)

  Data Validation

  - Define 50 data validation rules for ongoing incoming and outgoing data
  - Implement framework for automated data validation and reconciliation
  - Establish automated quality checks for completeness, accuracy, consistency, and timeliness, per pipeline
  - Validate data replication and completeness in target environments

- Validate end-to-end data flows

Data Migration

- Provide a migration plan with dependencies, milestones, and critical path and rollback plan
- Provide a data migration validation process and associated scripts to confirm completeness and accuracy
- Perform ongoing data integration in lower environments during development
- Perform one pre-production test data migration in a lower environment
- Execute the final data migration cutover process during the go-live event
- Perform data validation on data migrated to VISION

Virtual Data Platform

- Provide 20 database views from the VUDP available for usage by other internal systems or processes.

Workstream 3: Training Enablement

Enablement Artifacts

Provide training documentation aligned with the rollout roadmap of the system functionality and roles.

Produce end-user enablement materials (written guides, video tutorials, quick reference cards, etc.).

Up to three written user guides as required by the solution.

Up to five single-page job aids (for specific processes/focus areas) as required by the solution.

Up to five video tutorials (one – two minutes each) as required by the solution

One set of frequently asked questions (FAQs), organized for easy end user reference

Additional materials to be determined based on end user need and team capacity

Conduct either (a) five or fewer initial enablement (train-the-trainer) sessions each not to exceed two hours in duration OR two initial enablement (train-the-trainer) sessions, each not to exceed four hours in duration (as determined by customer needs and logistics).

Hypercare Support (post-production):

Provide a production rollout deployment plan

Prepare the production environment

Setup logging and monitoring for prod environment

Provide advisory guidance to the customer during rollout to the new production environment

Monitor the application for up to four weeks, as determined by Google, and provide troubleshooting/break-fix support

Prepare a handover report, which may include recommendations on future enhancements and changes

Workstream 4: Transition to Managed Services Provider (prior to the conclusion of engagement):

Conduct one, 90-minute configuration workshop to provide guidance on the following topics:

Configuring IAM access for SCC

Enabling Security Sources for SCC

Identifying required audit logs for Security Sources

Identifying additional views of SCC to be implemented (e.g., organizational, folder, project)

Mapping compliance frameworks to detectors

Conduct one 90-minute workshop on managing SCC findings. Topics include:

How to create, filter, and mute notifications from Event Threat Detection (ETD), Container Threat Detection (CTD), and VM Threat Detection (VMTD)

How to create, filter, and mute notifications from Security Health Analytics (SHA)

How to create custom modules for Threats and Vulnerabilities

Conduct one, 90-minute workshop on setting notifications and reviewing Security Command Center (SCC) dashboard. Topics include:

How to configure notifications with external destinations

How to export and analyze SCC findings into BigQuery

Conduct one 90-minute interactive session on reviewing SCC finding Playbooks Topics include:

Responding to a credential (user account) compromise from an ETD alert

Responding to a computing engine compromise from a VM Threat Detection alert

Responding to GKE container compromise from a Kubernetes Threat Detection alert

Schedule and conduct one remotely hosted, two-hour transition coordination meeting

Conduct two remotely hosted, two-hour knowledge transfer meetings scheduled within 10 business days of each other

Answer any outstanding questions or concerns within two business days following each transition meeting

Review Terraform code samples (as described in the SOW deliverables)

Walk through the Technical Design Document (TDD) during one remotely hosted, two-hour meeting.

HHS has developed a list of primary expectations and goals as part of DDI of the CCWIS. **These goals include the following:**

Intuitive Workflow: The new system will offer a streamlined entry sequence, with prompts that notify the worker if information is missing or appears incorrect. The system will guide field staff through the life of a case, mirroring child welfare best practices.

Ease of Data Entry: The new system will eliminate the reliance on memorization and repetition by replacing the code-heavy structure. It will also provide clarity on confusing terms or policies, encouraging a more standardized entry of information and increased data quality.

Decision Support: The new system will be built on business rules and alerts developed from child welfare practices. These rules and alerts will help guide social workers through a case by providing workflow prompts.

Personalized Dashboard and Reporting: The new system will provide users of the application with a customized dashboard based on their responsibilities and needs. The dashboard will help field staff view their caseload, showing upcoming tasks, and will offer more consistent reporting across HHS. Leadership, policy, and other stakeholders will have on-demand access to data.

Document Management: The new system will support electronic forms and allow the electronic storage of documents within the case file. The system will also support the collection of electronic signatures.

Eliminate Duplication: The new system will reuse and auto-populate previously entered information to populate appropriate locations, documents, and forms. This will improve data quality by reducing the opportunity for errors during entry.

Clear Relationship Connections and Mapping: The new system will provide field staff and analysts with a clear picture of a family system and the other relationships that occur within a case. The ability to view a family system will help workers understand a family's history with HHS, identify potential kin or fictive kinship placements for children when needed, and properly report on individual or family outcomes.

Comprehensive Data Collection: The new system will collect child welfare data more robustly through interfaces with external partners and a review of data reporting needs.

Real-Time Data Processing: The new system will manage data in real time whenever possible to eliminate lag in communication between systems and in reporting.

Mobility: The new system will offer mobile functionality, allowing field staff to view and edit information, manage electronic forms, and obtain electronic signatures when in the field.

Flexibility: The new system will be flexible enough to support changes to child welfare practice and policy, as well as any reporting requirements that may arise.

Along with the overarching goals above, the automated functions contained in a CCWIS will follow a modular design as stipulated in 45 CFR 1355.53. These functions will interface together and run seamlessly. The application will include data exchanges and/or portal interfaces with third parties such as the courts, contractors, youth and caregivers, Iowa Medicaid, Iowa DOE, and Public Health. HHS will review current reporting applications to determine if any may be absorbed into the new system and examine and revise processes as needed to help ensure all reporting requirements are met. Any outstanding data quality issues or PIPs will also be reviewed and addressed.

Once implemented, the CCWIS will be fully compliant with state and federal requirements, provide uniform and reliable information on children currently under HHS oversight or care, support HHS's service delivery organization, and support all associated day-to-day case management activities.

7.0 Project Management Plan

Project management requires not only building a schedule, assigning tasks, and monitoring progress, but also looking at a challenge, understanding the goals and objectives, assessing the resources available, plotting a course of action, and keeping tasks on track from start through completion. The Iowa HHS Project Management Plan (PMP®) summarizes the approach to managing project activities, deliverables, products, project organization, State and contractor resource needs, and anticipated system life. The PMP® also describes how and when the project activities will be conducted and outlines the associated project documentation and contractor deliverables.

7.1 Project Organization

HHS is committed to the successful development and implementation of a CCWIS. Executive level leadership, as well as the leadership from the steering committee and project team, have been involved in planning efforts and will continue to be involved in ongoing planning and development.

Executive and Steering Committees

The executive steering committee included in **Table 7-1** meets quarterly or more frequently as needed, making executive level decisions regarding project direction, approach, and funding. **Table 7-2** lists the steering committee, which meets monthly or more frequently as needed to make leadership decisions on project resources, timelines, and risk and issue management, and to oversee the project team.

Table 7-1: Executive Steering Committee

Organization	Name	Title	Role
Iowa HHS	Larry Johnson	Director of Health and Human Services	Executive Leader
Iowa HHS	Sarah Reisetter	Deputy Director of Health and Human Services	Executive Leader
Iowa HHS	Janee Harvey	Division Director	Executive Sponsor
Iowa HHS	Matt Haynes	Bureau Chief	Lead Business Owner
Iowa HHS	Julia Webb	Bureau Chief	Advisor
Google	Krystal Van Der Brink	Project Manager	HHS-Side Project Manager

Organization	Name	Title	Role
Contractor	TBD	Project Manager	DOM DoIT-Side Project Manager
Contractor	Edward Young	Architect	Project Technical Architect
Iowa DOM DoIT	Steve Kinney	Information Technology Administrator 4	Advisor

Table 7-2: Steering Committee

Organization	Name	Title	Role
Iowa HHS	Janee Harvey	Division Director	Executive Sponsor
Iowa HHS	Matt Haynes	Bureau Chief	Lead Business Owner
Iowa HHS	Julia Webb	Bureau Chief	Advisor
Iowa HHS	Mike Kitzman	Executive Officer 3	Advisor
Google	Krystal Van Der Brink	Project Manager	HHS-Side Project Manager
Contractor	TBD	Project Manager	DOM DoIT-Side Project Manager
Contractor	Edward Young	Architect	Project Technical Architect
Iowa DOM DoIT	Bob Norris	Information Technology Enterprise Expert	Product Owner
Iowa DOM DoIT	Steve Kinney	Information Technology Administrator 4	Advisor

Project Team

The project team meets daily to coordinate and review ongoing project activities and deliverables, ensuring deliverables remain on schedule. The team is responsible for identifying, documenting, and escalating project risks and issues, as well as decisions made by leadership. **Table 7-3** includes the project team members.

Table 7-3: Project Team

Organization	Name	Title	Role
Iowa HHS	Matt Haynes	Bureau Chief	Lead Business Owner
Iowa HHS	Mindi Jones	Social Worker 6	Co-Business Owner
Iowa HHS	Lisa Hamar	Social Worker 6	Co-Business Owner
Iowa HHS	Lara Riniker	Social Worker 2	Dedicated Practice SME
Iowa HHS	Ryan Foy	Social Worker 2	Dedicated Practice SME
Google	Krystal Van Der Brink	Project Manager	HHS-Side Project Manager
Iowa DOM DoIT	TBD	Project Manager	DOM DoIT-Side Project Manager
Iowa DOM DoIT	Edward Young	Architect	Project Technical Architect

7.2 Other Agency and Contractor Resource Needs

Dedicated state contractor resources playing crucial roles on the project are as follows:

Table 7-4: Other Agency and Contractor Resource Needs

Organization	Name	Role	Allocation
Google	Krystal Van Der Brink	HHS-Side Project Manager	70%
Iowa DOM DoIT	TBD	DOM DoIT-Side Project Manager	100%
Iowa DOM DoIT	Edward Young	Project Technical Architect	100%
Iowa DOM DoIT	Bob Norris	Product Owner	10%

Iowa will leverage the expertise of additional DOM DoIT, fiscal, procurement, and contracting staff as needed.

7.3 Project Procurement Activities

In January 2025, HHS contracted with BerryDunn through an existing master agreement. As discussed earlier in Section 5, HHS decided to pursue a COTS CCWIS solution and to cease further investment in a customized, in-house developed solution. To support this effort, HHS secured planning and procurement support services from BerryDunn, including:

- Assessing existing business requirements and identifying new requirements against ACF requirements
- Developing an RFP to gain an understanding of available CCWIS solutions in the marketplace
- Assisting HHS in the development of CCWIS vendor selection criteria

The goal of these services was to provide HHS with a clear understanding of the requirements needed for its CCWIS solution to be compliant with ACF requirements, identify available CCWIS solutions in the marketplace, and help HHS determine the CCWIS vendor(s) that best fulfill its needs and requirements.

In mid-2025, leaders from Google and Iowa discussed how Google might assist with its CCWIS project, given its involvement in other contracted projects.

In August 2025, the VISION Team helped Google's Rapid Innovation Team (RIT) develop a prototype of the initial case setup in 3 weeks, based on Iowa's detailed Lucid design work and limited JAD sessions.

The vast scope and detail of Iowa's Lucid design work and the Google prototype were presented in early September 2025 to members of Governor Reynolds' cabinet, HHS leadership, DOM DoIT leadership, ACF Principal Deputy Assistant Secretary Andrew Gradison, and several members of Mr. Gradison's staff. Iowa's VISION Team demonstrated it was ready to move into active development immediately, and Google demonstrated an uncanny ability to understand and develop functionality that surpassed Iowa's CCWIS expectations, coupling Iowa's design with spectacular AI assistant utility.

Given the incredible success of the joint HHS and Google demonstration, HHS and DOM DoIT representatives met with Google leadership and system development representatives in early November 2025 to answer questions and draft a plan to replace FACS by the end of 2026.

At the beginning of December 2025, HHS submitted the following documents for ACF review:

1. Sole Source Justification for Google
2. Procurement Justification for Use of Carahsoft Master Contract for CCWIS-Related Services
3. NASPO ValuePoint Participating Addendum (Carahsoft/Iowa)
4. Carahsoft Master Agreement
5. Statement of Work and Price Quote (Carahsoft/Google)

7.4 Planning Activities and Schedule

The following outlines project activities that occurred **July 1, 2024 – November 30, 2025**.

Functional Work (July 1, 2024 – November 30, 2025)

- Requested and attended presentations and demos from other states (Idaho, Maryland, Mississippi, Arkansas, and Utah) to hear critical lessons each learned during the development/implementation of their respective CCWIS systems.
- Continued efforts to establish Data Sharing Agreements (DSA)/MOUs with Iowa Judicial, Iowa DOE, Iowa Medicaid, Bureau of Child Care, Bureau of Child Support, Bureau of Immunization and Tuberculosis, and other entities, both within HHS and external to the agency.
- Assessed the need to develop an RFP for additional CCWIS vendor assistance, leading to a contract with BerryDunn.
- Developed clear, detailed business requirements and business process flows for the desired CCWIS, beginning to end, to serve as the basis for an RFP. A “case management pathway/workflow” focus structured this work. Though HHS ultimately decided against issuing an RFP, all documentation proved useful to Google in strategizing its approach to replacing and decommissioning FACS.
- Developed in Lucid, beginning to end, the level of system detail one arrives to at the end of months of Sprints and JAD sessions, at which point developers take the content and start system development. All system data fields have been identified for every screen, including all drop-down options and validations, the associated workflow, and all click progressions.
- Helped to ensure all required AFCARS, FFPSA, and ICWA reporting data elements were accounted for and mapped in the Lucid design documents.
- Helped to ensure all needed FACS data elements were accounted for and mapped in the Lucid design documents.
- Helped to ensure unnecessary or antiquated FACS data elements were not included in the Lucid design documents.
- Assisted Google’s Rapid Innovation Team (RIT) with developing a prototype of the initial case setup in three (3) weeks based on Iowa’s detailed Lucid design work and limited JAD sessions.
- The vast scope and detail of Iowa’s Lucid design work and the Google prototype were presented in early September 2025 to members of Governor Reynolds’ cabinet, HHS leadership, DOM DoIT leadership, ACF Principal Deputy Assistant Secretary Andrew Gradison, and a number of Mr. Gardison’s staff. Iowa’s VISION Team demonstrated it was ready to move into active development immediately, and Google demonstrated an

uncanny ability to understand and develop functionality that surpassed Iowa's CCWIS expectations, coupling Iowa's design with spectacular AI assistant utility.

- Given the incredible success of the joint HHS and Google demonstration, HHS and DOM DoIT representatives met with Google leadership and system development representatives in early November 2025 to answer questions and draft a plan to replace FACS by the end of 2026.

Technical Work (July 1, 2024 – May 31, 2025)

VISION Next Plan

- Developed overall technical strategy for VISION approach
- Presented at multiple steering meetings, guiding various technical options:
    - Build vs Buy
    - Configuration vs Customization
    - The vendor selection process

Reference Architecture

Developed the following:

- VISION Architecture Principles
- VISION Business Architecture
- VISION Application Architecture
- VISION Security Architecture
- VISION Data Lake Architecture
- VISION Analytics Framework
- Child Protection Services Analytics

Data Architecture

Developed the following:

- Enterprise Data and Business Intelligence Strategy-v3
- VUDP Retention and Archiving Strategy
- VISION Unified Data Encryption Policy
- VISION Unified Data Classification Policy
- VUDP Architecture Principles-v1.0
- VUDP Data Governance Policy and Plan

- VUDP AI Enhanced Data Governance
- VUDP Data Governance Alignment Matrix
- VUDP Data Quality Management
- VISION Unified Data and Regulatory Compliance Policy
- VISION Data Management Framework

Unified Data Platform

Developed the following:

- High-level Technology Approach for the Unified Data Platform
- Staffing plan proposing resource allocation for planning and design for VISION data strategy
- Comprehensive list of downstream FACS reports and data extracts

ACF Design Review

- Compiled, updated, and shared VISION documentation with ACF Architects
- Attended design review meetings/sessions with ACF Architects

Security Architecture

- Developed HHS Security Architecture VISION v3.0

ACF Documentation

- Developed and submitted Data Quality Plan (DQP)

Data Strategy Planning and Design (April 1, 2025 – November 30, 2025)

By the end of 2025, the goal is to have the Unified Data Platform planned and designed, and to determine who will be responsible for installing, configuring, developing, and providing ongoing support for the cloud infrastructure. It is anticipated an external vendor will be utilized to complete these tasks. This approach is consistent with Iowa DOM DoIT's Cloud First Strategy and move away from doing in-house custom development work.

VISION Data Platform Cloud Infrastructure Design

Focus: Design Data Platform based on ACF and Iowa DOM DoIT requirements

Method: DOM DoIT

Time Frame: April 2025 – September 2025

Staffing: One data architect (20 hours/week), one cloud architect (20 hours/week)

Action Steps: Work with Enterprise Architecture (EA) to determine resource availability

VISION Report Migration Planning and Design

Focus: Implementation and Ongoing Support

Method: DOM DoIT, Business Owners, and Practice SMEs

Time Frame: April 2025 – September 2025

Staffing: FACS team (40 hours/week), DOM DoIT Information Management team (40 hours/week)

Action Steps: Work with Bob Norris and Satya Ponnada to determine resource availability

VISION Vendor Selection for Cloud Configuration of Data Platform

Focus: Determine resources/vendor for the implementation phase

Method: DOM DoIT and VISION Steering Committees

Time Frame: October 2025 – December 2025

Staffing: FACS team (40 hours/week), DOM DoIT Information Management team (40 hrs/week), data architect (20 hours/week), and cloud architect (20 hours/week)

VISION Cloud Architecture Implementation

Focus: Implementation and Ongoing Support

Method: Vendor

Time Frame: January 2026 – June 2026 (MVP Production Date)

Staffing: TBD, a deliverable based SOW

VUDP Implementation

Focus: Implementation and Ongoing Support

Method: DOM DoIT, Business Owners, and Practice SMEs

Time Frame: January 2026 – June 2026 (MVP Production Date)

Staffing: TBD, deliverable based on data platform design and support needs

VISION Quality Assurance (QA)

Focus: User Acceptance Testing (UAT) and Regression

Method: Business Owners and Practice SMEs

Time Frame: Approximately April 2026 and ongoing

Staffing: TBD, deliverable based on data platform design and support needs

RFP Development with BerryDunn (1/1/25 – 8/31/25)

The following project plan in **Table 7-5** outlines the schedule of RFP development activities completed with BerryDunn.

Table 7-5: RFP Development with BerryDunn (January 1, 2025 – August 31, 2025)

Task Name	Duration	Start Date	End Date
Phase 1: Project Planning	**48 days**	**1/22/25**	**3/28/25**
Conduct an initial planning call	1 day	1/22/25	1/22/25
Identify key stakeholders, develop the Stakeholder Registry, and submit to HHS for review	6 days	1/23/25	1/30/25
HHS to review and provide updates to the Stakeholder Registry	7 days	1/31/25	2/10/25
Conduct Stakeholder Kickoff Meeting	**4 days**	**1/23/25**	**1/28/25**
• Develop Kickoff Meeting materials, schedule, and prepare for facilitation	2 days	1/23/25	1/24/25
• Review and finalize Kickoff Meeting materials with HHS	1 day	1/27/25	1/27/25
• Facilitate Kickoff Meeting with HHS Stakeholders	1 day	1/28/25	1/28/25
Request background information	**31 days**	**1/29/25**	**3/12/25**
• Develop initial background request document	13 days	1/29/25	2/14/25
• Review request with HHS core project teams	2 days	2/17/25	2/18/25
• Update request and submit to HHS PM	1 day	2/19/25	2/19/25
• HHS to review the information request and provide the requested information	14 days	2/21/25	3/12/25
Develop Project Work Plan and Schedule	**43 days**	**1/29/25**	**3/28/25**
• Develop draft Project Work Plan and Schedule	13 days	1/29/25	2/14/25
• Review draft Project Workplan and Schedule with HHS	1 day	2/18/25	2/18/25
• Revise the Project Work Plan and Schedule based on feedback and submit to HHS	1 day	2/20/25	2/20/25
• HHS Review and feedback on Project Work Plan and Schedule	10 days	2/24/25	3/7/25
• Update Project Work Plan and Schedule based on feedback and submit to IA HHS for approval. HHS to sign the Deliverable Acceptance Form	15 days	3/10/25	3/28/25
D1. Project Work Plan and Schedule	**0 days**	**3/28/25**	**3/28/25**
M: Project Planning Complete	**1 day**	**3/28/25**	**3/28/25**
Phase 2: RTM Analysis and Development	**76 days**	**3/13/25**	**6/27/25**
Review background documentation submitted in response to the request (including the previous RFP)	10 days	3/13/25	3/26/25

Task Name	Duration	Start Date	End Date
Review ACF requirements	10 days	3/13/25	3/26/25
Review the current inventory of the State's systems in use	12 days	3/27/25	4/11/25
Review the current functional and technical requirements	22 days	3/13/25	4/11/25
Plan and conduct up to eight discovery sessions to inform requirements (1.5 hours each)	27 days	3/13/25	4/18/25
Develop preliminary requirements traceability matrix (RTM)	25 days	3/17/25	4/18/25
Review and provide initial feedback in preparation for the review sessions	5 days	4/21/25	4/25/25
Plan and conduct up to nine two-hour joint requirements review sessions to close gaps and update requirements	15 days	4/28/25	5/16/25
Update the RTM and submit the draft RTM to HHS for review and feedback	14 days	5/19/25	6/6/25
HHS to review RTM and provide feedback	10 days	6/9/25	6/20/25
Update the RTM based on feedback and submit the final RTM. HHS to sign the Deliverable Acceptance Form.	5 days	6/23/25	6/27/25
D2. Functional and Technical RTM	**0 days**	**6/27/25**	**6/27/25**
M: RTM Analysis and Development Complete	**0 days**	**6/30/25**	**6/30/25**
Phase 3: RFP Development and Support	**196 days**	**3/13/25**	**12/19/25**
Review RFP Template (if available) or proposed RFP format with IA HHS	3 days	3/13/25	3/17/25
Conduct an RFP planning session with IA HHS to determine where and how the RFP will be posted, determine the goals of the RFI, and answer the questions the State would like to have answered	1 day	3/14/25	3/14/25
Develop the draft Statement of Work	40 days	3/17/25	5/9/25
Facilitate meetings with the State to verify vendor implementation requirements, M&O requirements, deliverables, acceptance criteria, and SLAs	29 days	4/1/25	5/9/25
Develop the proposal form to be used by bidders in their response to the RFP	5 days	5/12/25	5/16/25
Develop the cost worksheet and corresponding narrative (if applicable)	5 days	5/19/25	5/23/25
Update the CCWIS RFP package and the Statement of Work based on feedback	14 days	5/12/25	5/30/25
Review the RFP with HHS for feedback	5 days	6/2/25	6/6/25
HHS to review the RFP	25 days	6/9/25	7/14/25
Finalize the RFP, HHS to sign the Deliverable Acceptance Form	5 days	6/23/25	7/17/25

Task Name	Duration	Start Date	End Date
D3. Iowa HHS CCWIS RFP	**25 days**	**6/27/25**	**7/21/25**

BerryDunn provided the following deliverables to HHS by the end of August 2025:

1. RFP-185-2516-2025 CCWIS Implementation and Operations
2. Scope of Work
3. RFP Response Narrative
4. RTM
5. Business Process Diagrams and Descriptions
6. Cost Proposal Template
7. Exceptions to RFP and Contract Language Form
8. System Interfaces
9. Data Flow Diagram
10. Current State Architecture
11. Future State Architecture

7.5 Resource Statement

7.5.1 State Staff Roles and Description

HHS will include four staff members who are 100% allocated to the VISION Project. Figure 6.2 illustrates the state staff roles for this project.

Table 7-6: State Staff Roles

Organization	Name	Role	Salary/Cost	Allocation
HHS	Lisa Hamar	Dedicated Practice SME	$106,850	100%
HHS	Mindi Jones	Dedicated Practice SME	$86,830	100%
HHS	Ryan Foy	Dedicated Practice SME	$63,780	100%
HHS	Lara Riniker	Dedicated Practice SME	$80,960	100%
DOM DoIT	1	DOM DoIT-Side Project Manager	$220,000	2000 Hrs
DOM DoIT	4	Business Analyst	$660,000	6000 Hrs
DOM DoIT	7	QA Tester	$924,000	8,400 Hrs
DOM DoIT	1	JARVIS Interface	$165,000	1,500 Hrs
DOM DoIT	2	JARVIS Data Conversion	$165,000	1,500 Hrs

DOM DoIT	2	JARVIS SME	$220,000	2,000 Hrs
DOM DoIT	3	FACS Data Conversion	$330,000	3,000 Hrs
DOM DoIT	3	FACS SME	$220,000	2,000 Hrs
DOM DoIT	1	IABC SME	$22,000	200 Hrs
DOM DoIT	2	Data Team	$220,000	2,000 Hrs
DOM DoIT	1	Data Architect	$220,000	2,000 Hrs
DOM DoIT	1	CDW	$125,000	500 Hrs

7.5.3 Contractor Staff Roles and Responsibilities

The number and types of staff Google will be utilizing on this project is not known at the time of the submittal of this IAPD. Pre-kick off coordination activities will begin the week of December 8, 2025, during which it is anticipated this information will be covered.

7.6 Scope, Methodology, and Deliverables

This section outlines the scope and methodology of the CCWIS project. The HHS project management bureau follows the Project Management Institute's® (PMI®) Project Management Body of Knowledge® (PMBOK®). Any selected contractors for this project will be required to adhere to the same PMBOK® methodology as HHS. The systems development methodology will be Agile to support incremental design, testing, and validation. Significant work will be sequenced to help ensure progression from validated requirements to testing and deployment.

HHS will work to ensure all project participants are clear on basic project management procedures and adhere to PMBOK® best practices. **Project scope, methods, and deliverables are aligned with the proposed Google contract.**

7.6.1 Project Scope

Google Public Sector Services (GPSS) Statement of Work

HHS and Google Public Sector Services (GPSS or Google) developed the scope with the proposed contract Statement of Work to execute the critical modernization of its CCWIS. This initiative replaces the legacy FACS mainframe and JARVIS application with a modular, cloud-native system designated as "VISION." This SOW governs the first 12 months of the project, focusing strictly on the "Initial Case Setup" and "Core Case Management" modules. This will encompass producing a "single source of truth" for case information and achieving operational readiness within the fixed timeline through rigorous scope containment and governance.

Together, HHS and Google developed the proposed project scope as stated in the contract as **Workstream 2.1. The Services under Workstream 2.1 are governed by the Implementation Services Schedule, which is incorporated into the contract.**

Section 8 includes the project schedule for each workstream included in the proposed Google contract. **Workstream 2.1 includes the following sub-workstreams as negotiated with Iowa:**

- Workstream 0: Project Oversight and Governance
- Workstream 1: System Development
- Workstream 2: Data Platform
- Workstream 3: Training Enablement
- Workstream 4: Transition to Managed Services Provider

Workstream 0: Project Oversight and Governance

Strategic Program Enablement:

- Engage with key program stakeholders to define goals, gather requirements, and establish expected outcomes of the program reflected in a high-level plan of deployment activities to be completed based on Google's leading practice methodology
- Host a pre-project kick off meeting with the customer to determine if the prerequisites have been met, set expectations, and discuss project execution strategy
- Provide status reports
- External reporting:
    - Provide necessary information and artifacts related to the Google scope of work to support the Customer in its development and submission of required advance planning document (APD) updates

Establish the Program Governance Office (PGO):

- Outline the purpose of the Program Governance Office (PGO), how it relates to other bodies at the customer, and the extent of governance authority
- Document the role and responsibilities of the PGO, including, but not limited to:
    - Change Control Board
    - Standardized communication methods
    - Backlog curation and approval of scope modifications
- Define roles, responsibilities, and accountability across workstreams and decide how they will interact with the PGO
- Provide communication requirements to be used as a guide for teams who will need to escalate requests through the PGO and how decisions will be communicated through the PGO

Change Control Board:

- Empower customer stakeholders to manage collisions between various workflows
- Provision and manage a product backlog and approve or deny scope changes within 48 business hours

- Maintain a formal "Not Now" list for this 12-month release to record desired future features, functions, or requirements that are out of scope for the current SOW; this mechanism supports transparent backlog management and validates joint alignment with the State
- Engage with the customer to prioritize the capabilities/features within the scope of this project.

Workstream 1: System Development

Business Context and Requirement Knowledge Transfer:

- Conduct stakeholder group sessions (as required by Google) to conduct knowledge transfer of the existing documentation
- Identify business workarounds currently dependent on application or off-system processes
- Use tooling to extract business rules, logic flows, and calculations from legacy code

User Interface Design:

- Conduct review meetings with stakeholders to existing designs and determine remaining refinement
- Finalize up to 40 existing customer webpage/tab designs and/or UI/UX workflows, as identified in the business process documentation, by finishing design details, minor adjustments, or adherence to brand guidelines
- Conduct sessions to gather input from stakeholders, caseworkers, and supervisors, optimizing business processes where possible to inform the final design
- Apply human-centered design principles to layout, navigation, and interaction models
- Provide mobile-responsive designs that incorporate Section 508 accessibility tenets to support broad accessibility and flexibility
- Provide a material UI component library of design elements
- Provide UI workflows based on business process maps and user feedback; verify designs are mobile-responsive

Application Implementation:

- Integration Component Development:
    - Build and configure the ingestion/export pipelines specific to Core Case Management
    - Provision the APIs, data feeds, or event-driven components required for Core Case Management
    - Provide new translation or mapping layers needed to reconcile the Core Case Management data model with legacy systems.

- Design and write the initial case setup and Core Case Management functionality using cloud-native services, covering the following In-Scope Workflows, Tabs, Widgets and Dashboards:
    - Person Search and Profile:
        - Person Search interface
        - Person Details widget
        - Person Profile
        - Person-Level Electronic Case File
    - Case Setup Workflow:
        - Case Overview (central case hub)
        - Associations Workflow Step
            - Person-to-case linking, participant list
        - Services Workflow Step
            - Record Out-of-Home Living Arrangements and essential services
        - Checklist Workflow Step
        - Approval Workflow Step
        - Social Worker Case Manager (SWCM) Checklist Workflow Step
        - Workflow Progress Bar overlay
        - Living Arrangements
    - Electronic Case File:
        - Electronic Case File (ECF)
        - Case-Level ECF
        - Upload Document Widget (accessible throughout VISION)
    - Worker Safety:
        - Abuse and Criminal History
    - Minimum Court Development:
        - Court (Case Level)
        - Person-Level Court
    - Child Protection Worker (CPW) Dashboard:
        - All Cases

        - Draft Cases
        - Pending Assignment
        - Pending Transfer
    - SWCM Dashboard:
        - Active Cases
        - Pending
        - Pending
        - Active
        - Transferred
        - Shared With
- Analytical and Generative AI (GenAI) Capabilities:
    - Implement GenAI capability to read narrative reports, and extract key risk indicators (e.g., “drug use,” “unattended child,” “physical injury”).
    - Implement the ability for the system to identify why certain words or patterns triggered risk signals (for explainability).
    - Define three GenAI capabilities to implement for case management such as:
        - Draft case notes, court summaries, or service referrals using structured case data.
    - Implement defined GenAI capabilities into Case Management workflows
    - Implement speech-to-text and summarization for dictating notes into structured entries
    - Implement document parsing capabilities
- Application Testing & Validation:
    - Provide an automated (where possible) test suite covering unit, integration, and functional test cases
    - Align test scenarios with traceable requirements verified against Business Process and Business Logic
    - Execute continuous testing within the CI/CD pipeline
    - Collaborate with QA team and UAT testers to validate functional and non-functional behaviors
    - Conduct initial performance profiling to establish baseline performance benchmarks and monitor thresholds for key user flows

- Observability and Service Level Agreement (SLA) Monitoring:
    - Set up dashboards to monitor integration health (e.g., throughput, error rate, latency)
    - Define alerting thresholds for service-level breaches or data sync anomalies
    - Enable audit-level tracing for compliance and forensics

Future Module Roadmap Development:

- Prepare a roadmap for the remaining case management business process workflows and integrations that are not part of the critical path
- Prepare a roadmap for modernizing the remaining CCWIS modules:
    - Use tooling to extract business rules, logic flows, and calculations from legacy code
    - Provide a business rules catalog with rule descriptions, code references, and triggering conditions
    - Segment the remaining module capabilities based on business capability and technical cohesion
    - Provide recommended sequencing based on priority, complexity, and risk
    - Engage with the customer to prioritize the capabilities and develop a notional release plan based on the discovery and design activities

Cloud Foundation Deployment

- Conduct three – six remotely-hosted, two-hour sessions, as determined by Google, to collect and review the technical requirements and objectives. This activity will cover the following areas:
    - Organization structure and resource hierarchy:
        - Define requirements for organizational hierarchy, including folders, projects, and organizational policies
        - Define requirements for naming conventions and labeling approaches for folders, projects, and resources
        - Define requirements for access and security controls for folders, projects, and resources
        - Define requirements for cost management model, including spend visibility and budget alerting
    - User and access management:
        - Identify the process and requirements for user authentication and authorization and/or Single Sign-On (SSO) federation to an Identity Provider (IdP)

        - Define conventions for users, groups, and service accounts roles in Google Cloud Platform (GCP) and the policies associated with these roles
    - Technical Infrastructure:
        - Define Virtual Private Cloud (VPC) requirements
        - Define specific roles for specific service administration (storage, identity, etc.)
        - Define any network connectivity requirements (site-to-site VPN, cloud interconnect)
    - Infrastructure-as-Code (IaC):
        - Define Git-based repository for storing and managing Infrastructure-As-Code (IaC)
    - Logging:
        - Define a framework to integrate Google Cloud logs to the customer's Security Incident Event Management System (SIEM)
    - Migration:
        - Review current storage assets, analyze data volume, access patterns, and performance requirements to define migration patterns and strategies
    - Assured Workloads:
        - Review customer requirements for the advanced security features to store or process regulated or sensitive data (e.g., HIPAA, FedRAMP, ICWA, CJIS)

Design:

- Provide a solution that meets customer requirements collected in the assessment phase and provide leading practices, recommendations, and design guidance
- Provide a Technical Design Document (TDD) draft that describes the future state and design details in the following areas:
    - Organization structure and access:
        - Design organizational hierarchy, user access, and roles
    - Network:
        - Design initial network topology to facilitate connectivity to service applications that will be deployed in Customer's GCP, on-premises, and/or other cloud environments
        - Design network security controls (firewall configuration)

- Storage:
  - Identify policy needs for areas including storage, data storage, data residency
  - Design for backup, recovery, disaster recovery, high availability
  - Provide guidance on data governance, data management, data life cycle management, data ingestion, data processing, naming conventions, environments, data sharing and federation
  - Provide guidance on data security and encryption policies
- Security:
  - [redacted]
- Billing and Cost Control:
  - Validate billing account and billing administrator roles are correctly configured
- Logging/Monitoring/Alerting:
  - Define resource and audit logging tooling and processes for accessing relevant logs
- Automation:
  - Recommend automation strategy for repeatable deployments and management of the GCP environment
- Disaster Recovery:
  - Design for failure based on customer defined recovery time objective (RTO) and recovery point objective (RPO) requirements
- High Availability:
  - Design for customer high availability requirements
- Assured Workloads:

- [redacted]

Implementation Support:

- Infrastructure Deployment:
  - Implement IaC to deploy all infrastructure resources
- User and Access Management:
  - Integrate identity provider according to the design specifications and conduct functional testing
- Logging and Monitoring:
  - Activate Cloud Logging for all relevant GCP projects and services
  - Define log retention policies
  - Activate Cloud Monitoring for all relevant GCP projects and services
  - Define alerting policies and configure notification channels
  - Implement Google Cloud log shipment to the Customer's SIEM
- Security:
  - [redacted]
- Test all delivered resources, including:
  - Conduct testing to validate the functionality of the logging and monitoring setup

- Trigger test alerts to verify notification channels and response procedures
- Simulate load and stress conditions to validate that the logging and monitoring system can handle production volumes

Standard Implementation Support:

- Guide the customer's technical team for the customer's design and implementation of any SSO federation requirements.
- Implement and validate the following:
  - IaC source repository and CI/CD using Cloud Build
  - Root GCP organization and the organization administrator access and roles
  - Billing account, payment profile, and billing administrator access
  - Folder and project hierarchical structure and labels according to the defined labeling convention
  - Initial user groups in Cloud Identity and user groups role assignment in Identity and Access Management (IAM) according to the defined user access management convention
  - Access control for the foundational resource hierarchy
  - Network topology including VPC (single or shared mode) and network security controls (firewalls, routing policies)
  - Cost management and tracking model
  - Cloud Operations Suite project for Cloud Monitoring and Cloud Logging
  - SCC Standard (SCC Premium or Enterprise may be substituted if customer has purchased the required SKUs)

Integration Services Design:

- Define source to target mapping for VISION Integrations
- Evaluate and design data migration process to migrate data FACS to VISION
- Evaluate requirements for receiving data from JARVIS to VISION for specified tables
- Define integration patterns (real-time API vs. batch) for required interfaces supporting the case management application
- Design API contracts and endpoints for synchronous data (table) exchanges
- Design batch (table) integrations from required systems

- Validate integration availability and usability
- Define integration boundaries and service interfaces for mainframe-to-cloud system interactions
- Generate early-stage infrastructure and integration service recommendations

Implementation:

- Build and configure up to 28 required integrations, as requested by customer, to VISION (e.g., overnight case updates), including necessary data transformations.
- Expose up to five, as determined by Google, integrations that make VISION data available to other internal systems.
- Implement retry logic, error capture for failed exports.
- Integrations:
    - Person Lookup and State ID Management:
        - Implement API client wrapper for Iowa Automated Benefit Calculation System (IABC) person search and State ID operations
        - Implement duplicate prevention logic using search results to identify correct identities
        - Provision data transformation layer mapping VISION person model to/from IABC formats
    - Global360 Document Retrieval:
        - Implement an API client for document search and retrieval for files
    - FACS Data Loads:
        - Implement FACS to VISION data integration for required tables
    - JARVIS ODS:
        - Implement integration for the required JARVIS ODS data to VISION
    - JARVIS Magic Button Draft Case Creation:
        - Provision pipeline extracting completed incident data from JARVIS ODS
- Implement integrations or read-only views to utilize required data from out-of-scope systems or functionality:
    - Living Arrangements:
        - Configure read-only provider lookup from FACS I3 for licensed home selection

        - Provide Read, Update Delete operations for Living Arrangement type, start/end dates, AFCARS minimum data elements
    - Associations:
        - Leverage existing JARVIS Magic Button to auto-pull case members/associations
        - Leverage Person Search for duplicate prevention when adding associations
    - Subsidized Guardianship:
        - Read-only display of existing subsidy payment history from FACS mainframe

Workstream 2: Data Platform

App Deployment, Data Deployment, CI/CD:

- Set up CI/CD pipelines to support automated deployment, testing, and policy validation across environments
- CI/CD pipeline(s) will implement the following:
    - Artifact repository for pipeline templates and scripts
    - Provision artifacts (containers/functions) and push to the registry with versioning
    - Utilize secrets management for credentials and API keys
    - Multi-stage pipeline with approval gates and additional approvals for Prod deployment
- Implement logging patterns and monitoring configurations for pipeline runs

Data Strategy and Parallel Migration:

- Data Profiling
    - Conduct four, 60-minute meetings to review existing Data Dictionary documentation and source-to-target mapping documents
    - Validate or conduct script-based profiling of source tables, gathering volume, size and shape data to inform the target data model
    - Perform a Data Loss Prevention review of required data to identify potential PII/PHI
- Data Model Discovery and Mapping

- Use AI-driven assessment tools to scan and inventory legacy code artifacts (COBOL, JCL, DDL) to extract business rules and data structures
- Confirm design for VUDP
- Design data model including core entities, relationships, constraints, and dependencies relevant to child welfare domains:
  - [redacted]
  - [redacted]
- Provide field-level mappings from source to target schema
- Provide Entity-Relationship Diagrams (ERDs) and mapping diagrams
- Provide a physical data model

- Data Pipeline
  - Provision reusable pipeline templates with error handling and logging
  - Produce up to 50 data pipelines, as determined by Google, for ingestion and transformation datasets
  - Implement transformations and business logic
  - Utilize code conversation techniques to convert legacy business logic to modern language
  - Integrate automated data quality checks (up to 10 checks per pipeline, as determined by Google)

  Data Validation

  - Define 50 data validation rules for ongoing incoming and outgoing data
  - Implement framework for automated data validation and reconciliation
  - Establish automated quality checks for completeness, accuracy, consistency, and timeliness, per pipeline
  - Validate data replication and completeness in target environments

- Validate end-to-end data flows

Data Migration

- Provide a migration plan with dependencies, milestones, and critical path and rollback plan
- Provide a data migration validation process and associated scripts to confirm completeness and accuracy
- Perform ongoing data integration in lower environments during development
- Perform one pre-production test data migration in a lower environment
- Execute the final data migration cutover process during the go-live event
- Perform data validation on data migrated to VISION

Virtual Data Platform

- Provide 20 database views from the VUDP available for usage by other internal systems or processes.

Workstream 3: Training Enablement

Enablement Artifacts

- Provide training documentation aligned with the rollout roadmap of the system functionality and roles.
- Produce end-user enablement materials (written guides, video tutorials, quick reference cards, etc.).
- Up to three written user guides as required by the solution.
- Up to five single-page job aids (for specific processes/focus areas) as required by the solution.
- Up to five video tutorials (one – two minutes each) as required by the solution
- One set of frequently asked questions (FAQs), organized for easy end user reference
- Additional materials to be determined based on end user need and team capacity
- Conduct either (a) five or fewer initial enablement (train-the-trainer) sessions each not to exceed two hours in duration OR two initial

enablement (train-the-trainer) sessions, each not to exceed four hours in duration (as determined by customer needs and logistics).

Hypercare Support (post-production):

- Provide a production rollout deployment plan
- Prepare the production environment
- Setup logging and monitoring for prod environment
- Provide advisory guidance to the customer during rollout to the new production environment
- Monitor the application for up to four weeks, as determined by Google, and provide troubleshooting/break-fix support
- Prepare a handover report, which may include recommendations on future enhancements and changes

Workstream 4: Transition to Managed Services Provider (prior to the conclusion of engagement):

- Conduct one, 90-minute configuration workshop to provide guidance on the following topics:
  - Configuring IAM access for SCC
  - Enabling Security Sources for SCC
  - Identifying required audit logs for Security Sources
  - Identifying additional views of SCC to be implemented (e.g., organizational, folder, project)
  - Mapping compliance frameworks to detectors
- Conduct one, 90-minute workshop on managing SCC findings. Topics include:
  - How to create, filter, and mute notifications from Event Threat Detection (ETD), Container Threat Detection (CTD), and VM threat Detection (VMTD)
  - How to create, filter, and mute notifications from Security Health Analytics (SHA)
  - How to create custom modules for Threats and Vulnerabilities
  - Conduct one, 90-minute workshop on setting notifications and reviewing Security Command Center (SCC) dashboard. Topics include:
    - How to configure notifications with external destinations
    - How to export and analyze SCC findings into BigQuery

- Conduct one, 90-minute interactive session on reviewing SCC finding Playbooks Topics include:
    - Responding to credential (user account) compromise from an ETD alert
    - Responding to computing engine compromise from a VM Threat Detection alert
    - Responding to GKE container compromise from a Kubernetes Threat Detection alert
    - Schedule and conduct one remotely hosted, two-hour transition coordination meeting
- Conduct two remotely hosted, two-hour knowledge transfer meetings scheduled within 10 business days of each other
    - Answer any outstanding questions or concerns within two business days following each transition meeting
    - Review Terraform code samples (as described in the SOW deliverables)
    - Walk through the Technical Design Document (TDD) during one remotely hosted, two-hour meeting

HHS has developed a list of primary expectations and goals as part of DDI of the CCWIS. **These goals include the following:**

- **Intuitive Workflow**: The new system will offer a streamlined entry sequence with system prompts notifying the worker if information is missing or appears incorrect. The system will guide field staff through the life of a case, mirroring child welfare best practices.
- **Ease of Data Entry**: The new system will eliminate the reliance on memorization and repetition by replacing the code-heavy structure. It will also provide clarity on confusing terms or policies, encouraging a more standardized entry of information and increased data quality.
- **Decision Support**: The new system will be built on business rules and alerts developed from child welfare practices. These rules and alerts will help guide social workers through a case by providing workflow prompts.
- **Personalized Dashboard and Reporting**: The new system will provide users of the application with a customized dashboard based on their responsibilities and needs. The dashboard will help field staff view their caseload, allowing them to see what tasks are upcoming and will offer more consistent reporting across HHS. Leadership, policy, and other stakeholders will have on-demand access to data.
- **Document Management**: The new system will support the use of electronic forms and will allow the electronic storage of documents within the case file. The system will also support the collection of electronic signatures.
- **Eliminate Duplication**: The new system will reuse and auto-populate information that has been previously entered to populate appropriate locations, documents, and forms. This will increase data quality as the opportunity for errors during entry is reduced.

- **Clear Relationship Connections and Mapping**: The new system will provide field staff and analysts with a clear picture of a family system and the other relationships that occur within a case. The ability to view a family system will help workers understand a family's history with HHS, identify potential kin or fictive kinship placements for children when needed, and properly report on individual or family outcomes.
- **Comprehensive Data Collection**: The new system will collect child welfare data in a more robust way through interfaces with external partners and a review of data reporting needs.
- **Real-Time Data Processing**: The new system will manage data in real-time whenever possible to eliminate lag time in communication between systems and in reporting.
- **Mobility**: The new system will offer mobile functionality, allowing field staff to view and edit information, manage electronic forms, and obtain electronic signatures when in the field.
- **Flexibility**: The new system will be flexible enough to support changes to child welfare practice and policy and any reporting requirements that may arise.

Along with the overarching goals above, the automated functions contained in a CCWIS will follow a modular design as stipulated in 45 CFR 1355.53. These functions will interface with one another and run seamlessly. The application will include data exchanges and/or portal interfaces with third parties such as the courts, contractors, youth and caregivers, Iowa Medicaid, Iowa DOE, and Public Health. HHS will review current reporting applications to determine if any may be absorbed into the new system and examine and revise processes as needed to help ensure all reporting requirements are met. Any outstanding data quality issues or PIPs will also be reviewed and addressed.

Once implemented, the CCWIS will be fully compliant with state and federal requirements, provide uniform and reliable information on children currently under HHS oversight or care, support HHS's service delivery organization, and support all associated day-to-day case management activities.

7.6.2 Project Methodology

The State's approach to management and operations emphasizes transparency, accountability, and effective oversight of vendor performance. The support model for HHS and DOM DoIT for this project will include:

- **Clear Project Governance**\ to align support across teams for seamless planning and implementation of the project.
- **Regular Monitoring and Reporting** of deliverables, performance measures, and compliance with contract requirements.
- **Structured Operations Transition Plan** to support eventual turnover to Iowa HHS or a subsequent vendor, ensuring long-term sustainability.
- **Phased Implementation Oversight** covering planning, execution, stabilization, and closeout, so that operations and maintenance are supported without disruption.

This approach ensures that ongoing operations are managed collaboratively, with HHS retaining final authority over decision-making and system direction.

7.6.3 Project Deliverables

Table 6-7 lists all Google CCWIS project deliverables identified to date. It is expected that the deliverables will be the responsibility of Google.

Table 7-7: Google CCWIS Project Deliverables

ID	Deliverable	Description/Acceptance Criteria
D1	Project Charter	A document containing goals, requirements, KPIs, major milestones, and governance procedures.
D2	Weekly Status Report	A document prepared weekly, outlining progress against project timelines, key risks and issues to be addressed (RADI log), decisions or change requests, and progress on milestones and KPIs.
D3	Project Plan	A document containing milestones, deployment activities, and completion dates of tasks and all specified deliverables.
D4	Product Roadmap	A strategic document that communicates the system vision and direction, focusing on outcomes, modernization recommendations, and goals.
D5	Technical Design Document (TDD)	A document containing a detailed design of the technical architecture within Google's scope of work, including an inventory of GCP configuration options, decisions, and recommendations for next steps for the architecture.
D6	Infrastructure-as-Code (IaC) Scripts	All source code and infrastructure-as-code were developed for the environment/solution.
D7	Technical Security Recommendations Document	A document that summarizes leading practices on Vulnerability Management, Ingress/Egress Security, Data Encryption, Access Controls, and Traceability Matrices.
D8	User Interface Designs	Figma user interface designs, User Workflows in Figma/PDF form, UI Component Library.
D9	Data Architecture Definition Document	A technical document defining the data architecture, data lineage, and strategy for the Virtual Data Warehouse.
D10	Code Extraction Output	Automated reports and data files generated by Google tooling, representing the business rules and logic extracted from legacy code.
D11	Virtual Data Warehouse Configuration	Configuration scripts and documentation establishing the Virtual Data Warehouse environment for In-Scope workflows.
D12	Integration Code (APIs/Batch)	The API definitions and batch processing scripts are required to support the integrations for In-Scope and Partial Scope workflows.
D13	Change Management & User Experience	User journey maps for Core Case Management workflows, Change impact assessment, Stakeholder engagement plan, and Communication templates.

ID	Deliverable	Description/Acceptance Criteria
	Deliverables (Documentation)	
D14	Initial Security Assessment Report	A preliminary report from Mandiant detailing the findings from the architecture review and initial penetration testing activities.
D15	Training Artifacts (Job Aids/User Guide)	A package containing the specific User Guide, Job Aids, and Video Tutorials defined in Workstream 4.
D16	Final Comprehensive Security Review Report	A final report from Mandiant detailing the comprehensive results of penetration testing, identified vulnerabilities, and remediation recommendations.
D17	Final Go-Live Report	A summary document detailing the deployment outcome, system status, and transition to Hypercare/Operations.

8.0 Proposed Project Schedule

Table 8-1 includes four phases of the proposed Google schedule by milestone, associated deliverables, and the percentage of total engagement of the scope of work. This schedule will be developed into a Project Plan (Deliverable Three) during Quarter 1 of the project. This work will take place over 12 months.

Table 8-1 Project Phase, Milestone, and Deliverables

Phase	Milestone	Associated Deliverables*	Percentage of Total Engagement
Phase 1 (Q1)	M1.1 Infrastructure & Test Planning	D1 Project Charter D2 Weekly Status Reports* D3 Project Plan* D4 Product Roadmap D5 Technical Design Document* D6 Infrastructure as Code (IaC) Scripts	9%
	M1.2 Design & Security	D7 Technical Security Recommendations Document* D8 User Interface Designs	9%
	M1.3 Data Platform Architecture	D9 Data Architecture Definition Document	5%
	M1.4 Rules Extraction	D10 Code Extraction Output	5%
Phase 2 (Q2)	M2.1 Application, Data & Enablement Artifacts	D11 Virtual Data Warehouse Configuration D12 Integration Code (APIs/Batch) D13 Change Management & User Experience Deliverables*	24%
Phase 3 (Q3)	M3.1 Security Assessment	D14 Initial Security Assessment Report	16%
	M3.2 Testing & Enablement	D15 Training Artifacts*	8%
Phase 4 (Q4)	M4.1 Security & Regression Completion	D6 Infrastructure as Code (IaC) Scripts D16 Final Comprehensive Security Review Report	16%
	M4.2 Final Report Out	D17 Final Go-Live Report	8%

9.0 Budget

9.1 Project Budget

Iowa has been utilizing both state resources and staff augmentation to complete planning and development activities. Costs related to the Executive Steering Committee and the Steering Committee are not charged to the project. DOM DoIT's state augmentation process includes a Master Contract with Computer Aid Inc. (CAI). Under this master contract, managed by the State's Office of the Chief Information Officer (OCIO), the DOM DoIT secures contractors by:

1. Providing CAI with details about the procurement:
    a. Level and type of experience needed (years of experience, experience with specific tools/software, etc.)
    b. Standard positions and rates established by CAI and the OCIO
2. CAI posts the procurement to the vendor community. For a staffing vendor to participate in the procurement process, it must be registered with CAI.
3. CAI reviews the resumes provided by the vendor community to help ensure candidates meet the needs of the DOM DoIT specified in the requisition document.
4. Candidates are presented to DOM DoIT to interview.

    DOM DoIT's hiring decision is passed to CAI who facilitates the engagement of the contractor.

Table 9-1 shows the budget and actual costs for the State's fiscal year 2025 (SFY2025),

Table 9-1: Budgeted and Actual Costs for July 1, 2024 – June 30, 2025

State Fiscal Year 2025 (SFY25)										
Budgeted and Actual Costs by Quarter	Q1 Jul - Sep		Q2 Oct - Dec		Q3 Jan - Mar		Q4 Apr - Jun		TOTAL COST SFY25	
All changes have been saved		get	Actual	Budget	Actual	Budget	Actual	Budget	Actual	Budget
State Staff Salary & Benefits	$ 23,658	$ 17,966	$ 28,569	$ 17,966	$ 26,724	$ 17,966	$ 31,787	$ 17,966	$ 110,738	$ 71,864
Operating Expenses	$ -	$ 750	$ 1,233	$ 750	$ -	$ 750	$ -	$ 750	$ 1,233	$ 3,000
Travel/Training	$ -	$ -	$ 195	$ 8,500	$ -	$ -	$ 1,085	$ -	$ 1,279	$ 8,500
Hardware/Software/ Licenses	$ 139,000	$ 26,481	$ 19,435	$ 1,140,479	$ 8,641	$ 17,655	$ 9,047	$ 12,813	$ 176,123	$ 1,197,427
SUBTOTAL	$ 162,658	$ 45,197	$ 49,431	$ 1,167,695	$ 35,365	$ 36,371	$ 41,919	$ 31,529	$ 289,373	$ 1,280,791
Central IT Services	$ -	$ -	$ -	$ -	$ -	$ -	$ -	$ -	$ -	$ -
Staff Augmentation Contracts	$ 30,975	$ 155,305	$ 93,269	$ 250,312	$ 136,383	$ 500,561	$ 383,470	$ 503,489	$ 644,097	$ 1,409,668
Administrative Overhead	$ -	$ 1,000	$ -	$ 1,000	$ -	$ 1,000	$ -	$ 1,000	$ -	$ 4,000
Total Cost	$ 193,634	$ 201,502	$ 142,700	$ 1,419,007	$ 171,749	$ 537,932	$ 425,389	$ 536,018	$933,470	$ 2,694,459
IV-E Eligible Share	$ 193,634	$ 201,502	$ 142,700	$ 1,419,007	$ 171,749	$ 537,932	$ 425,389	$ 536,018	$933,470	$ 2,694,459
Total State Share	$ 96,817	$ 100,751	$ 71,350	$ 709,504	$ 85,874	$ 268,966	$ 212,694	$ 268,009	$466,735	$ 1,347,229
Total Federal Financial	$ 96,817	$ 100,751	$ 71,350	$ 709,504	$ 85,874	$ 268,966	$ 212,694	$ 268,009	$466,735	$ 1,347,229

Table 9-2 shows the budget and actual costs for July 1, 2025 – June 30, 2026. The new solution costs included in the proposed Google contract begin in quarter three (Q3) of SFY26, as shown in the Hardware/Software/Licenses and Staff Augmentation Contracts table rows in Figure 9-2.

Table 9-2: Budgeted and Actual Costs for July 1, 2025 - June 30, 2026

State Fiscal Year 2026 (SFY26)										
Budgeted and Actual Costs by Quarter	Q1 Jul - Sep		Q2 Oct - Dec		Q3 Jan - Mar		Q4 Apr - Jun		TOTAL COST SFY26	
	Actual	Budget	Actual	Budget	Actual	Budget	Actual	Budget	Actual	Budget
State Staff Salary & Benefits	$ -	$ 32,000		$ 32,000		$ 957,500		$ 957,500	$ -	$ 1,979,000
Operating Expenses	$ -	$ 750		$ 750		$ 750		$ 750	$ -	$ 3,000
Travel/Training	$ -	$ -		$ 8,500		$ -		$ -	$ -	$ 8,500
Hardware/Software/ Licenses	$ 9,127	$ 10,000		$ 10,000		$ 295,000		$ 295,000	$ 9,127	$ 610,000
SUBTOTAL	$ 9,127	$ 42,750		$ 51,250		$ 1,253,250		$ 1,253,250	$ 9,127	$ 2,600,500
Central IT Services	$ 84,433	$ 85,000		$ 85,000		$ -		$ -	$ -	$ 170,000
Staff Augmentation Contracts	$ 46,000	$ 150,000		$ 150,000		$10,188,750		$10,188,750	$ 46,000	$ 20,677,500
Administrative Overhead	$ -	$ 1,000		$ 1,000		$ 1,000		$ 1,000	$ -	$ 4,000
Total Cost	$ 139,560	$ 278,750		$ 287,250		$11,443,000		$11,443,000	$55,127	$ 23,452,000
IV-E Eligible Share	$139,560	$ 278,750		$ 287,250		$11,443,000		$11,443,000	$55,127	$ 23,452,000
Total State Share	$69,780	$ 139,375		$ 143,625		$5,721,500		$5,721,500	$27,563	$ 11,726,000
Total Federal Financial	$ 69,780	$ 139,375		$ 143,625		$5,721,500		$5,721,500	$27,563	$ 11,726,000

Table 9-3 shows the budget and actual costs for July 1, 2026 – June 30, 2027. The new solution costs included in the proposed Google contract are included in quarters one and two (Q1 and Q2) of SFY27, as shown in the Hardware/Software/Licenses and Staff Augmentation Contracts table rows in Figure 9-2.

Table 9-3: Budgeted and Actual Costs for July 1, 2026 – June 30, 2027

State Fiscal Year 2027 (SFY27)										
Budgeted and Actual Costs by Quarter	Q1 Jul - Sep		Q2 Oct - Dec		Q3 Jan - Mar		Q4 Apr - Jun		TOTAL COST SFY27	
	Actual	Budget	Actual	Budget	Actual	Budget	Actual	Budget	Actual	Budget
State Staff Salary & Benefits		$ 957,500		$ 957,500					$ -	$ 1,915,000
Operating Expenses		$ 750		$ 750					$ -	$ 1,500
Travel/Training		$ -		$ 14,250					$ -	$ 14,250
Hardware/Software/ Licenses		$ 295,000		$ 295,000					$ -	$ 590,000
SUBTOTAL		$ 1,253,250		$ 1,267,500					$ -	$ 2,520,750
Central IT Services		$ -		$ -					$ -	$ -
Staff Augmentation Contracts		$ 10,188,750		$10,188,750					$ -	$ 20,377,500
Administrative Overhead		$ 1,000		$1,000					$ -	$ 2,000
Total Cost		$11,443,000		$11,457,250					$ -	$ 22,900,250
IV-E Eligible Share		$11,443,000		$11,457,250					$ -	$ 22,900,250
Total State Share		$5,721,500		$5,728,625					$ -	$ 11,450,125
Total Federal Financial		$5,721,500		$5,728,625					$ -	$ 11,450,125

9.2 Summary Budget

Year	Work Category	State Share	Federal Share	Total
Year 1 (2026)	DDI	$22,893,125	$22,893,125	$45,786,250
Year 2 (2027)	DDI	$11,000,000	$11,000,000	$22,000,000
Year 3 (2028)	DDI and M&O	$8,000,000	$8,000,000	$16,000,000
Year 4 (2029)	M&O	$2,500,000	$2,500,000	$5,000,000
Year 5 (2030)	M&O	$2,500,000	$2,500,000	$5,000,000
Year 6 (2031)	M&O	$2,500,000	$2,500,000	$5,000,000
Year 7 (2032)	M&O	$2,500,000	$2,500,000	$5,000,000
Year 8 (2033)	M&O	$2,500,000	$2,500,000	$5,000,000
		$54,393,125	**$54,393,125**	**$108,786,250**

9.3 Program Cost Distribution

As indicated under the Summary Budget section above, 50% of the project costs will be allocated to IV-E in the amount of $54,393,125, and $54,393,125 will be paid with General Fund appropriated dollars from HHS's Division of Family Well-Being and Protection budget.

Project Name	Task Code	State Share	Federal Share	Total
FACS/JARVIS Replacement - CCWIS (VISION) DDI	P130	$39,393,125	$39,393,125	$78,786,250
CCWIS System M&O	TBD	$15,000,000	$15,000,000	$30,000,000

The FFP for the IV-E distributed costs are 100% and will be fully applied to the $54,393,125 across an eight-year period. This distribution is reasonable as the program allows for 50/50 matching.

9.4 Cost Allocation Plan

In accordance with applicable federal regulations, HHS recognizes that all costs partially and fully associated with the development of our CCWIS must be appropriately allocated to each benefiting program.

45 CFR 1355.57 states that the design, development, installation, and operation of a system that complies with requirements can be charged to Title IV-E without regard to whether the system is used with respect to foster or adoptive children other than those on behalf of whom foster care maintenance or adoption assistance payments are made under Title IV-E. However, 45 CFR 1355.57 also states that cost allocation and cost distribution are required when the system includes functions, processing, information collections, management, equipment, or

services that are not directly related to the administration of programs carried out under the State Plan approved under Title IV-B or Title IV-E

The purpose of the new CCWIS is to support the child welfare needs of the state of Iowa. All of the functionality included in this project will support child welfare. HHS anticipates that a majority of the programs and services that will be automated within the new system are included in the federally approved State Plans for Titles IV-B and IV-E. Because this is only a projection, HHS will continue to examine and amend, if necessary, the actual cost allocation methodology as the CCWIS implementation progresses.

For this IAPD, the state of Iowa anticipates requesting regular federal financial participation (FFP) funding at a rate of 50 percent. Title IV-E is the primary benefiting program; therefore, HHS's cost allocation plan will be to charge Title IV-E for 50% of the costs, and the state will fund 50% of the costs through the use of general funds.

9.5 System Duration Statement

HHS will maintain VISION well into the foreseeable future. It will be developed using state-of-the-art technologies to meet HHS's specific case management practice and policy needs, and will be enhanced in the coming years via M&O as technology, practice, and policy changes necessitate. In the event VISION becomes technologically antiquated as FACS currently is, HHS will seek to replace VISION.

10.0 Contracts

At this time, it is unknown what vendor will be utilized for M&O services.

Table 10-1 provides the expected contracts that will be used between Google and HHS and the anticipated procurement strategy.

Table 10-1: Development Phase Contracts

Contract Name	Type	Procurement Strategy	Total Cost	Time Frame		
				Base Start	Base End	Option Years
Google-9A92-40AE-8D00	New - GCP	NASPO Master Contract	$1,140,583.70	1/1/26	12/31/26	0
Google-CP-PROF-SVC-CON	New - PSO	NASPO Master Contract	$40,755,000.00	1/1/26	12/31/26	0

11.0 Interface Requirements and Security

11.1 Interfaces

During requirements gathering and information sharing, HHS identified current and potential interfaces to the new CCWIS. The current interfaces that will also potentially occur in the new system will be listed in both tables. Also identified for each system is the type of interface, which includes inquiry services, batch processes, or manual processes. Inquiry services are real-time interfaces; batch processes involve exchanging a file and then feeding it into the system; and manual processes include entering data from the system into the database through manual look-up and entry or exchanging data between systems through manual entry.

Each interface planned for the new system will include an Interface Agreement Specification detailing the data to be exchanged and how it will be exchanged. The external interfaces will also include an MOU to solidify the agreement between HHS and partnering agencies. **Table 11-1** lists all current FACS and/or JARVIS interfaces that occur.

Table 11-1: Description of Current Internal and External Data Interfaces

System	Description	Internal/ External	Type of Interface
AFCARS	Collects case-level information from state and tribal Title IV-E agencies on all children in foster care and those who have been adopted with Title IV-E agency involvement	External	Batch
Central Print	Central printing and mailing system	External	Batch
Children's Justice	Weekly data file sent to the Courts	External	Batch
CJIS	System that uploads and downloads information to and from the Courts	External	Batch
Data Warehouse	Data analytics and reporting	Internal	Batch
Drug Testing	System automating the drug testing authorization process	External	Inquiry
Early ACCESS	Education and health information for children under the age of three who have a condition or disability that could cause growth or developmental delays	External	Inquiry
Electronic Invoices	Application where providers can submit invoices	Internal	Batch
EPS	Bridge system between HHS systems and Iowa Advantage.	Internal	Batch
FCRU	Exchanges financial data for Social Security (SSA) or Supplemental Security Income (SSI) benefits	External	Batch
FFPSA Prevention Plan Data Reporting	Reporting elements required for Prevention Plan data reporting.	External	Batch

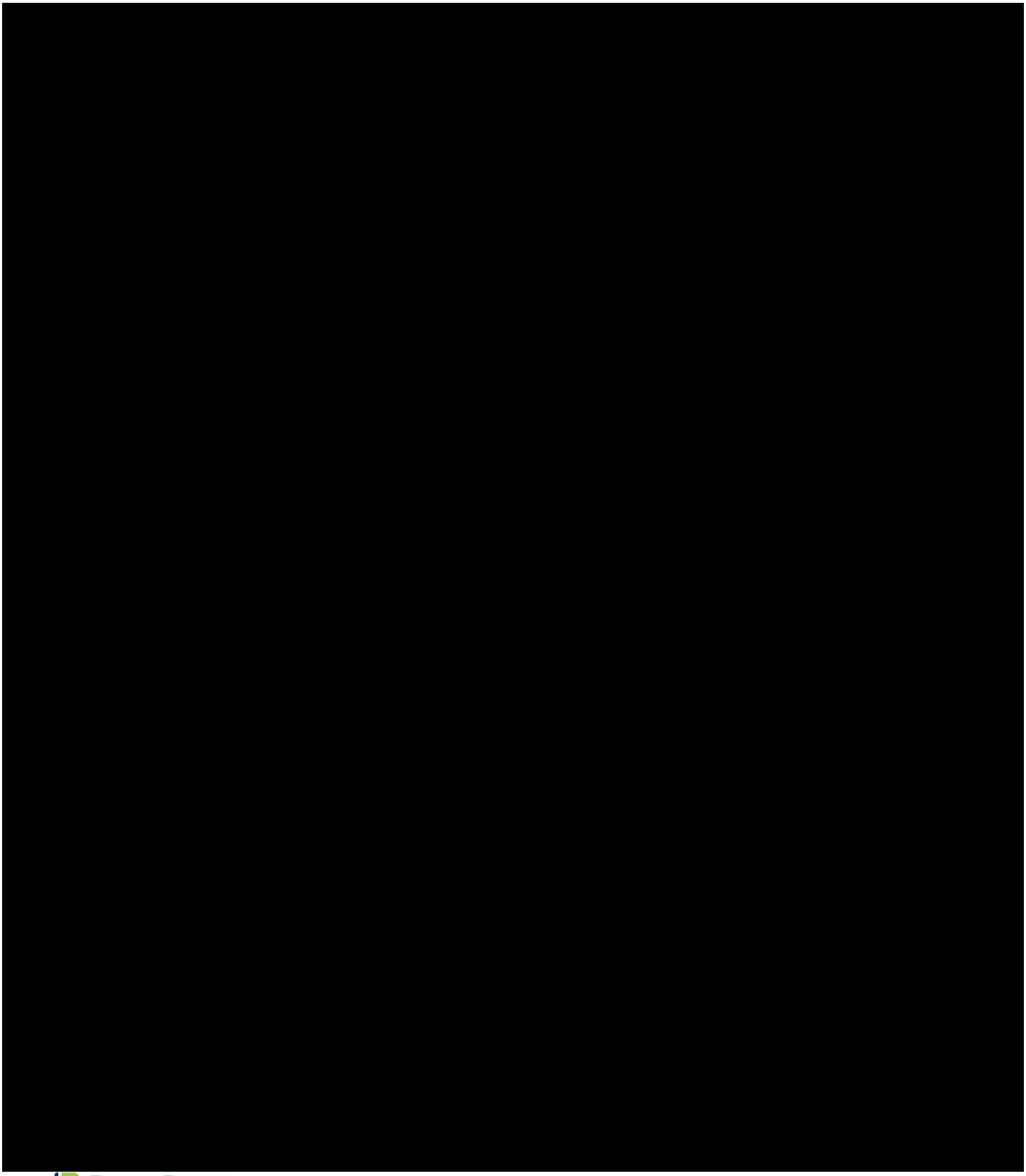
System	Description	Internal/ External	Type of Interface
Foster Care Youth Credit	Youth credit report information	Internal	Batch and Manual
Global 360	Electronic document storage	Internal	Inquiry
Iowa Advantage	State's financial system	External	Inquiry
Iowa Automated Benefits Calculation (ABC)	Establishes benefit eligibility and calculates benefit levels for Supplemental Nutrition Assistance Program (SNAP), Temporary Assistance for Needy Families (TANF), and Medicaid	Internal	Inquiry and Batch
Iowa Collections and Reporting (ICAR)	Establishes and enforces child and medical support orders, and processes support payments	Internal	Batch
IDOE	Education related information	External	Batch
Iowa Medicaid	Portal application which provides several services	Internal	Inquiry
JARVIS – DARES Modules	Dependent adult abuse evaluations	Internal	Inquiry
JARVIS – STAR Modules	Intake and assessment of child protective assessments including child abuse assessments, family assessments and CINA assessments. Child abuse registry.	Internal	Inquiry
JARVIS – Child Services Modules	Case Plans, Contact Notes, Relative Notices, Prevention Plans, Safe Plans of Care, etc.	Internal	Inquiry
JARVIS – IVE Eligibility Module	Title IV-E program eligibility determination.	Internal	Inquiry
NCANDS	A data collection system that gathers information about reports of child abuse and neglect	External	Batch
NYTD	Tracks which independent living services states provide and assesses the collective outcomes of youth	Internal	Batch
Okta	Statewide identity access management (IAM) system	External	Inquiry
Open Text	Electronic document storage	Internal	Inquiry
ROM	Results-Oriented Management reporting tool	External	Batch
SSA	Foster care updates for SSA file.	External	Batch

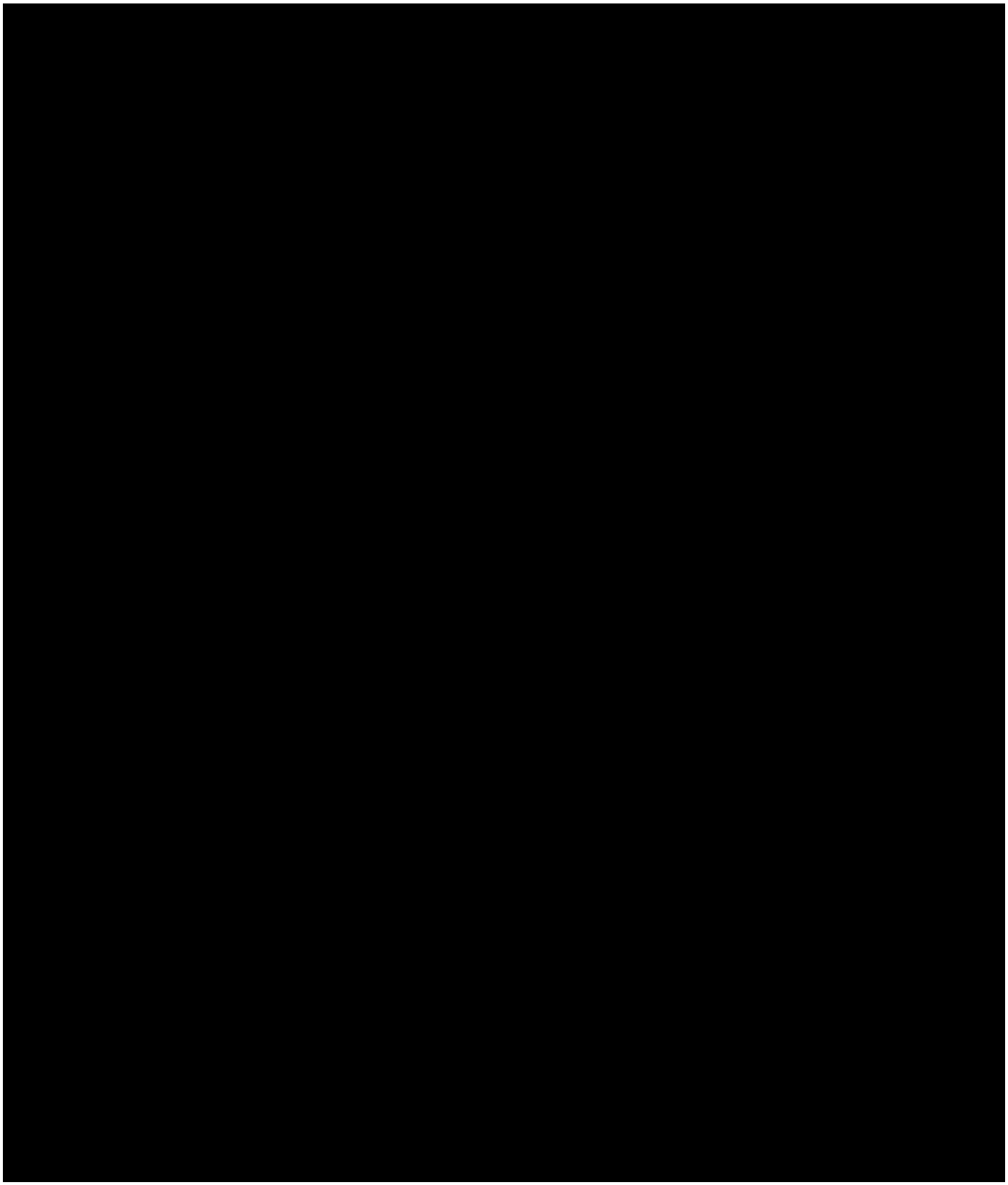
Table 11-2 lists all potential interfaces to be implemented with the new CCWIS.

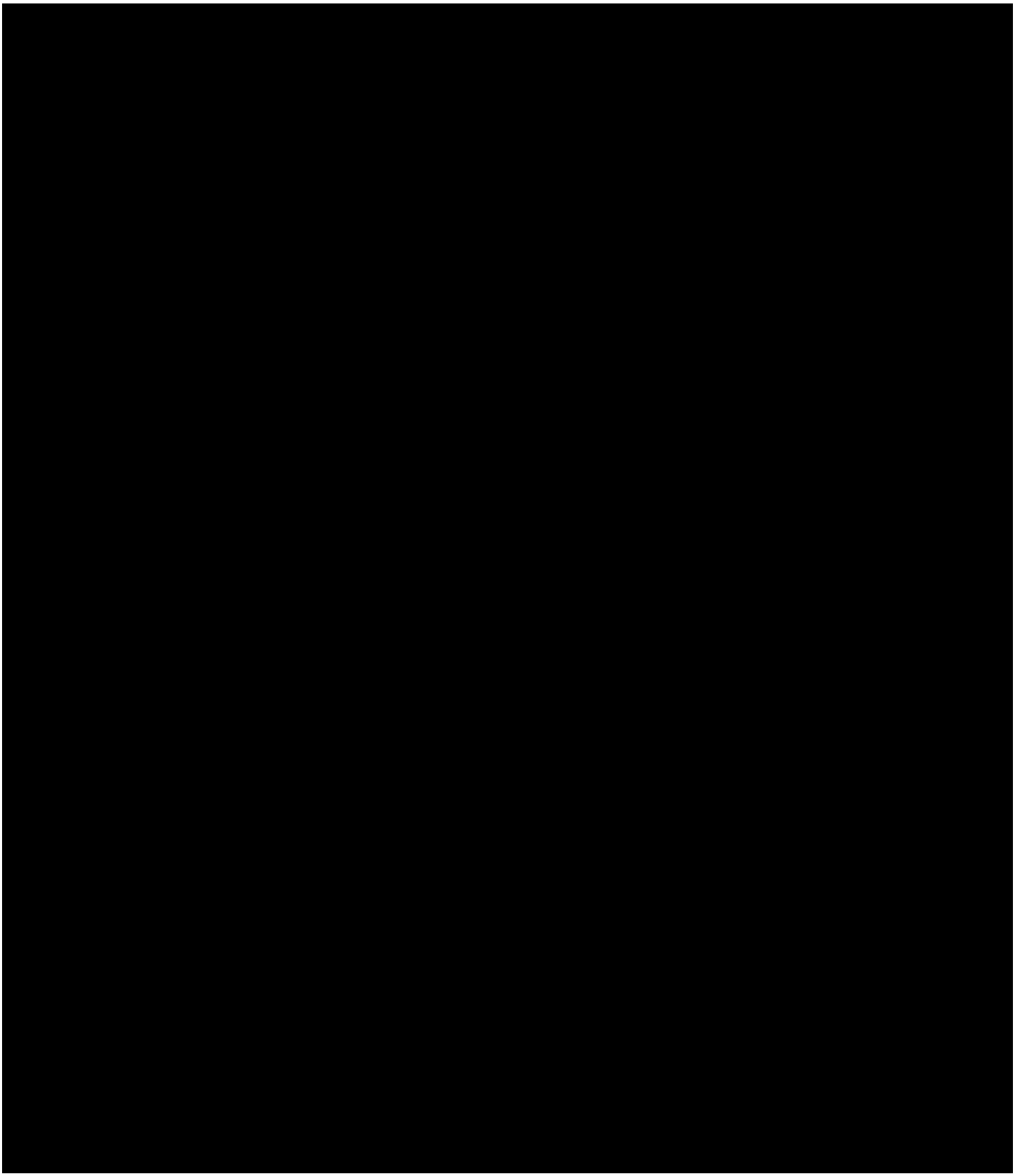
Table 11-2: Description of Potential Internal and External Data Interfaces

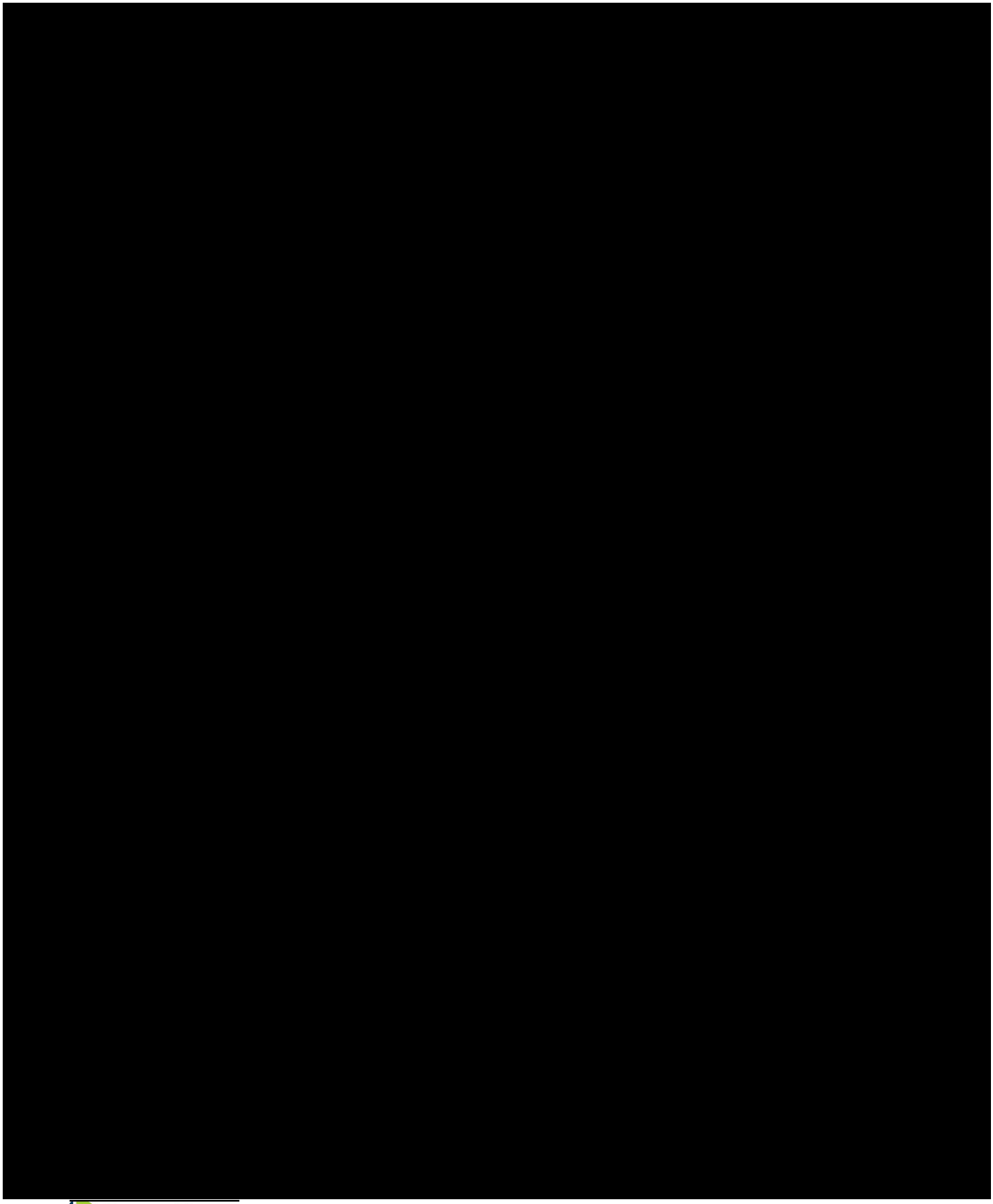
System	Description	Internal/ External	Type of Interface
ARTS	System operated by the DOT used for driver's license scoping	External	Inquiry
CareMatch	System maintained by FivePoints Technology Group used predominantly by foster/adoption parent recruitment, retention, and child matching provider.	External	Inquiry
CLM	Contracting system for Iowa HHS.	Internal	Inquiry
Eligibility Integrated Application Solution (ELIAS)	Establishes eligibility and calculates benefit levels for Modified Adjusted Gross Income (MAGI) Medical	Internal	Inquiry
Google Maps	Performs geographical lookups both for addresses and mapping.	External	Inquiry
Income Eligibility Verification System (IEVS)	Requests, receives, processes and stores information from various data sources (e.g. Internal Revenue Service (IRS), Social Security Administration (SSA), National Directory of New Hires (NDNH), Iowa Department of Public Health (IDPH), etc.)	Internal	Inquiry
Iowa Courts Online	Criminal history used for system checks during intake	External	Inquiry
Public Health	Immunization and vital statistics related information	Internal	Inquiry
ISIS/IoWANS	Waiver service information	Internal	Inquiry
Iowa Workforce Development	State agency providing employment services	External	Inquiry
KinderTrack	System to manage and regulate childcare	Internal	Inquiry
NEICE Clearinghouse	System to create and process ICPC cases	External	Inquiry
SendGrid	Provides email and calendar-based functionality.	Internal	Inquiry
Sex Offender Registry	Access to sex offender registry for background check information	External	Inquiry
WISE	Stores and retrieves data related to the management of Income Maintenance and financial eligibility programs	Internal	Inquiry

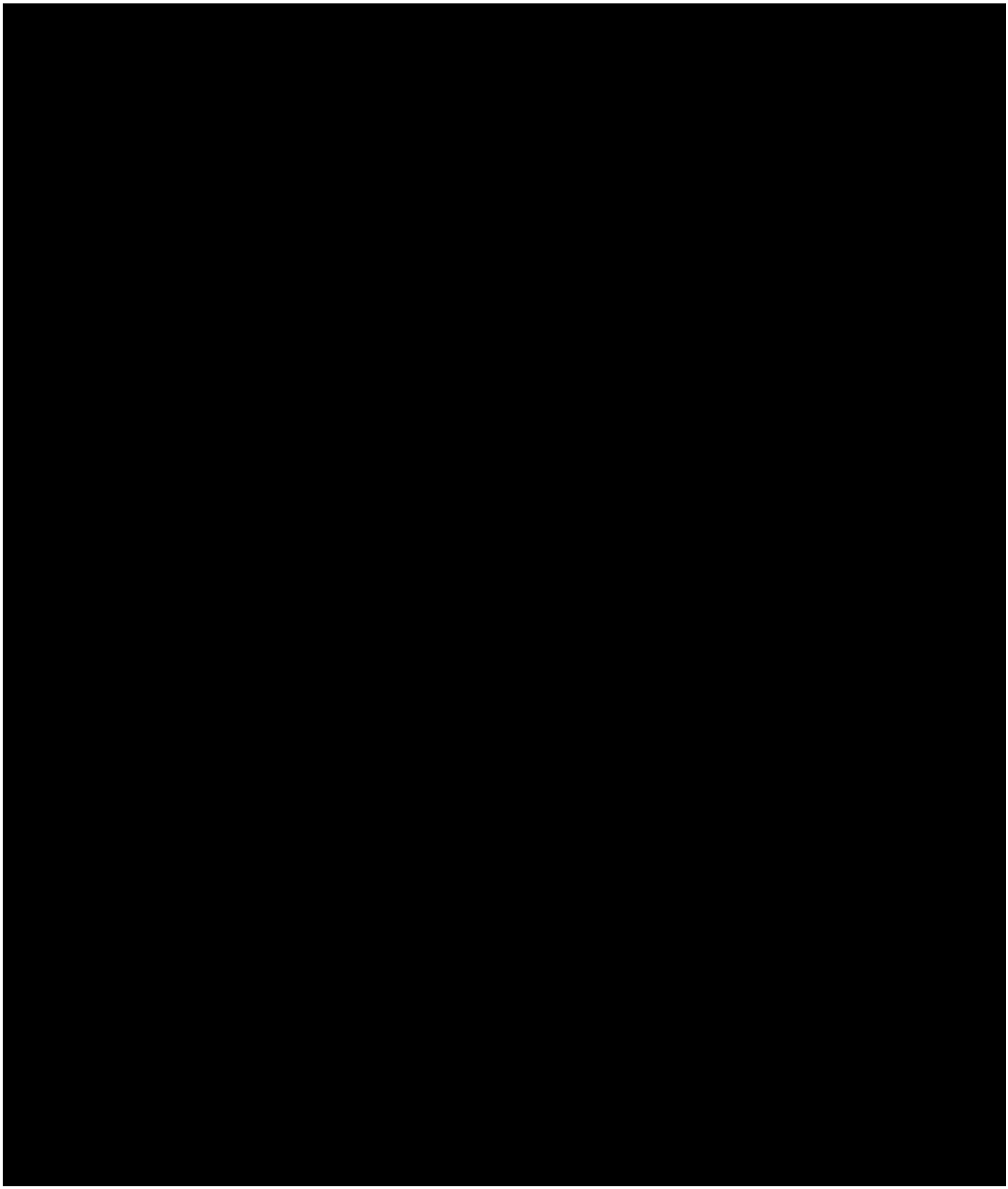
11.2 Security

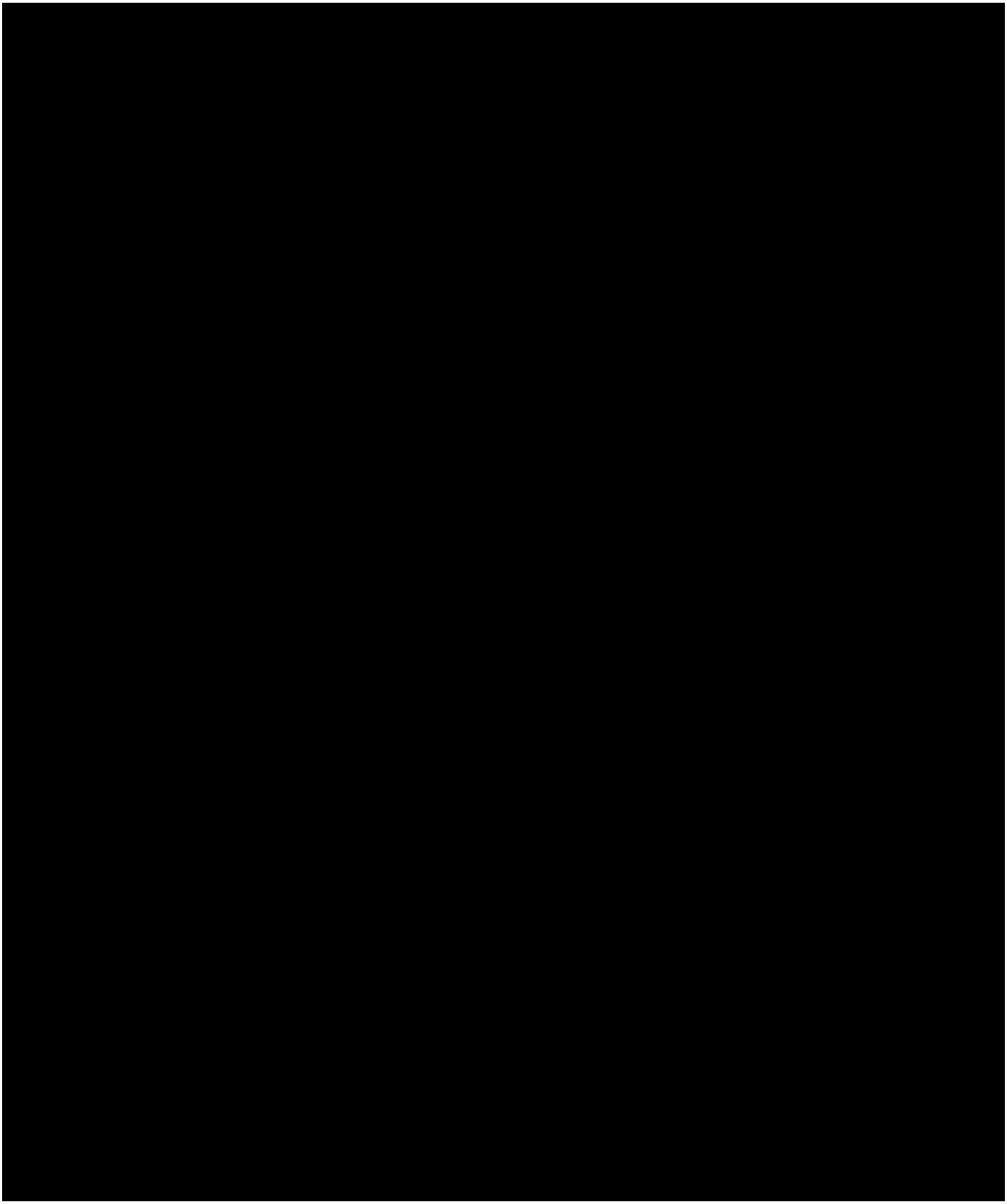














11.3 Disaster Recovery and Business Continuity

Disaster recovery is a framework that consists of policies, tools, and procedures to help prevent or minimize data loss after disruptive events (e.g., cyberattacks, natural disasters, outages). Business continuity is a strategy that helps ensure operations continue after disruptions, focusing on maintaining essential functions with minimal interruption. Disaster recovery and business continuity involve extensive strategic planning, proper resource allocation, and ongoing testing.

The vendor must develop and submit a disaster recovery/Business Continuity Plan that:

- Aligns with federal requirements and is consistent with the agency's requirements

- Describes the Vendor's plan to protect the system and maintain critical business processes in the event of natural disasters, hardware and software failures, human error, or other contingencies that could interrupt services
- Addresses recovery of business functions, human resources, and technology infrastructure associated with the system
- Documents recovery point objectives (RPOs) and recovery time objectives (RTOs)
- Outlines steps required for troubleshooting, replacing, reconfiguring, and restoring the solution hardware, software, and infrastructure
- Outlines processes utilized to verify the health and accuracy of agency solution backups
- Includes the following:
    - Strategies and related procedures for recovery of facilities, hardware, software, data, customer services, and key people responsible for the strategies and related procedures
    - System and data backup, retention, and restoration processes
    - Testing and training procedures for the disaster recovery/Business Continuity Plan
    - Align with disaster recovery provisions set forth in the SLAs

Testing and QA

- The vendor must develop a test plan that includes a summary of processes for disaster recovery testing.

Appendix A: Acronyms

Table A-1: Glossary of Acronyms

Acronym	Definition
AAPDU	Annual Advanced Planning Document Update
ABC	Automated Benefits Calculation
ACCESS	A Comprehensive Community-Based Employment and Support Services
ACF	Administration for Children and Families
AFCARS	Adoption and Foster Care Analysis and Reporting System
APD	Advance Planning Document
API	Application Programming Interface
APSR	Annual Progress and Service Report
ARTS	Automated Records and Tracking System
ATAG	At-a-Glance
BI	Business Intelligence
BPD	Business Process Document
BRD	Business Requirements Document
CAI	Computer Aid Inc.
CAPTA	Child Abuse Prevention and Treatment Act
CB	Children's Bureau
CBA	Cost Benefit Analysis
CCWIS	Comprehensive Child Welfare Information System
CFCIP	Chafee Foster Care Independence Program
CFR	Code of Federal Regulations
CINA	Child in Need of Assistance
CIO	Chief Information Officer
CJIS	Criminal Justice Information Systems
CMS	Centers for Medicare & Medicaid Services
COTS	Commercial-off-the-shelf
DARES	Dependent Adult Abuse Reports
DDI	Design, development, and implementation
DOGE	Department of Government Efficiency
DoIT	Department of Information Technology
DOM	Department of Management

Acronym	Definition
DOT	Department of Transportation
DQP	Data Quality Plan
DSA	Data Sharing Agreements
EA	Enterprise Architecture
EDMS	Electronic Data Management System
ELIAS	Eligibility Integrated Application Solution
FACS	Family and Children Services
FFP	Federal Financial Participation
FFPSA	Family First Prevention Services Act
FRS	Functional Requirements Specification
FS	Feasibility Study
FWP	Family Wellbeing and Protection
FY	Fiscal Year
HHS	Department of Health and Human Services
IA	Iowa
IAPD	Implementation Advance Planning Document
IAS	Interface Agreement Specification
ICAR	Iowa Collections and Reporting
ICPC	Interstate Compact on the Placement of Children
ICWA	Indian Child Welfare Act
IDMS	Integrated Database Management System
IDOC	Iowa Department of Corrections
IDOE	Iowa Department of Education
IDPH	Iowa Department of Public Health
IEVS	Income Eligibility Verification System
IMERS	Iowa Medicaid Electronic Record System
IMPA	Iowa Medicaid Portal Access
IRS	Internal Revenue Service
IV&V	Independent Verification and Validation
JAD	Joint Application Development
JARVIS	Joining Applications and Reports from Various Information Systems
JCS	Juvenile Court Services

Acronym	Definition
MAGI	Modified Adjusted Gross Income
MMIS	Medicaid Management Information System
MOU	Memorandum of Understanding
MVP	Minimum Viable Product
NCANDS	National Child Abuse and Neglect Data System
NDNH	National Directory of New Hires
NEICE	National Electronic Interstate Compact Enterprise
NYTD	National Youth in Transition Database
OAPD	Operation Advanced Planning Document
OCIO	Office of the Chief Information Officer
PAPD	Planning Advance Planning Document
PIP	Performance Improvement Plans
PMBOK®	Project Management Institute's® (PMI®) Project Management Body of Knowledge
PMI®	Project Management Institute®
PMP®	Project Management Plan
QA	Quality Assurance
QC	Quality Control
REP	Abbreviation used for reporting requirements
REST	Representational State Transfer
RFI	Request for Information
RFP	Request for Proposals
ROM	Results Oriented Management
RTM	Requirements Traceability Matrix
SACWIS	Statewide Automated Child Welfare Information System
SBT	Service Business Team
SCC	Security Command Center
SFY	State Fiscal Year
SLA	Service Level Agreement
SME	Subject Matter Expert
SNAP	Supplemental Nutrition Assistance Program
SSA	Social Security Administration
STAR	Safety, Timeliness, Assessment, and Response

Acronym	Definition
TANF	Temporary Assistance for Needy Families
TPR	Termination of Parental Rights
UAT	User Acceptance Testing
VPN	Virtual Private Network
VUDP	Vision Unified Data Platform
WISE	Web Integrated Service Environment

Attachment A: Results Traceability Matrix (RTM)

The Results Traceability Matrix developed with BerryDunn will be submitted separately as part of the submission.